ENGLISH FOR WORK / ENGLISH FOR PATIENT-FACING CLINIC TEAMS

Medical Assisting English

Listen carefully. Clarify the details.



Phil Stilwell

© 2026 Phil Stilwell. All rights reserved.
No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the author, except as permitted by law.

Eight practical lessons. Eleven extended conversations.
Vocabulary, phrase banks, guided speaking, and explained answers.

Revised 2026 edition / Upper-intermediate to advanced

Your route through the book

01 Clarifying the visit at check-in, page 4

Clarify a mismatch between a scheduled visit and a patient concern while respecting the preferred name.

02 Rooming and factual patient reports, page 14

Give a concise patient report that separates the patient words, observed behavior, and unassessed clinical questions.

03 Medication histories with clear sources, page 27

Document a reported medication change without treating an unverified history as a new clinical instruction.

04 Laboratory paperwork and specimen queries, page 37

Preserve collection and arrival times accurately when a colleague proposes an unnecessary record change.

05 Referrals and appointment dependencies, page 49

Explain referral stages and a named follow-up date without accidentally confirming an appointment.

06 Supporting questions during a clinical visit, page 59

Support a patient request for a private explanation while preserving the original clinical instruction.

07 Telephone concerns and clinical escalation, page 72

Confirm a received clinical handoff and record its actual outcome without inventing an assessment or delaying care.

08 Records, corrections, and follow-up ownership, page 82

Correct an appointment message with a clear apology, verified booking details, and an accurate contact record.

Professional communication notes, page 3

Three additional workplace cases, page 95

Answers and explanations, page 109

Phrase finder, page 167

Vocabulary index, page 169

References and publication notes, page 174

Suggested study range: B2-C1. This is an editorial study range, not a certified proficiency assessment.

Accurate details. Respectful conversations.

Medical assistants move between patient language, clinical terminology, and administrative systems. Clear communication preserves what the patient said, distinguishes a recorded event from a judgment, and makes the next responsible person visible.

Attribute a report

Keep the source attached to information. A patient report, an observation, a measurement, and a clinician assessment have different meanings. Do not turn one into another to make a note sound more authoritative.
"Pat reports tiredness since Monday; I have not assessed its cause."

Name the unresolved detail

A useful clarification states exactly what differs. Read the appointment type, medicine-list entry, event time, or message back before proposing a correction. A discrepancy does not prove which source is wrong.
"The local list still includes it; the patient reports an outside instruction."

Give a concrete route

A role boundary should lead to the right person or process. Preserve the question and explain the immediate next step. Do not leave a clinical concern waiting for a routine administrative update.
"Nurse Dana is available through our clinical-call route now."

Separate progress from completion

Received does not mean booked, and an attempted call does not mean the patient received the correction. Name the actual stage, unfinished work, and owner so a colleague can continue accurately.
"The referral is under review; Friday is a status check, not an appointment."

Professional boundaries

All patients, records, times, and cases are fictional. This book teaches workplace English, not clinical assessment, triage, prescribing, specimen procedures, legal advice, or professional certification. Duties depend on local law, training, delegation, supervision, and clinic policy. Follow actual clinical and emergency procedures; these dialogues do not establish that any symptom can safely wait. US sources provide context, not universal rules. Use actual dates and required identifiers in real records.

A Read the situation

LESSON 01 / CLARIFYING THE VISIT AT CHECK-IN

Clarify a mismatch between a scheduled visit and a patient concern while respecting the preferred name.

The schedule and the expectation

The schedule lists an annual review for patient Alexandra, whose verified record has already been matched through the clinic process. The patient prefers Alex and expected to discuss a recent concern. Assistant Sam cannot decide whether the appointment type is suitable. Nurse Dana is available to clarify the visit plan. No decision about the concern, billing, urgency, or an additional appointment has been supplied. Sam must acknowledge the expectation, preserve accurate identification, use Alex in conversation, and ask Dana to clarify the clinical plan.

People: Alex, Patient; Sam, Medical assistant.

Read the record

Choose one answer per question using the facts above.

1. What does the schedule show?

A. An annual review
B. A confirmed procedure for the recent concern
C. An emergency assessment already completed
D. A canceled visit

2. What should Sam do with the name preference?

A. Open another record under Alex to avoid the mismatch.
B. Replace verified identifiers without following the correction process.
C. Use Alexandra in conversation until the appointment type changes.
D. Use Alex while preserving the verified record match.

3. Who can clarify the visit plan?

A. Another waiting patient
B. A billing code alone
C. Nurse Dana
D. Sam making an independent clinical decision

Answers and explanations, page 109

B Vocabulary in use

CORE TERMS

check-in
Arrival process confirming a patient and the scheduled visit.

complete check-in

annual review
Periodic review visit recorded as occurring yearly.

attend an annual review

chief complaint
Clinical-record term for the main reason for seeking care.

clarify the chief complaint

preferred name
Name a person asks others to use when addressing them.

use the preferred name

patient identifier
Information used to match a person with the correct record.

confirm patient identifiers

registration
Creation or updating of required administrative patient information.

complete registration

appointment type
Category describing the scheduled purpose or format of a visit.

verify the appointment type

presenting concern
Issue the patient brings to the current visit.

record the presenting concern

visit agenda
Topics planned for discussion during the encounter.

clarify the visit agenda

recorded name
Name shown in the relevant patient record.

verify the recorded name

demographic details
Administrative identity and contact information in a record.

verify demographic details

encounter
A particular interaction or visit involving healthcare services.

document the encounter

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

check-in CHECK-IN

complete check-in

Keep the meaning precise: Arrival process confirming a patient and the scheduled visit.

visit agenda VISIT AGENDA

clarify the visit agenda

Keep the meaning precise: Topics planned for discussion during the encounter.

B Precision and professional shorthand

EXTEND YOUR RANGE

appointment slot

Reserved time in a schedule.

confirm the appointment slot

scheduled purpose

Reason for a visit as recorded in the booking.

state the scheduled purpose

patient expectation

What a patient believes will happen during a service or visit.

acknowledge the patient expectation

clinical suitability

Whether a plan or appointment is appropriate for the clinical need.

refer a suitability question

scope of practice

Activities permitted within a role under applicable requirements.

work within scope of practice

delegation

Assignment of an activity under the applicable professional rules.

clarify the delegation

supervision

Required oversight of work by an appropriate professional.

confirm supervision arrangements

clinical handoff

Transfer of relevant information to a receiving clinical professional.

complete a clinical handoff

visit plan

Intended approach to the current appointment.

clarify the visit plan

billing category

Classification used for charging or reimbursement purposes.

verify the billing category

identity match

Confirmation that a person corresponds to the correct record.

establish the identity match

name pronunciation

The way a person's name is spoken.

check name pronunciation

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

encounter ENCOUNTER

document the encounter

Keep the meaning precise: A particular interaction or visit involving healthcare services.

scope of practice SCOPE OF PRACTICE

work within scope of practice

Keep the meaning precise: Activities permitted within a role under applicable requirements.

C Phrases that move the work forward

Use the preferred name
Thank you, Alex; I will use that name.

Acknowledge the expectation
You expected to discuss a recent concern.

Avoid a clinical decision
I cannot decide whether this appointment type meets that need.

Preserve identification
We will keep the verified record linked correctly.

Separate billing
I cannot determine charges from this appointment label alone.

Protect privacy
We can use the appropriate setting for personal details.

Ask for clarification
Dana, can you clarify the visit plan with Alex?

Avoid duplicate records
We should use the correctly matched existing record.

State the schedule
The schedule lists an annual review.

Name the difference
The recorded visit type and your expectation are not the same.

Offer a named route
Nurse Dana can clarify the visit plan.

Avoid blame
Let us clarify the plan before assuming how the mismatch happened.

Keep the concern visible
I will include your stated concern in the handoff.

Do not promise treatment
I cannot promise a particular clinical outcome.

Confirm the preference
The verified record says Alexandra; the preferred spoken name is Alex.

Close the loop
I will explain the mismatch to Dana so you do not have to start from nothing.

Precision in real exchanges

Purpose: Name the difference
Useful wording: The recorded visit type and your expectation are not the same.

Purpose: Avoid a clinical decision
Useful wording: I cannot decide whether this appointment type meets that need.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which handoff preserves both facts?

A. The schedule says annual review; Alex expected to discuss a recent concern.
B. Alex booked incorrectly and must leave.
C. Annual review guarantees treatment of every concern.
D. The record is definitely for another person.

2. What does using Alex change here?

A. The billing category automatically
B. The respectful form of spoken address, not the verified identity match
C. The need for all patient identifiers
D. The clinical urgency of the concern

3. A colleague needs the term for this meaning: Issue the patient brings to the current visit. Which term fits?

A. presenting concern
B. patient expectation
C. patient identifier
D. visit plan

D Choose the response that fits / continued

4. Rebuild the exchange from "The schedule and the expectation". Alex: "Good. I worry that the appointment category means nobody will hear the reason I actually came. Do I need to book something else?" Sam: [missing response] Alex: "I do not want to explain personal details here in front of everyone. Can the question reach Dana without a discussion across reception?" Which response belongs in the gap?

A. Yes. I can identify the mismatch and use the appropriate private route for the presenting concern. We do not need to announce personal details to the waiting room.

B. I cannot decide clinical suitability from the schedule label. Dana can clarify how the visit should address your concern before we assume that another booking is required.

C. Correct. I will record the appointment type accurately and pass on your expectation. We will keep the two facts clear while Dana clarifies the next step.

D. I will handle that preference through our record process while preserving the identity match. The name used in conversation and the correctly matched record need not conflict.

Answers and explanations, page 109

> **One useful distinction**
>
> An appointment label describes a scheduled category, not a clinical judgment about every concern raised that day. Do not promise that the concern will be resolved, reject it automatically, or invent a billing outcome from the schedule alone.

E Conversation

The schedule and the expectation

The schedule and the expectation

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

scheduled purpose / patient expectation / appointment type / billing category / visit plan / preferred name / clinical handoff / presenting concern / clinical suitability / identity match

01 **Alex:** I am here for my appointment, but I noticed the reminder says annual review. I thought this was the visit for the concern I called about.

02 **Sam:** The [1] _____ is an annual review. Thank you for explaining what you expected; I will ask nurse Dana to clarify the plan.

03 **Alex:** Before we go further, please call me Alex. Alexandra is the name in my records, but I use Alex in conversation and at work.

04 **Sam:** Of course, Alex. Your [2] _____ is clear, and we will keep the verified record correctly linked. I am not creating another record because you use a shorter name.

05 **Alex:** Good. I worry that the appointment category means nobody will hear the reason I actually came. Do I need to book something else?

06 **Sam:** I cannot decide [3] _____ from the schedule label. Dana can clarify how the visit should address your concern before we assume that another booking is required.

07 **Alex:** I do not want to explain personal details here in front of everyone. Can the question reach Dana without a discussion across reception?

08 **Sam:** Yes. I can identify the mismatch and use the appropriate private route for the [4] _____. We do not need to announce personal details to the waiting room.

09 **Alex:** Will you tell Dana that I expected this to be the concern visit, rather than just saying that I am unhappy about the reminder?

10 **Sam:** I will preserve that [5] _____. The handoff will say the schedule lists an annual review and you expected a discussion of a recent concern.

Conversation continues on the next page.

The schedule and the expectation

CONVERSATION / CONTINUED

11 **Alex:** Is this a billing problem too? I have heard different visit categories can mean different charges, and I do not want a surprise.

12 **Sam:** A [6] _____ is another question to clarify through the proper process. I cannot promise a charge or coverage outcome from the label alone.

13 **Alex:** Please check before anyone promises it is covered. I took time off for this visit, and I need to understand what will actually happen today.

14 **Sam:** I can confirm the schedule and your expectation, then request a [7] _____ to Dana. I cannot promise how the clinician will resolve the concern.

15 **Alex:** Please also make sure the team uses Alex. Sometimes one person agrees, but the preference disappears as soon as I move to another room.

16 **Sam:** I will handle that preference through our record process while preserving the [8] _____. The name used in conversation and the correctly matched record need not conflict.

17 **Alex:** Thank you. I do not need you to change the appointment yourself; I need someone qualified to clarify what today's visit can include.

18 **Sam:** That is the [9] _____ question for Dana. I will not turn the appointment label into a refusal or a guarantee before that discussion.

19 **Alex:** All right. Can you let Dana know I am here? I would like to settle the visit plan before I explain everything again.

20 **Sam:** Correct. I will record the [10] _____ accurately and pass on your expectation. We will keep the two facts clear while Dana clarifies the next step.

CHECK THE GAPS

1. scheduled purpose / **2.** preferred name / **3.** clinical suitability / **4.** presenting concern / **5.** patient expectation / **6.** billing category / **7.** clinical handoff / **8.** identity match / **9.** visit plan / **10.** appointment type

Why these answers fit, page 110

F Check accuracy and scope

The schedule and the expectation

Return to the case

The schedule lists an annual review for patient Alexandra, whose verified record has already been matched through the clinic process. The patient prefers Alex and expected to discuss a recent concern. Assistant Sam cannot decide whether the appointment type is suitable. Nurse Dana is available to clarify the visit plan. No decision about the concern, billing, urgency, or an additional appointment has been supplied. Sam must acknowledge the expectation, preserve accurate identification, use Alex in conversation, and ask Dana to clarify the clinical plan.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which reassurance is unsupported?

A. The schedule says annual review.
B. Sam cannot decide clinical suitability.
C. This visit will definitely resolve the concern at no additional cost.
D. Dana can clarify the plan.

2. Which action is appropriate?

A. Pass the mismatch to Dana while keeping the correctly matched record.
B. Create another record because the patient prefers Alex.
C. Dismiss the concern because it is not in the schedule label.
D. Change the clinical plan independently.

Answers and explanations, page 112

Listen for the distinction

Switch roles. Repeat the privacy request and preferred-name exchange without changing the verified identity facts.

Respect the person and verify the record

A preferred name and a verified record name can coexist. Use the requested form of address while following the actual identification process. A mismatch in visit expectations deserves clarification rather than blame for booking or an automatic refusal to discuss the concern.

F Say it / transfer to a new case

Clarify a different appointment expectation

The schedule says follow-up. Patient Morgan expected an initial consultation. Assistant Lee cannot decide suitability. Nurse Inez is available to clarify the plan.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

Inez / follow-up / initial consultation / established

01 **Lee:** The recorded visit is a [1] _____.

02 **Morgan:** I expected an [2] _____.

03 **Lee:** Nurse [3] _____ can clarify the plan.

04 **Lee:** Clinical suitability is not yet [4] _____.

CHECK THE GAPS

1. follow-up / **2.** initial consultation / **3.** Inez / **4.** established

Why these answers fit, page 112

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "The schedule and the expectation". Alex: "Please check before anyone promises it is covered. I took time off for this visit, and I need to understand what will actually happen today." Sam: [missing response] Alex: "Please also make sure the team uses Alex. Sometimes one person agrees, but the preference disappears as soon as I move to another room." Which response belongs in the gap?

A. That is the visit plan question for Dana. I will not turn the appointment label into a refusal or a guarantee before that discussion.

B. I can confirm the schedule and your expectation, then request a clinical handoff to Dana. I cannot promise how the clinician will resolve the concern.

C. A billing category is another question to clarify through the proper process. I cannot promise a charge or coverage outcome from the label alone.

D. I will preserve that patient expectation. The handoff will say the schedule lists an annual review and you expected a discussion of a recent concern.

Answers and explanations, page 113

A Read the situation

LESSON 02 / ROOMING AND FACTUAL PATIENT REPORTS

Give a concise patient report that separates the patient words, observed behavior, and unassessed clinical questions.

Report, observation, assessment

During rooming, patient Pat says, "I have felt tired since Monday." Assistant Rosa observed Pat answering questions while seated. Rosa has not assessed the cause or severity, and no measurement values are supplied. Nurse Dana is receiving the report. The conversation must preserve the onset as reported by Pat, state the limited observation, and avoid filling missing fields with normal values. Sitting and answering questions do not establish that the concern is mild or safe to defer. Actual urgency must follow the clinic clinical process.

People: Rosa, Medical assistant; Dana, Receiving nurse.

Read the record

Choose one answer per question using the facts above.

1. What onset did Pat report?

A. This morning confirmed by testing
B. No symptoms ever
C. Monday
D. A date measured by Rosa

2. What did Rosa observe?

A. A completed severity assessment
B. Pat answering questions while seated
C. Normal blood pressure
D. A diagnosed cause of tiredness

3. What should remain unassessed in the report?

A. Cause and severity
B. Whether Pat spoke
C. Whether Dana is receiving the report
D. Whether Monday was mentioned

Answers and explanations, page 114

B Vocabulary in use

CORE TERMS

rooming
Preparing a patient for the clinical encounter under the clinic process.

complete the rooming process

patient-reported symptom
Experience described by the patient rather than independently observed.

attribute a patient-reported symptom

onset
When a symptom or condition began.

clarify the reported onset

duration
Length of time an experience or condition has continued.

document symptom duration

severity
Degree or intensity of a condition or symptom.

assess severity

observation
Something directly noticed and described without inventing its cause.

record an observation

subjective information
Information based on a person's reported experience.

attribute subjective information

objective finding
Observed or measured finding, with its source and context.

document an objective finding

baseline
Usual or earlier state used for comparison.

establish the baseline

change from baseline
Difference from a known usual or previous state.

report a change from baseline

associated symptom
Additional symptom occurring with the main concern.

ask about associated symptoms

functional impact
Effect on a person's ability to perform ordinary activities.

describe functional impact

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

rooming ROOMING

complete the rooming process

Keep the meaning precise: Preparing a patient for the clinical encounter under the clinic process.

observation OBSERVATION

record an observation

Keep the meaning precise: Something directly noticed and described without inventing its cause.

B Precision and professional shorthand

EXTEND YOUR RANGE

vital signs

Physiological measurements such as pulse and temperature.

record measured vital signs

blood pressure

Pressure exerted by blood within the circulation, measured with relevant units.

document blood pressure

pulse rate

Number of palpable arterial pulses per unit of time.

measure pulse rate

respiratory rate

Number of breaths per unit of time.

record respiratory rate

oxygen saturation

Percentage of hemoglobin oxygen-binding sites occupied by oxygen, estimated or measured by an appropriate method.

report oxygen saturation

temperature

Measure of how hot or cold the body or another object is.

record the measured temperature

unit of measurement

Standard used to express a measured quantity.

include the unit of measurement

measurement time

Time at which a measurement was actually taken.

preserve the measurement time

clinical assessment

Qualified evaluation of health information and the patient's condition.

request clinical assessment

unassessed

Not evaluated through the relevant assessment.

mark severity as unassessed

B Precision and professional shorthand / continued

pertinent negative

Relevant absence established by asking or assessing, not by silence.

document a pertinent negative

source attribution

Identification of where a statement or finding came from.

preserve source attribution

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

functional impact FUNCTIONAL IMPACT

describe functional impact

Keep the meaning precise: Effect on a person's ability to perform ordinary activities.

oxygen saturation OXYGEN SATURATION

report oxygen saturation

Keep the meaning precise: Percentage of hemoglobin oxygen-binding sites occupied by oxygen, estimated or measured by an appropriate method.

C Phrases that move the work forward

Attribute the words
Pat reports feeling tired since Monday.

Quote accurately
Pat said, "I have felt tired since Monday."

Describe the observation
I observed Pat answering questions while seated.

Name the limit
I have not assessed the cause or severity.

Do not invent values
No measurement values are available in this report.

Separate sources
That is the patient report, not a measured finding.

Keep onset attributed
Monday is the onset Pat reported.

Avoid a vague reassurance
I cannot describe the concern as mild from that observation.

State what is missing
The relevant clinical assessment is still needed.

Do not imply absence
An unreported symptom is not automatically absent.

Keep the handoff concise
I will give the report, observation, and outstanding assessment separately.

Use units accurately
Any actual measurement needs its correct value, unit, and context.

Refer the judgment
Dana, please assess the clinical concern through our process.

Preserve uncertainty
I do not have a verified explanation for the tiredness.

Confirm receipt
Have you received Pat's report and the limits of my observation?

Close factually
The cause and severity remain unassessed in my report.

Precision in real exchanges

Purpose: Name the limit
Useful wording: I have not assessed the cause or severity.

Purpose: Do not invent values
Useful wording: No measurement values are available in this report.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which report is supported?

A. Pat is safe to wait because Pat can speak.

B. Pat reports tiredness since Monday; I observed Pat answering questions while seated.

C. Pat has a mild condition caused by poor sleep.

D. All vital signs are normal.

2. What belongs in the temperature field when no measurement is available?

A. A normal category inferred from Pat answering questions

B. A numeric value inferred from the reported tiredness

C. No invented numeric value

D. A previous visit's value entered as today's reading

3. A colleague needs the term for this meaning: Length of time an experience or condition has continued. Which term fits?

A. baseline

B. clinical assessment

C. pulse rate

D. duration

D Choose the response that fits / continued

4. Rebuild the exchange from "Report, observation, assessment". Rosa: "The handoff draft says looks fine. I will replace that with what I saw: Pat was seated and answering my questions." Dana: [missing response] Rosa: "I also need to flag the blank measurement fields. I have no values to report; those fields should not be read as normal findings." Which response belongs in the gap?

A. Exactly. Keep the observation specific. Answering questions while seated does not establish the severity of tiredness or whether a concern can safely wait.

B. That clearly identifies what is unassessed. It does not minimize the concern, and it leaves the clinical questions visible for the proper assessment.

C. Right. A baseline is not something we should invent. Your report can remain useful without adding a comparison that has not been obtained.

D. No. A pertinent negative must come from the relevant question or assessment. Lack of a statement in this report is not a denial.

Answers and explanations, page 114

One useful distinction

Not reported does not mean absent, and not measured does not mean normal. A pertinent negative requires a relevant question or assessment. Do not populate a record from expectation or convert an empty field into reassurance.

E Conversation

Report, observation, assessment

Report, observation, assessment

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

baseline / source attribution / observation / patient-reported symptom / pertinent negative / Vital signs / unassessed / Clinical assessment / onset / severity

01 **Rosa:** Dana, I have Pat's rooming report. Pat said, "I have felt tired since Monday." I want to keep that separate from what I observed.

02 **Dana:** Please give the [1] _____ clearly. What came from Pat, what did you directly observe, and what has not been assessed?

03 **Rosa:** The tiredness and Monday starting point came from Pat. I observed Pat answering questions while seated. I have not assessed the cause or severity.

04 **Dana:** Good. The reported [2] _____ is Monday. Do not replace it with a guessed date or calculate a duration from an assumed calendar.

05 **Rosa:** The handoff draft says looks fine. I will replace that with what I saw: Pat was seated and answering my questions.

06 **Dana:** Exactly. Keep the [3] _____ specific. Answering questions while seated does not establish the severity of tiredness or whether a concern can safely wait.

07 **Rosa:** I also need to flag the blank measurement fields. I have no values to report; those fields should not be read as normal findings.

08 **Dana:** Correct. [4] _____ need actual values and appropriate context. A missing measurement cannot be converted into normal by choosing a familiar number.

09 **Rosa:** Should I write that Pat denied other symptoms? None are included in the information I am handing over, but I have not established their absence.

10 **Dana:** No. A [5] _____ must come from the relevant question or assessment. Lack of a statement in this report is not a denial.

Conversation continues on the next page.

Report, observation, assessment

CONVERSATION / CONTINUED

11 **Rosa:** I will keep the original wording in quotation marks when I use it exactly. If I summarize, I will still identify Pat as the source.

12 **Dana:** That preserves the [6] _____. Do not translate tired into a specific diagnosis or imply that Pat used a technical term they did not actually say.

13 **Rosa:** I also cannot say this is a change from Pat's normal condition unless I have an established usual state to compare it with.

14 **Dana:** Right. A [7] _____ is not something we should invent. Your report can remain useful without adding a comparison that has not been obtained.

15 **Rosa:** The handoff can be short: Pat reports tiredness since Monday; observed answering questions while seated; cause and severity not assessed by me.

16 **Dana:** That clearly identifies what is [8] _____. It does not minimize the concern, and it leaves the clinical questions visible for the proper assessment.

17 **Rosa:** Have you received those points? I do not want the phrase answering questions to be passed on as a formal severity rating later.

18 **Dana:** I have received the report and its limits. [9] _____ is separate, and the concern will follow our clinical process rather than an administrative assumption.

19 **Rosa:** I will record the report accordingly and preserve what I actually observed. I will not add a measurement, diagnosis, or assurance about waiting.

20 **Dana:** Thank you. Keep the [10] _____ question open in that record. Clear limits help the receiving team know what still requires attention.

CHECK THE GAPS

1. source attribution / **2.** onset / **3.** observation / **4.** Vital signs / **5.** pertinent negative / **6.** patient-reported symptom / **7.** baseline / **8.** unassessed / **9.** Clinical assessment / **10.** severity

Why these answers fit, page 115

F Check accuracy and scope

Report, observation, assessment

Return to the case

During rooming, patient Pat says, "I have felt tired since Monday." Assistant Rosa observed Pat answering questions while seated. Rosa has not assessed the cause or severity, and no measurement values are supplied. Nurse Dana is receiving the report. The conversation must preserve the onset as reported by Pat, state the limited observation, and avoid filling missing fields with normal values. Sitting and answering questions do not establish that the concern is mild or safe to defer. Actual urgency must follow the clinic clinical process.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Why is "Pat is fine" inadequate?

A. It preserves the patient quotation.

B. It identifies a measured oxygen level.

C. It substitutes a broad conclusion for the limited observation.

D. It gives the exact onset.

2. Which statement about other symptoms is accurate?

A. Every blank field can be completed as normal.

B. No other symptoms are supplied; that does not prove their absence.

C. Every unmentioned symptom was denied.

D. Pat has no associated symptoms.

Answers and explanations, page 117

Listen for the distinction

Switch roles. Repeat the unmeasured-vitals and pertinent-negative exchanges without adding a normal finding.

Neutral is not dismissive

A factual report can be both concise and attentive. Use the patient words and name the limits of the observation. Avoid vague labels such as fine or difficult, which replace useful information with an impression and can distort the clinical handoff.

F Say it / transfer to a new case

Attribute a different symptom report

Patient Lee reports dizziness since breakfast. Assistant Noor observed Lee speaking while seated. No measurements or cause assessment are supplied. Nurse Inez receives the report.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

dizziness / seated / unassessed / breakfast

01 **Noor:** Lee reports [1] _____ since breakfast.

02 **Inez:** The reported onset is [2] _____.

03 **Noor:** I observed Lee speaking while [3] _____.

04 **Inez:** The cause remains [4] _____.

CHECK THE GAPS

1. dizziness / **2.** breakfast / **3.** seated / **4.** unassessed

Why these answers fit, page 117

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Report, observation, assessment". Rosa: "I also cannot say this is a change from Pat's normal condition unless I have an established usual state to compare it with." Dana: [missing response] Rosa: "The handoff can be short: Pat reports tiredness since Monday; observed answering questions while seated; cause and severity not assessed by me." Which response belongs in the gap?

A. Exactly. Keep the observation specific. Answering questions while seated does not establish the severity of tiredness or whether a concern can safely wait.

B. No. A pertinent negative must come from the relevant question or assessment. Lack of a statement in this report is not a denial.

C. That clearly identifies what is unassessed. It does not minimize the concern, and it leaves the clinical questions visible for the proper assessment.

D. Right. A baseline is not something we should invent. Your report can remain useful without adding a comparison that has not been obtained.

Answers and explanations, page 118

Review 1 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Clarifying the visit at check-in" and "Rooming and factual patient reports". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Periodic review visit recorded as occurring yearly. Which term fits?

A. annual review
B. scheduled purpose
C. clinical handoff
D. recorded name

2. A colleague needs the term for this meaning: Observed or measured finding, with its source and context. Which term fits?

A. vital signs
B. unit of measurement
C. rooming
D. objective finding

3. Rebuild the exchange from "The schedule and the expectation". Alex: "Will you tell Dana that I expected this to be the concern visit, rather than just saying that I am unhappy about the reminder?" Sam: [missing response] Alex: "Is this a billing problem too? I have heard different visit categories can mean different charges, and I do not want a surprise." Which response belongs in the gap?

A. A billing category is another question to clarify through the proper process. I cannot promise a charge or coverage outcome from the label alone.
B. I can confirm the schedule and your expectation, then request a clinical handoff to Dana. I cannot promise how the clinician will resolve the concern.
C. That is the visit plan question for Dana. I will not turn the appointment label into a refusal or a guarantee before that discussion.
D. I will preserve that patient expectation. The handoff will say the schedule lists an annual review and you expected a discussion of a recent concern.

Review 1 / Meaning and context / continued

4. Rebuild the exchange from "Report, observation, assessment". Rosa: "Have you received those points? I do not want the phrase answering questions to be passed on as a formal severity rating later." Dana: [missing response] Rosa: "I will record the report accordingly and preserve what I actually observed. I will not add a measurement, diagnosis, or assurance about waiting." Which response belongs in the gap?

A. No. A pertinent negative must come from the relevant question or assessment. Lack of a statement in this report is not a denial.

B. I have received the report and its limits. Clinical assessment is separate, and the concern will follow our clinical process rather than an administrative assumption.

C. That preserves the patient-reported symptom. Do not translate tired into a specific diagnosis or imply that Pat used a technical term they did not actually say.

D. Exactly. Keep the observation specific. Answering questions while seated does not establish the severity of tiredness or whether a concern can safely wait.

Answers and explanations, page 149

A Read the situation

LESSON 03 / MEDICATION HISTORIES WITH CLEAR SOURCES

Document a reported medication change without treating an unverified history as a new clinical instruction.

> **Two sources, one unresolved entry**
>
> Patient Ren reports that another clinic stopped one medicine. The local medication list still includes it. Ren does not have the outside instruction available. Assistant Talia cannot verify discontinuation, change the list as a clinical decision, or tell Ren to start, stop, or restart treatment. Nurse Dana can review the discrepancy with the clinician. The immediate communication task is to record the patient report and the conflicting local entry with clear sources, then connect the question with Dana. No medicine name, dose, or timing decision is supplied.

People: Ren, Patient; Talia, Medical assistant.

Read the record

Choose one answer per question using the facts above.

1. What does Ren report?

A. A new verified prescription has arrived here.
B. The local clinician has confirmed a restart.
C. The medication list is already reconciled.
D. Another clinic stopped one medicine.

2. What conflicts with that report?

A. The medicine remains on the local list.
B. A supplied laboratory result
C. An established allergy reaction
D. A confirmed replacement dose

3. What can Dana do?

A. Guarantee that Ren should restart treatment
B. Prove the outside clinic was wrong without evidence
C. Treat the local list as a new prescription automatically
D. Review the discrepancy with the clinician

Answers and explanations, page 119

B Vocabulary in use

CORE TERMS

medication history
Account of medicines used or reported, with relevant sources and timing.

obtain a medication history

medication reconciliation
Process of comparing medication information and resolving discrepancies appropriately.

support medication reconciliation

local medication list
Medicine record held by the current organization.

review the local medication list

outside instruction
Direction originating from another healthcare source.

verify the outside instruction

discontinuation
Ending a medicine or order through the relevant clinical process.

confirm discontinuation

patient report
Information supplied by the patient about their experience or history.

document the patient report

active entry
Record item currently marked active in a system.

verify an active entry

historical entry
Record of earlier information rather than necessarily current treatment.

distinguish a historical entry

duplicate entry
Repeated record item that may need clarification.

investigate a duplicate entry

omission
Information missing from a list or account.

identify a possible omission

discrepancy
Difference between sources requiring clarification.

flag a medication discrepancy

prescribing clinician
Professional authorized to prescribe the relevant medicine.

contact the prescribing clinician

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

medication history MEDICATION HISTORY

obtain a medication history

Keep the meaning precise: Account of medicines used or reported, with relevant sources and timing.

patient report PATIENT REPORT

document the patient report

Keep the meaning precise: Information supplied by the patient about their experience or history.

B Precision and professional shorthand

EXTEND YOUR RANGE

dose

Amount intended to be taken or administered at one time.

verify the prescribed dose

strength

Amount of active ingredient in a specified unit of product.

distinguish strength from dose

dosage form

Physical form of a medicine, such as tablet or solution.

confirm the dosage form

route

Way a medicine is taken or administered.

verify the route

frequency

How often a medicine is taken or administered.

clarify the frequency

as-needed instruction

Direction allowing use under specified conditions rather than a fixed schedule alone.

verify the as-needed instruction

over-the-counter medicine

Medicine available without a prescription under applicable rules.

record over-the-counter medicines

dietary supplement

Product used to supplement dietary intake, subject to relevant regulation.

ask about dietary supplements

allergy history

Account of suspected or confirmed allergic reactions with relevant details.

document allergy history

adverse reaction

Harmful or unwanted response associated with a medicine or other exposure.

report an adverse reaction

adherence

Extent to which medicine use corresponds with the agreed plan.

discuss adherence without blame

verification source

Person, document, or system used to confirm information.

identify the verification source

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

prescribing clinician PRESCRIBING CLINICIAN

contact the prescribing clinician

Keep the meaning precise: Professional authorized to prescribe the relevant medicine.

frequency FREQUENCY

clarify the frequency

Keep the meaning precise: How often a medicine is taken or administered.

C Phrases that move the work forward

Acknowledge the report

You report that another clinic stopped this medicine.

State the local source

The local medication list still includes it.

Name the discrepancy

Those two sources need to be reconciled.

Avoid judging memory

I will record what you remember and ask the clinical team to check the difference.

Keep attribution

I will record that this information came from you.

Identify the missing evidence

The outside instruction is not available here.

Give the review route

Dana can review the discrepancy with the clinician.

Avoid treatment advice

I cannot tell you to start, stop, or restart it.

Do not silently delete

We need the appropriate review before treating the entry as resolved.

Preserve the details

I will keep the original source and the reported change clear.

Separate strength and dose

The product strength and prescribed amount are different details.

Include nonprescription use

Relevant nonprescription medicines and supplements also belong in an accurate history.

Do not invent a reaction

No allergy or adverse reaction is established by this discrepancy.

Keep urgency separate

A current medicine concern needs the appropriate clinical route.

Confirm the handoff

Dana will receive both the patient report and the local-list difference.

Close without a new instruction

This conversation records the question; it does not change your treatment.

Precision in real exchanges

Purpose: Avoid judging memory
Useful wording: I will record what you remember and ask the clinical team to check the difference.

Purpose: Keep attribution
Useful wording: I will record that this information came from you.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which note is accurate?

A. Discontinuation verified from the patient's memory alone.

B. Patient is nonadherent and the outside clinic is wrong.

C. Restart approved because the computer says active.

D. Patient reports outside discontinuation; local list still includes medicine; instruction unavailable; referred to Dana.

2. What does the active local entry establish by itself?

A. The patient must take it now.

B. The outside report is false.

C. Reconciliation is complete.

D. The medicine remains listed locally, not that a new instruction has been verified.

3. A colleague needs the term for this meaning: Direction originating from another healthcare source. Which term fits?

A. duplicate entry

B. allergy history

C. dosage form

D. outside instruction

D Choose the response that fits / continued

4. Rebuild the exchange from "Two sources, one unresolved entry". Ren: "Then please do not remove it and say everything is sorted either. I want the right person to check what the other clinic actually intended." Talia: [missing response] Ren: "I appreciate that. Sometimes questions about medicines sound as though I am being accused of not following instructions, even when I am trying to explain a change." Which response belongs in the gap?

A. The local medication list is a record we need to review. It does not let me give you a new instruction to take, stop, or restart a medicine.

B. An active entry tells us how the item is marked. It does not settle this conflicting information, so we must not treat that label as the answer.

C. Exactly. We need the outside instruction and appropriate review. Nurse Dana can take the discrepancy to the clinician; I cannot verify the change myself from this conversation.

D. I will route that clinical question appropriately. Medication reconciliation and clinical guidance need the proper owner, and recording a history does not authorize me to change treatment.

Answers and explanations, page 119

> **One useful distinction**
>
> An active entry can reflect the state of a record rather than a verified current instruction. Conversely, a patient report deserves documentation but does not automatically complete reconciliation. Preserve both until the authorized review resolves the discrepancy.

E Conversation

Two sources, one unresolved entry

Two sources, one unresolved entry

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

patient report / active entry / discontinuation / adherence / local medication list / Medication reconciliation / discrepancy / verification source / Strength / outside instruction

01 **Ren:** Your list still shows a medicine that another clinic told me to stop. I do not have their instruction with me, but I remember the conversation.

02 **Talia:** Thank you for telling me. I will record your [1] _____ and the difference in our list. I am not going to dismiss it because the computer still shows an entry.

03 **Ren:** Does the computer mean I should be taking it after all? I do not want two clinics giving me opposite directions without explaining why.

04 **Talia:** The [2] _____ is a record we need to review. It does not let me give you a new instruction to take, stop, or restart a medicine.

05 **Ren:** Then please do not remove it and say everything is sorted either. I want the right person to check what the other clinic actually intended.

06 **Talia:** Exactly. We need the [3] _____ and appropriate review. Nurse Dana can take the discrepancy to the clinician; I cannot verify the change myself from this conversation.

07 **Ren:** I appreciate that. Sometimes questions about medicines sound as though I am being accused of not following instructions, even when I am trying to explain a change.

08 **Talia:** I will not label this an [4] _____ problem without evidence. The immediate issue is that your report and our entry differ, and we need to clarify the plan.

09 **Ren:** I do not have the other clinic's instructions with me. Can you help get them checked? I am worried about being given conflicting advice again.

10 **Talia:** Yes. The [5] _____ is still to be established through the review. I will distinguish your report from a verified clinical decision.

Conversation continues on the next page.

Two sources, one unresolved entry

CONVERSATION / CONTINUED

11 **Ren:** The medicine being marked active is what confused me. Is that label always current, or can the record need updating after another clinic makes a change?

12 **Talia:** An [6] _____ tells us how the item is marked. It does not settle this conflicting information, so we must not treat that label as the answer.

13 **Ren:** I also do not have the exact strength or dose in front of me. I would rather say that than give you a number from memory.

14 **Talia:** That is helpful. [7] _____ and dose are different details, and neither should be guessed. We will preserve what is known and identify what requires verification.

15 **Ren:** Can Dana speak with me about it? The list worries me because I still need to know which instruction I should follow.

16 **Talia:** It is a [8] _____ needing the appropriate review. I will pass on both sources and the missing instruction rather than silently choose one over the other.

17 **Ren:** Please make sure any current question about what I should do reaches the clinical team. I do not want the record discussion mistaken for actual advice.

18 **Talia:** I will route that clinical question appropriately. [9] _____ and clinical guidance need the proper owner, and recording a history does not authorize me to change treatment.

19 **Ren:** Then I understand the position: I reported an outside change, your list still includes the medicine, and Dana will review the difference with the clinician.

20 **Talia:** Correct. The reported [10] _____ is not yet verified here. I will keep the source clear, preserve the question, and connect the review with Dana.

CHECK THE GAPS

1. patient report / **2.** local medication list / **3.** outside instruction / **4.** adherence / **5.** verification source / **6.** active entry / **7.** Strength / **8.** discrepancy / **9.** Medication reconciliation / **10.** discontinuation

Why these answers fit, page 120

F Check accuracy and scope

Two sources, one unresolved entry

Return to the case

Patient Ren reports that another clinic stopped one medicine. The local medication list still includes it. Ren does not have the outside instruction available. Assistant Talia cannot verify discontinuation, change the list as a clinical decision, or tell Ren to start, stop, or restart treatment. Nurse Dana can review the discrepancy with the clinician. The immediate communication task is to record the patient report and the conflicting local entry with clear sources, then connect the question with Dana. No medicine name, dose, or timing decision is supplied.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which reply crosses Talia's stated role?

A. I cannot verify the outside instruction here.
B. Restart it until we hear otherwise.
C. Dana can review the discrepancy.
D. I will record the source of your report.

2. Which additional conclusion is unsupported?

A. The sources conflict.
B. The outside instruction is unavailable.
C. The clinician review is still needed.
D. The discrepancy proves an allergy.

Answers and explanations, page 122

Listen for the distinction

Switch roles. Repeat the exchange about the outside instruction without adding a dose or restart recommendation.

A discrepancy is not a credibility contest

Do not make the patient defend their memory against the computer. Acknowledge the report and preserve both sources for review. Respect does not require treating an unverified instruction as confirmed, and careful verification does not require dismissing the patient.

F Say it / transfer to a new case

Report a different medication-list change

Patient Ellis reports an outside dose change. The local record is unchanged, and the outside instruction is unavailable. Nurse Priya accepts the query. No new dose is supplied.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

Priya / unavailable / dose / unchanged

01 **Assistant:** Ellis reports an outside [1] _____ change.

02 **Nurse:** The local record remains [2] _____.

03 **Assistant:** The outside instruction is [3] _____.

04 **Nurse:** The receiving owner is [4] _____.

CHECK THE GAPS

1. dose / **2.** unchanged / **3.** unavailable / **4.** Priya

Why these answers fit, page 122

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Two sources, one unresolved entry". Ren: "I also do not have the exact strength or dose in front of me. I would rather say that than give you a number from memory." Talia: [missing response] Ren: "Can Dana speak with me about it? The list worries me because I still need to know which instruction I should follow." Which response belongs in the gap?

A. Correct. The reported discontinuation is not yet verified here. I will keep the source clear, preserve the question, and connect the review with Dana.

B. That is helpful. Strength and dose are different details, and neither should be guessed. We will preserve what is known and identify what requires verification.

C. Exactly. We need the outside instruction and appropriate review. Nurse Dana can take the discrepancy to the clinician; I cannot verify the change myself from this conversation.

D. It is a discrepancy needing the appropriate review. I will pass on both sources and the missing instruction rather than silently choose one over the other.

Answers and explanations, page 123

A Read the situation

LESSON 04 / LABORATORY PAPERWORK AND SPECIMEN QUERIES

Preserve collection and arrival times accurately when a colleague proposes an unnecessary record change.

Two times, two events

A requisition records specimen collection at 09:20. The courier sheet records arrival at 10:05. Colleague Ben proposes replacing 09:20 with 10:05 so the paperwork looks consistent. Assistant Imani notices that the fields describe different events. No patient-identity mismatch, specimen-quality problem, or test result is stated. The pair must retain the event labels and source records, clarify any genuinely ambiguous field through the laboratory process, and avoid creating a false collection time. The 45-minute interval alone does not establish specimen acceptability.

People: Ben, Clinic colleague; Imani, Medical assistant.

Read the record

Choose one answer per question using the facts above.

1. Which event occurred at 09:20 according to the requisition?

A. Courier arrival
B. Result authorization
C. Patient discharge
D. Collection

2. What does 10:05 describe?

A. A revised collection time
B. A confirmed rejection
C. A reported test result
D. Arrival on the courier sheet

3. What does the interval establish by itself?

A. A 45-minute difference between the recorded events
B. Specimen acceptability for every test
C. A proven transport failure
D. A diagnostic conclusion

Answers and explanations, page 124

B Vocabulary in use

CORE TERMS

requisition
Request document specifying laboratory testing and relevant information.
check the requisition

specimen
Material collected for examination or testing.
identify the specimen

collection time
Time the specimen was obtained from its source.
record the collection time

arrival time
Time an item reached the stated destination.
verify the arrival time

receipt time
Time the receiving service recorded receiving the specimen.
document the receipt time

courier sheet
Transport record containing delivery or movement information.
reconcile the courier sheet

accession number
Identifier assigned by a laboratory to track a case or specimen.
confirm the accession number

specimen label
Information attached to the specimen container for identification.
verify the specimen label

specimen source
Body site or material origin relevant to the test.
record the specimen source

collection method
Technique by which a specimen was obtained.
document the collection method

transport interval
Time between specified transport-related events.
calculate the transport interval

preanalytical phase
Steps before analysis that can affect laboratory testing.
review the preanalytical phase

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

requisition REQUISITION

check the requisition

Keep the meaning precise: Request document specifying laboratory testing and relevant information.

courier sheet COURIER SHEET

reconcile the courier sheet

Keep the meaning precise: Transport record containing delivery or movement information.

B Precision and professional shorthand

EXTEND YOUR RANGE

analytical phase

Stage in which the laboratory examination or measurement is performed.

distinguish the analytical phase

postanalytical phase

Steps after analysis, including review and reporting.

describe the postanalytical phase

acceptance criteria

Requirements for accepting a specimen or process.

check acceptance criteria

rejection criteria

Conditions under which a specimen cannot be accepted for the intended test.

apply rejection criteria

specimen integrity

Condition of the specimen relevant to reliable testing.

assess specimen integrity

hemolysis

Breakdown of red blood cells that can affect some tests.

report hemolysis

insufficient quantity

Amount below what is required for the intended test.

report insufficient quantity

transport medium

Material used to preserve or support a specimen during movement.

verify the transport medium

temperature condition

Required or recorded temperature circumstances.

document temperature conditions

event label

Description of the event to which a time or value belongs.

preserve the event label

record amendment

Traceable correction or addition through the approved record process.

make a record amendment

turnaround time

Interval between defined start and completion events for a service.

define the turnaround time

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

preanalytical phase PREANALYTICAL PHASE

review the preanalytical phase

Keep the meaning precise: Steps before analysis that can affect laboratory testing.

specimen integrity SPECIMEN INTEGRITY

assess specimen integrity

Keep the meaning precise: Condition of the specimen relevant to reliable testing.

C Phrases that move the work forward

Identify the first field
The requisition says collection at 09:20.

Identify the second field
The courier sheet says arrival at 10:05.

Name the distinction
Those times describe different events.

Question the proposed change
Why would an arrival time replace a collection time?

Keep both facts
We should preserve each time with its event label.

Avoid cosmetic correction
Matching numbers would make the record less accurate here.

Read back the sequence
Collected at 09:20; arrival recorded at 10:05.

Clarify an ambiguous field
Let us ask the laboratory which event this field requires.

Do not invent a receipt event
The sheet says arrival; it does not establish every later laboratory step.

Separate calculation and judgment
The interval is 45 minutes, not an acceptability decision.

Name missing criteria
We do not have the test-specific acceptance criteria here.

Keep identifiers separate
No identity discrepancy is established by these different times.

Preserve the source
Retain the original requisition and courier references.

Use a traceable process
Any genuine correction needs the approved amendment process.

Do not infer a result
The timing records do not tell us the test result.

Close accurately
Both entries can remain accurate when their event labels are preserved.

Precision in real exchanges

Purpose: Question the proposed change
Useful wording: Why would an arrival time replace a collection time?

Purpose: Keep both facts
Useful wording: We should preserve each time with its event label.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which combined entry preserves the facts?

A. Collection 10:05; arrival 10:05

B. Collection and arrival both unknown

C. Test authorized at 09:20

D. Collection 09:20; arrival 10:05

2. Why should 09:20 not be replaced merely for consistency?

A. All timestamps must always be identical.

B. Arrival is a more important word than collection.

C. The test is known to have failed.

D. The replacement would change a collection event into an arrival time.

3. A colleague needs the term for this meaning: Time an item reached the stated destination. Which term fits?

A. temperature condition

B. acceptance criteria

C. arrival time

D. specimen source

D Choose the response that fits / continued

4. Rebuild the exchange from "Two times, two events". Ben: "Let me read it back: collected at nine twenty; arrival at ten oh five. The difference is forty-five minutes, but each time has its own meaning." Imani: [missing response] Ben: "I was about to enter ten oh five in a field labeled received. I should check what that field means before assuming the courier entry is enough." Which response belongs in the gap?

A. That preserves the arrival time correctly. We also should not rename arrival as a later laboratory event unless the record actually defines it that way.

B. Yes. Receipt time depends on the receiving record and process. Ask the laboratory which event the destination field requires rather than copying a nearby value.

C. It gives the transport interval between these entries. It does not establish acceptability without the relevant specimen, test, handling conditions, and actual laboratory requirements.

D. First check the event label. The requisition records collection, while the courier sheet records arrival. Those are not two competing values for the same event.

Answers and explanations, page 124

One useful distinction

Collection, courier arrival, laboratory receipt, analysis, and result release are distinct events. Do not assume that arrival on a transport sheet means formal laboratory receipt unless the record defines it that way. Always preserve the stated event label.

E Conversation

Two times, two events

Two times, two events

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

Receipt time / record amendment / Acceptance criteria / arrival time / requisition / transport interval / collection time / Turnaround time / specimen label / event label

01 **Ben:** I have two different times on this paperwork. The requisition says 09:20, but the courier sheet says 10:05. Should I make them match?

02 **Imani:** First check the [1] _____. The requisition records collection, while the courier sheet records arrival. Those are not two competing values for the same event.

03 **Ben:** So replacing the earlier time would not simply clean up the paperwork. It would say the specimen was collected later than the requisition records.

04 **Imani:** Exactly. The [2] _____ is 09:20 in that source. Keeping the record accurate matters more than making two boxes look identical.

05 **Ben:** Let me read it back: collected at nine twenty; arrival at ten oh five. The difference is forty-five minutes, but each time has its own meaning.

06 **Imani:** That preserves the [3] _____ correctly. We also should not rename arrival as a later laboratory event unless the record actually defines it that way.

07 **Ben:** I was about to enter ten oh five in a field labeled received. I should check what that field means before assuming the courier entry is enough.

08 **Imani:** Yes. [4] _____ depends on the receiving record and process. Ask the laboratory which event the destination field requires rather than copying a nearby value.

09 **Ben:** The patient details match in these records. I was only looking at the two times; I do not have evidence of a labeling problem.

10 **Imani:** That would be a separate [5] _____ or identification question. No identity discrepancy is established here, so we should not turn an event-time difference into a patient mismatch.

Conversation continues on the next page.

Two times, two events

CONVERSATION / CONTINUED

11 **Ben:** A colleague called that a quick delivery and marked the specimen acceptable. We have the journey time, but have we checked the requirements for this test?

12 **Imani:** It gives the [6] _____ between these entries. It does not establish acceptability without the relevant specimen, test, handling conditions, and actual laboratory requirements.

13 **Ben:** Then I will avoid writing acceptable or rejected. Neither result follows just from the times we have read on these two documents.

14 **Imani:** Correct. [7] _____ belong to the real laboratory process. A language clarification about timestamps is not a quality decision.

15 **Ben:** If the laboratory later confirms that a field genuinely was entered incorrectly, we can correct it through the proper process while keeping the history clear.

16 **Imani:** That would be a [8] _____. It should preserve what was changed and why under our requirements, rather than silently overwrite the original source.

17 **Ben:** I also should not tell the patient that these times predict when a result will be ready. Collection-to-arrival is not the entire testing process.

18 **Imani:** Exactly. [9] _____ must have defined start and end points. These entries do not supply an analysis completion or result-release time.

19 **Ben:** I will keep both source references, preserve 09:20 as collection and 10:05 as arrival, and clarify the receiving field before entering anything there.

20 **Imani:** Good. The [10] _____ and courier sheet can both be accurate. Our task is to carry their meanings forward without creating a false match.

CHECK THE GAPS

1. event label / **2.** collection time / **3.** arrival time / **4.** Receipt time / **5.** specimen label / **6.** transport interval / **7.** Acceptance criteria / **8.** record amendment / **9.** Turnaround time / **10.** requisition

Why these answers fit, page 125

F Check accuracy and scope

Two times, two events

Return to the case

A requisition records specimen collection at 09:20. The courier sheet records arrival at 10:05. Colleague Ben proposes replacing 09:20 with 10:05 so the paperwork looks consistent. Assistant Imani notices that the fields describe different events. No patient-identity mismatch, specimen-quality problem, or test result is stated. The pair must retain the event labels and source records, clarify any genuinely ambiguous field through the laboratory process, and avoid creating a false collection time. The 45-minute interval alone does not establish specimen acceptability.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which conclusion is unsupported?

A. The records describe two events.
B. The interval is 45 minutes.
C. No result is supplied.
D. The specimen is acceptable because only 45 minutes elapsed.

2. If a destination field is unclear, what is the next step?

A. Clarify the required event with the laboratory through the approved process.
B. Copy whichever time is later.
C. Replace all original values.
D. Invent an accession number.

Answers and explanations, page 127

Listen for the distinction

Switch roles. Repeat the received-field clarification and the distinction between elapsed time and acceptability.

Question the field, not the colleague

A correction request can be challenged without making the exchange personal. Name the field and event that would change, then propose a read-back. Two different values can both be correct when they refer to different events.

F Say it / transfer to a new case

Keep a second pair of events distinct

The collection record says 08:40. The courier sheet says arrival 09:15. The destination field meaning is unclear. No acceptability decision is supplied.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

distinct / 09:15 / clarification / 08:40

01 **Assistant:** Collection was recorded at [1] _____.

02 **Colleague:** Arrival was recorded at [2] _____.

03 **Assistant:** The event labels must remain [3] _____.

04 **Colleague:** The destination field needs [4] _____.

CHECK THE GAPS

1. 08:40 / **2.** 09:15 / **3.** distinct / **4.** clarification

Why these answers fit, page 127

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Two times, two events". Ben: "Then I will avoid writing acceptable or rejected. Neither result follows just from the times we have read on these two documents." Imani: [missing response] Ben: "If the laboratory later confirms that a field genuinely was entered incorrectly, we can correct it through the proper process while keeping the history clear." Which response belongs in the gap?

A. Good. The requisition and courier sheet can both be accurate. Our task is to carry their meanings forward without creating a false match.

B. Correct. Acceptance criteria belong to the real laboratory process. A language clarification about timestamps is not a quality decision.

C. Exactly. Turnaround time must have defined start and end points. These entries do not supply an analysis completion or result-release time.

D. Exactly. The collection time is 09:20 in that source. Keeping the record accurate matters more than making two boxes look identical.

Answers and explanations, page 128

Review 2 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Medication histories with clear sources" and "Laboratory paperwork and specimen queries". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Medicine record held by the current organization. Which term fits?

A. historical entry
B. local medication list
C. dietary supplement
D. strength

2. A colleague needs the term for this meaning: Information attached to the specimen container for identification. Which term fits?

A. insufficient quantity
B. requisition
C. analytical phase
D. specimen label

3. Rebuild the exchange from "Report, observation, assessment". Rosa: "Should I write that Pat denied other symptoms? None are included in the information I am handing over, but I have not established their absence." Dana: [missing response] Rosa: "I will keep the original wording in quotation marks when I use it exactly. If I summarize, I will still identify Pat as the source." Which response belongs in the gap?

A. That clearly identifies what is unassessed. It does not minimize the concern, and it leaves the clinical questions visible for the proper assessment.
B. Exactly. Keep the observation specific. Answering questions while seated does not establish the severity of tiredness or whether a concern can safely wait.
C. No. A pertinent negative must come from the relevant question or assessment. Lack of a statement in this report is not a denial.
D. Right. A baseline is not something we should invent. Your report can remain useful without adding a comparison that has not been obtained.

Review 2 / Meaning and context / continued

4. Rebuild the exchange from "Two times, two events". Ben: "I also should not tell the patient that these times predict when a result will be ready. Collection-to-arrival is not the entire testing process." Imani: [missing response] Ben: "I will keep both source references, preserve 09:20 as collection and 10:05 as arrival, and clarify the receiving field before entering anything there." Which response belongs in the gap?

A. Good. The requisition and courier sheet can both be accurate. Our task is to carry their meanings forward without creating a false match.

B. Exactly. The collection time is 09:20 in that source. Keeping the record accurate matters more than making two boxes look identical.

C. Correct. Acceptance criteria belong to the real laboratory process. A language clarification about timestamps is not a quality decision.

D. Exactly. Turnaround time must have defined start and end points. These entries do not supply an analysis completion or result-release time.

Answers and explanations, page 152

A Read the situation

LESSON 05 / REFERRALS AND APPOINTMENT DEPENDENCIES

Explain referral stages and a named follow-up date without accidentally confirming an appointment.

> **Friday is a status check**
>
> Referral F8 was sent Monday and received by the specialist office Tuesday. The office reports that it is under review; no appointment is booked. Coordinator Lia has agreed to check its status Friday. Patient Morgan believes receipt means an appointment on Friday. Assistant Omar must clarify the actual sequence, distinguish a status check from a booking, and identify Lia as the follow-up owner. No review deadline, appointment date, coverage decision, or reason for the review duration is supplied. Clinical concerns require the clinical route, not a wait for this administrative check.

People: Morgan, Patient; Omar, Medical assistant.

Read the record

Choose one answer per question using the facts above.

1. What happened Tuesday?

A. The patient attended a specialist appointment.
B. Lia completed the Friday check.
C. A coverage decision was issued.
D. The specialist office received referral F8.

2. What is Friday for?

A. Lia's status check
B. A confirmed patient appointment
C. A guaranteed review deadline
D. A promised test result

3. What remains unconfirmed?

A. An appointment date
B. That the referral was sent Monday
C. That receipt occurred Tuesday
D. Lia's ownership of the status check

Answers and explanations, page 129

B Vocabulary in use

CORE TERMS

referral
Request for another service or professional to assess or provide care.

send a referral

receiving service
Organization or team to which a referral is sent.

contact the receiving service

referral identifier
Reference used to track a particular referral.

quote the referral identifier

receipt acknowledgment
Confirmation that the receiving organization obtained the referral.

obtain a receipt acknowledgment

booking confirmation
Verified notice that an appointment has been scheduled.

obtain booking confirmation

referral coordinator
Person organizing referral tracking and related communication.

name the referral coordinator

referring clinician
Professional initiating a referral.

identify the referring clinician

specialist office
Practice or department providing care in a particular specialty.

check with the specialist office

sent status
Record that a referral or message was transmitted.

verify the sent status

under review
Being assessed through a process before a further decision or action.

explain under review

appointment availability
Times or capacity potentially available for booking.

check appointment availability

follow-up checkpoint
Agreed point for checking progress or reporting status.

set a follow-up checkpoint

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

referral REFERRAL

send a referral

Keep the meaning precise: Request for another service or professional to assess or provide care.

sent status SENT STATUS

verify the sent status

Keep the meaning precise: Record that a referral or message was transmitted.

B Precision and professional shorthand

EXTEND YOUR RANGE

dependency

Event or requirement that another step relies on.

identify a booking dependency

supporting documentation

Information accompanying a request to support its assessment.

supply supporting documentation

referral criteria

Requirements used by a service to assess an appropriate referral.

clarify referral criteria

clinical priority

Urgency or relative need determined through clinical assessment.

refer clinical-priority questions

waiting list

Record of people awaiting an available service.

verify waiting-list status

cancellation slot

Appointment time made available when an existing booking is canceled.

check a cancellation slot

appointment offer

Proposed date or time not necessarily accepted or finalized.

confirm an appointment offer

attendance

Actual presence at an appointment or service.

confirm attendance

referral completion

Completion of the intended referral process, with its definition made clear.

document referral completion

consultation report

Account returned after a specialist or other consultation.

obtain the consultation report

care coordination

Organization of care activities and information across people or services.

support care coordination

access barrier

Practical obstacle to obtaining an intended service.

identify an access barrier

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

follow-up checkpoint FOLLOW-UP CHECKPOINT

set a follow-up checkpoint

Keep the meaning precise: Agreed point for checking progress or reporting status.

waiting list WAITING LIST

verify waiting-list status

Keep the meaning precise: Record of people awaiting an available service.

C Phrases that move the work forward

Identify the referral
We are discussing referral F8.

State receipt
The specialist office received it Tuesday.

Correct the appointment inference
No appointment has been booked.

Name the owner
Lia has agreed to check the status.

Keep an offer distinct
An offered time still needs the relevant booking confirmation.

Preserve the next step
Lia will check what stage the referral has reached.

Clarify access needs
We can communicate relevant access needs through the proper process.

Check understanding
The referral is received and under review, with no booked appointment.

State transmission
It was sent Monday.

Explain the stage
They report that it is under review.

Define Friday
Friday is Lia's status check, not your appointment.

Avoid a review guarantee
We do not have a confirmed review-completion date.

Do not invent a cause
I do not have a verified reason for the review duration.

Route clinical concern
A new or urgent concern needs our clinical route.

Avoid a coverage claim
Referral receipt does not itself confirm insurance coverage.

Close precisely
The confirmed next action is Lia's Friday status check.

Precision in real exchanges

Purpose: Explain the stage
Useful wording: They report that it is under review.

Purpose: Correct the appointment inference
Useful wording: No appointment has been booked.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which update accurately uses all three days?

A. Review must finish Friday because Lia will call.

B. Sent Monday, received Tuesday, and Lia checks status Friday; no appointment is booked.

C. Booked Monday, attended Tuesday, and results Friday.

D. Received Tuesday, so Friday attendance is confirmed.

2. Which sentence creates a false expectation?

A. Lia will check the status Friday.

B. The referral is under review.

C. No appointment is booked.

D. We will see you at the specialist on Friday.

3. A colleague needs the term for this meaning: Practice or department providing care in a particular specialty. Which term fits?

A. referral criteria

B. referral completion

C. specialist office

D. booking confirmation

D Choose the response that fits / continued

4. Rebuild the exchange from "Friday is a status check". Morgan: "Let me get this straight. It went out Monday and reached them Tuesday. What exactly is Lia doing on Friday if I am not booked in?" Omar: [missing response] Morgan: "Does under review mean the referral has been rejected, or that the office is still deciding what happens next?" Which response belongs in the gap?

A. Correct. A booking confirmation is different from the status check. At present, no appointment date or time is confirmed.

B. Under review is the status they gave us. We should not turn it into either a rejection or an appointment confirmation without further information.

C. The referral identifier is F8. It was sent Monday, received Tuesday, and Lia has agreed to check the status Friday.

D. Let me clarify the follow-up checkpoint. Friday is when Lia will check its status. No specialist appointment has been booked.

Answers and explanations, page 129

One useful distinction

Sent, received, reviewed, offered, booked, attended, and reported are different stages. Even referral complete can have different system meanings. State the actual event instead of assuming a status label proves that the patient has been seen.

E Conversation

Friday is a status check

Friday is a status check

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

follow-up checkpoint / referral coordinator / clinical priority / referral identifier / Under review / receipt acknowledgment / booking confirmation / appointment offer / care coordination / receiving service

01 **Morgan:** I heard the specialist received my referral on Tuesday, and someone mentioned Friday. What time should I arrive for the appointment?

02 **Omar:** Let me clarify the [1] _____. Friday is when Lia will check its status. No specialist appointment has been booked.

03 **Morgan:** I thought received meant they had accepted me into a slot. Is there still another step before I can arrange time away from work?

04 **Omar:** The [2] _____ means the office received the referral. They say it is under review, and we do not yet have a date for you to attend.

05 **Morgan:** Let me get this straight. It went out Monday and reached them Tuesday. What exactly is Lia doing on Friday if I am not booked in?

06 **Omar:** The [3] _____ is F8. It was sent Monday, received Tuesday, and Lia has agreed to check the status Friday.

07 **Morgan:** Does under review mean the referral has been rejected, or that the office is still deciding what happens next?

08 **Omar:** [4] _____ is the status they gave us. We should not turn it into either a rejection or an appointment confirmation without further information.

09 **Morgan:** Who is Lia in this process? I need to know whether she is the person checking the status or the clinician deciding about my care.

10 **Omar:** Lia is the [5] _____ handling this follow-up. Her Friday check does not control the specialist's decision or guarantee that their review will finish that day.

Conversation continues on the next page.

Friday is a status check

CONVERSATION / CONTINUED

11 **Morgan:** I nearly asked for Friday off work. Please make the next message clear about whether it is an update or an actual appointment offer.

12 **Omar:** Correct. A [6] _____ is different from the status check. At present, no appointment date or time is confirmed.

13 **Morgan:** If they offer a time later, I may need to explain my work schedule and transport arrangements. I cannot assume every offered slot is possible.

14 **Omar:** An [7] _____ should be handled through the booking process. Relevant access needs can be communicated appropriately rather than assuming an offered time has already been accepted.

15 **Morgan:** I also wondered whether the referral being received means my plan has approved the visit. I do not want to confuse the two processes.

16 **Omar:** The [8] _____ receiving a referral does not itself confirm coverage. Any coverage question needs the appropriate verification, separate from the current referral status.

17 **Morgan:** What if the concern I was referred for changes before Friday? Should I simply wait because that is when Lia has promised to check?

18 **Omar:** No administrative checkpoint establishes that waiting is safe. A change or urgent concern needs our clinical route for [9] _____, not an assumption based on Lia's diary.

19 **Morgan:** Understood. The referral reached them Tuesday and remains under review. Lia will check Friday, but I do not have an appointment yet.

20 **Omar:** Exactly. That is the current [10] _____ position. I will keep the status and ownership clear, and any actual booking information must be confirmed separately.

CHECK THE GAPS

1. follow-up checkpoint / **2.** receipt acknowledgment / **3.** referral identifier / **4.** Under review / **5.** referral coordinator / **6.** booking confirmation / **7.** appointment offer / **8.** receiving service / **9.** clinical priority / **10.** care coordination

Why these answers fit, page 130

F Check accuracy and scope

Friday is a status check

Return to the case

Referral F8 was sent Monday and received by the specialist office Tuesday. The office reports that it is under review; no appointment is booked. Coordinator Lia has agreed to check its status Friday. Patient Morgan believes receipt means an appointment on Friday. Assistant Omar must clarify the actual sequence, distinguish a status check from a booking, and identify Lia as the follow-up owner. No review deadline, appointment date, coverage decision, or reason for the review duration is supplied. Clinical concerns require the clinical route, not a wait for this administrative check.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What does receipt establish?

A. Insurance coverage is guaranteed.
B. The patient attended.
C. The referral reached the receiving office, not that a slot is booked.
D. The specialist completed an assessment.

2. How should a new clinical concern be handled?

A. By treating the referral identifier as an urgency score
B. Through the clinic's appropriate clinical route rather than waiting for Lia's routine check
C. By assuming Friday is always safe
D. By inventing a specialist assessment over reception

Answers and explanations, page 132

Listen for the distinction

Switch roles. Repeat the work-schedule question and explain the actual referral stage without inventing a booking.

Explain the stage in ordinary language

A word such as received may sound final to someone outside the clinic. Give the plain-language meaning and the next known action. Repeating under review without explanation can sound evasive; inventing a likely appointment can be misleading.

F Say it / transfer to a new case

Separate another checkpoint from booking

Referral G3 was received Wednesday and remains under review. No appointment is booked. Coordinator Noor will check Monday.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

G3 / under review / Wednesday / Monday

01 **Assistant:** The referral identifier is [1] _____.

02 **Patient:** Receipt was confirmed [2] _____.

03 **Assistant:** The status remains [3] _____.

04 **Patient:** Noor will check on [4] _____.

CHECK THE GAPS

1. G3 / **2.** Wednesday / **3.** under review / **4.** Monday

Why these answers fit, page 132

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Friday is a status check". Morgan: "If they offer a time later, I may need to explain my work schedule and transport arrangements. I cannot assume every offered slot is possible." Omar: [missing response] Morgan: "I also wondered whether the referral being received means my plan has approved the visit. I do not want to confuse the two processes." Which response belongs in the gap?

A. The receipt acknowledgment means the office received the referral. They say it is under review, and we do not yet have a date for you to attend.

B. No administrative checkpoint establishes that waiting is safe. A change or urgent concern needs our clinical route for clinical priority, not an assumption based on Lia's diary.

C. An appointment offer should be handled through the booking process. Relevant access needs can be communicated appropriately rather than assuming an offered time has already been accepted.

D. Lia is the referral coordinator handling this follow-up. Her Friday check does not control the specialist's decision or guarantee that their review will finish that day.

Answers and explanations, page 133

A Read the situation

LESSON 06 / SUPPORTING QUESTIONS DURING A CLINICAL VISIT

Support a patient request for a private explanation while preserving the original clinical instruction.

> **Clarify before leaving**
>
> Patient Jules has not yet left the clinic and does not understand a phrase in the after-visit summary. Jules wants an explanation away from reception before leaving. Dr Chen is available. Assistant Mina cannot reinterpret or change the clinical instruction. No exact instruction, treatment detail, or revised plan is supplied. Mina must identify the document and unclear phrase, pass the question to Dr Chen, and arrange the appropriate private conversation. A patient saying yes or receiving the document does not by itself establish understanding.

People: Jules, Patient; Mina, Medical assistant.

Read the record

Choose one answer per question using the facts above.

1. Which document contains the unclear phrase?

A. A new prescription invented by Mina
B. The after-visit summary
C. A supplied insurance appeal
D. A courier manifest

2. Who is available to explain the clinical instruction?

A. Another patient in reception
B. An automated guess
C. Dr Chen
D. Mina acting outside the stated role

3. What does Jules request?

A. A silent change to the instruction
B. A public discussion across the waiting area
C. A claim that understanding is already confirmed
D. An explanation before leaving, away from reception

Answers and explanations, page 134

B Vocabulary in use

CORE TERMS

after-visit summary
Document summarizing relevant information and instructions after a clinical encounter.

review the after-visit summary

clinical instruction
Direction concerning care or treatment from an authorized professional.

clarify a clinical instruction

plain language
Clear wording suited to the intended audience.

explain in plain language

technical term
Specialized word used within a professional field.

explain a technical term

jargon
Specialized expressions that may be unfamiliar outside a group.

clarify unfamiliar jargon

abbreviation
Shortened form of a word or phrase.

expand an abbreviation

ambiguity
Wording that permits more than one interpretation.

resolve an ambiguity

clarification
Explanation that makes meaning more definite or understandable.

request clarification

reinterpretation
Assigning a meaning that changes or newly determines the original instruction.

avoid unauthorized reinterpretation

teach-back
Checking an explanation by inviting the person to express their understanding.

use teach-back appropriately

show-me method
Checking understanding by asking the person to demonstrate an instructed action.

use the show-me method

chunk and check
Giving information in short portions and checking understanding between them.

use chunk and check

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

after-visit summary AFTER-VISIT SUMMARY

review the after-visit summary

Keep the meaning precise: Document summarizing relevant information and instructions after a clinical encounter.

abbreviation ABBREVIATION

expand an abbreviation

Keep the meaning precise: Shortened form of a word or phrase.

B Precision and professional shorthand

EXTEND YOUR RANGE

health literacy

Ability to find, understand, and use health information and services.

support health literacy

qualified interpreter

Person with the relevant skills and qualifications to interpret accurately in context.

request a qualified interpreter

interpretation

Rendering spoken or signed communication into another language, or explaining meaning in another context.

clarify the required interpretation

private discussion

Conversation conducted with appropriate protection from unnecessary hearing.

arrange a private discussion

understanding check

Method used to establish whether an explanation was understood.

conduct an understanding check

language access

Appropriate support enabling communication across language differences.

arrange language access

translation

Rendering written information into another language.

obtain an appropriate translation

accessible format

Presentation adapted to a person's communication or access needs.

provide an accessible format

patient question

Specific issue the person wants clarified.

preserve the patient question

instruction source

Document or professional from which a direction came.

identify the instruction source

B Precision and professional shorthand / continued

revised plan

Updated clinical plan established by the appropriate professional.

confirm a revised plan

question handoff

Transfer of a specific question to the person who can address it.

complete a question handoff

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

chunk and check CHUNK AND CHECK

use chunk and check

Keep the meaning precise: Giving information in short portions and checking understanding between them.

interpretation INTERPRETATION

clarify the required interpretation

Keep the meaning precise: Rendering spoken or signed communication into another language, or explaining meaning in another context.

C Phrases that move the work forward

Welcome the question
Thank you for telling me which part is unclear.

Preserve the wording
I will keep the original wording with your question.

Name the available clinician
Dr Chen is available to explain it.

Respect privacy
We can arrange the appropriate conversation away from reception.

Make the handoff specific
Dr Chen, Jules has a question about this phrase in the summary.

Invite a clearer explanation
Please ask Dr Chen to explain the phrase in language that works for you.

Distinguish support needs
An interpreter request and a clinical meaning question may require different support.

Keep changes attributed
Any revised clinical plan must come from the authorized clinician.

Identify the document
The phrase is in your after-visit summary.

State the boundary
I cannot reinterpret this clinical instruction.

Respect the timing
You would like clarification before you leave.

Avoid a guessed meaning
I do not want to give you an explanation that changes the instruction.

Do not imply understanding
Receiving the summary does not mean every instruction is clear.

Keep the question intact
I will not reduce your concern to a generic request for information.

Avoid shaming
Asking again is a reasonable part of understanding the plan.

Close the route
I will connect the question with Dr Chen before you leave.

Precision in real exchanges

Purpose: State the boundary
Useful wording: I cannot reinterpret this clinical instruction.

Purpose: Name the available clinician
Useful wording: Dr Chen is available to explain it.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which response best preserves Jules's request?

A. Dr Chen is available; I will pass on this phrase and arrange the appropriate private explanation before you leave.

B. I will guess what the instruction probably means.

C. You received the paper, so clarification is complete.

D. Ask another patient what they did.

2. What does handing over the summary establish?

A. Teach-back was completed.

B. A revised plan was approved.

C. Every abbreviation was explained.

D. The document was provided, not that the patient understands every instruction.

3. A colleague needs the term for this meaning: Specialized word used within a professional field. Which term fits?

A. technical term

B. qualified interpreter

C. reinterpretation

D. understanding check

D Choose the response that fits / continued

4. Rebuild the exchange from "Clarify before leaving". Jules: "I would prefer not to discuss the personal details across reception. There are several people waiting, and this is not something I want them to hear." Mina: [missing response] Jules: "Thank you. I sometimes nod because I recognize a term, then realize I do not understand how it applies to the instruction." Which response belongs in the gap?

A. Thank you for raising it. This is your after-visit summary. Please indicate the phrase, and I will pass the actual wording and your question to Dr Chen.

B. I will preserve the patient question with the wording. That gives Dr Chen the actual issue rather than a vague message that could miss it.

C. Recognizing a technical term does not always make the instruction clear. Asking again is reasonable; receiving the paper does not mean every question has been answered.

D. We can arrange an appropriate private discussion. I will tell Dr Chen that you want clarification before leaving and that the setting matters to you.

Answers and explanations, page 134

> **One useful distinction**
>
> Clarifying where a phrase appears is different from deciding what the clinical instruction means. Mina can preserve the question and connect Jules with Dr Chen. This does not authorize a paraphrase that changes treatment, timing, or conditions.

E Conversation

Clarify before leaving

Clarify before leaving

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

private discussion / question handoff / technical term / teach-back / revised plan / instruction source / patient question / clinical instruction / plain language / after-visit summary

01 **Jules:** Mina, I have not left yet because one phrase on this paper is unclear. I can read the words, but I do not know what I am meant to do.

02 **Mina:** Thank you for raising it. This is your [1] _____. Please indicate the phrase, and I will pass the actual wording and your question to Dr Chen.

03 **Jules:** Could you just explain what it means? I know everyone is busy, but I do not want to go home and make the wrong assumption.

04 **Mina:** I cannot reinterpret the [2] _____. Dr Chen is available, so I can connect you with the clinician who can explain it properly.

05 **Jules:** I would prefer not to discuss the personal details across reception. There are several people waiting, and this is not something I want them to hear.

06 **Mina:** We can arrange an appropriate [3] _____. I will tell Dr Chen that you want clarification before leaving and that the setting matters to you.

07 **Jules:** Thank you. I sometimes nod because I recognize a term, then realize I do not understand how it applies to the instruction.

08 **Mina:** Recognizing a [4] _____ does not always make the instruction clear. Asking again is reasonable; receiving the paper does not mean every question has been answered.

09 **Jules:** Could you show Dr Chen this line on the summary? It is this phrase, not the whole page, that I cannot make sense of.

10 **Mina:** I will preserve the [5] _____ with the wording. That gives Dr Chen the actual issue rather than a vague message that could miss it.

Conversation continues on the next page.

Clarify before leaving

CONVERSATION / CONTINUED

11 **Jules:** I would like a plain explanation, not just the same phrase repeated more loudly. I heard it; the meaning is what I need.

12 **Mina:** You can request [6] _____ from Dr Chen. I will not substitute my own clinical interpretation while arranging that explanation.

13 **Jules:** After Dr Chen explains it, can I say back what I would actually do? I sometimes say yes too quickly when someone asks whether I understand.

14 **Mina:** Yes. That kind of [7] _____ can help the clinician identify what still needs clarification. It is not a test of your intelligence or a reason to feel embarrassed.

15 **Jules:** I also want the original wording available during the conversation. Otherwise someone might explain a shortened version and miss the part that confused me.

16 **Mina:** Keeping the [8] _____ with the question will help. We should not edit the direction simply to make the handoff shorter.

17 **Jules:** So there is no new instruction from this conversation yet. The next step is for Dr Chen to clarify the existing phrase privately before I leave.

18 **Mina:** Correct. No [9] _____ has been established here. I am arranging the explanation, not changing the clinical direction.

19 **Jules:** Please tell Dr Chen that I am still here and that I want to understand the instruction before going. I appreciate having a clear next step.

20 **Mina:** I will make that [10] _____ now: the phrase in your summary, your request for privacy, and clarification before leaving. Dr Chen is the available clinician for that conversation.

CHECK THE GAPS

1. after-visit summary / **2.** clinical instruction / **3.** private discussion / **4.** technical term / **5.** patient question / **6.** plain language / **7.** teach-back / **8.** instruction source / **9.** revised plan / **10.** question handoff

Why these answers fit, page 135

F Check accuracy and scope

Clarify before leaving

Return to the case

Patient Jules has not yet left the clinic and does not understand a phrase in the after-visit summary. Jules wants an explanation away from reception before leaving. Dr Chen is available. Assistant Mina cannot reinterpret or change the clinical instruction. No exact instruction, treatment detail, or revised plan is supplied. Mina must identify the document and unclear phrase, pass the question to Dr Chen, and arrange the appropriate private conversation. A patient saying yes or receiving the document does not by itself establish understanding.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which statement misuses teach-back?

A. It may require another explanation.
B. It is a test to prove the patient is intelligent enough.
C. It helps check whether an explanation was clear.
D. It can reveal a misunderstanding.

2. What must remain unchanged in Mina's handoff?

A. A new dose invented for simplicity
B. An unsupported promise of treatment success
C. The original instruction and the substance of the patient's question
D. A guessed clinical meaning supplied by Mina

Answers and explanations, page 137

Listen for the distinction

Switch roles. Repeat the request to explain understanding in the patient's own words without inventing the clinical instruction.

Make a question welcome

Patients may hesitate to ask again when staff appear busy. Thank the person for identifying the unclear wording, and avoid treating clarification as failure. A request for privacy can be handled without announcing the personal question across reception.

F Say it / transfer to a new case

Route an unclear abbreviation

Patient Ari asks about an abbreviation in the visit summary before leaving. Assistant Lee cannot interpret the clinical instruction. Dr Malik is available for a private explanation.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

abbreviation / Malik / private / leaving

01 **Lee:** The unclear item is an [1] _____.

02 **Ari:** I want clarification before [2] _____.

03 **Lee:** Dr [3] _____ is available.

04 **Ari:** I would like the explanation to be [4] _____.

CHECK THE GAPS

1. abbreviation / **2.** leaving / **3.** Malik / **4.** private

Why these answers fit, page 137

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Clarify before leaving". Jules: "After Dr Chen explains it, can I say back what I would actually do? I sometimes say yes too quickly when someone asks whether I understand." Mina: [missing response] Jules: "I also want the original wording available during the conversation. Otherwise someone might explain a shortened version and miss the part that confused me." Which response belongs in the gap?

A. Yes. That kind of teach-back can help the clinician identify what still needs clarification. It is not a test of your intelligence or a reason to feel embarrassed.

B. Recognizing a technical term does not always make the instruction clear. Asking again is reasonable; receiving the paper does not mean every question has been answered.

C. Thank you for raising it. This is your after-visit summary. Please indicate the phrase, and I will pass the actual wording and your question to Dr Chen.

D. I will make that question handoff now: the phrase in your summary, your request for privacy, and clarification before leaving. Dr Chen is the available clinician for that conversation.

Answers and explanations, page 138

Review 3 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Referrals and appointment dependencies" and "Supporting questions during a clinical visit". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Organization or team to which a referral is sent. Which term fits?

A. supporting documentation
B. attendance
C. under review
D. receiving service

2. A colleague needs the term for this meaning: Explanation that makes meaning more definite or understandable. Which term fits?

A. health literacy
B. clarification
C. after-visit summary
D. private discussion

3. Rebuild the exchange from "Two times, two events". Ben: "The patient details match in these records. I was only looking at the two times; I do not have evidence of a labeling problem." Imani: [missing response] Ben: "A colleague called that a quick delivery and marked the specimen acceptable. We have the journey time, but have we checked the requirements for this test?" Which response belongs in the gap?

A. Yes. Receipt time depends on the receiving record and process. Ask the laboratory which event the destination field requires rather than copying a nearby value.
B. First check the event label. The requisition records collection, while the courier sheet records arrival. Those are not two competing values for the same event.
C. That preserves the arrival time correctly. We also should not rename arrival as a later laboratory event unless the record actually defines it that way.
D. That would be a separate specimen label or identification question. No identity discrepancy is established here, so we should not turn an event-time difference into a patient mismatch.

Review 3 / Meaning and context / continued

4. Rebuild the exchange from "Clarify before leaving". Jules: "So there is no new instruction from this conversation yet. The next step is for Dr Chen to clarify the existing phrase privately before I leave." Mina: [missing response] Jules: "Please tell Dr Chen that I am still here and that I want to understand the instruction before going. I appreciate having a clear next step." Which response belongs in the gap?

A. Correct. No revised plan has been established here. I am arranging the explanation, not changing the clinical direction.

B. I cannot reinterpret the clinical instruction. Dr Chen is available, so I can connect you with the clinician who can explain it properly.

C. Keeping the instruction source with the question will help. We should not edit the direction simply to make the handoff shorter.

D. You can request plain language from Dr Chen. I will not substitute my own clinical interpretation while arranging that explanation.

Answers and explanations, page 155

A Read the situation

LESSON 07 / TELEPHONE CONCERNS AND CLINICAL ESCALATION

Confirm a received clinical handoff and record its actual outcome without inventing an assessment or delaying care.

> **After the clinical connection**
>
> Caller Ellis asked assistant Noor whether a new concern could wait until next week's routine appointment. Noor could not assess urgency and immediately connected Ellis with nurse Dana through the clinic process. Dana received the call. This later conversation reviews the handoff record; it does not delay the live clinical call. No symptom details, clinical assessment, or waiting advice are supplied. Noor must record the actual recipient and outcome without inventing a clinical conclusion. Applicable emergency procedures take priority over routine call handling or documentation.

People: Noor, Medical assistant; Dana, Receiving nurse.

Read the record

Choose one answer per question using the facts above.

1. What is Ellis asking Noor to decide?

A. A completed medication review
B. Whether a new concern can safely wait until next week's visit
C. The spelling of a street name
D. A confirmed test result

2. Who is available now?

A. Nurse Dana through the clinical-call route
B. An unspecified clinician next month
C. A specialist appointment already booked
D. No one in the clinic

3. What does the lack of supplied symptom details establish?

A. The concern is harmless.
B. The caller has no symptoms.
C. A routine visit is definitely enough.
D. The exercise has not established urgency or safety to wait.

Answers and explanations, page 139

B Vocabulary in use

CORE TERMS

clinical-call route
Established pathway for calls requiring clinical review.

use the clinical-call route

triage
Clinical assessment that determines urgency and appropriate next care.

refer for triage

urgency assessment
Evaluation of how promptly a concern requires attention.

obtain an urgency assessment

routine appointment
Scheduled visit not itself proof that a new concern can wait.

distinguish a routine appointment

new concern
Newly raised issue requiring appropriate handling.

report a new concern

clinical escalation
Moving an issue to the appropriate level of clinical attention.

initiate clinical escalation

warm transfer
Call transfer with an introduction or relevant handoff to the receiving person.

arrange a warm transfer

blind transfer
Transfer without a direct introduction or confirmed contextual handoff.

avoid an unsupported blind transfer

receiving clinician
Clinical professional accepting the concern or handoff.

identify the receiving clinician

callback queue
List of requests awaiting a return call.

check the callback queue

callback commitment
Agreed promise about a return contact, with scope made clear.

confirm a callback commitment

call disposition
Recorded outcome or next handling status of a call.

document the call disposition

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

clinical-call route CLINICAL-CALL ROUTE

use the clinical-call route

Keep the meaning precise: Established pathway for calls requiring clinical review.

clinical escalation CLINICAL ESCALATION

initiate clinical escalation

Keep the meaning precise: Moving an issue to the appropriate level of clinical attention.

B Precision and professional shorthand

EXTEND YOUR RANGE

dropped call

Telephone connection that ends unexpectedly.

follow the dropped-call process

identity check

Process of confirming who is communicating.

complete the identity check

red flag

Finding or feature that may signal a serious concern requiring appropriate action.

escalate a reported red flag

symptom onset

Time a reported symptom began.

relay symptom onset

reassurance

Statement intended to reduce concern, which must not exceed evidence or authority.

avoid unsupported reassurance

SBAR

Situation, background, assessment, recommendation: a structured communication framework.

use SBAR within the actual role

verified callback number

Return-contact number checked through the appropriate procedure.

obtain a verified callback number

emergency pathway

Established route for immediate emergency response.

follow the emergency pathway

safety-net advice

Professional guidance about changes or circumstances requiring further help.

provide authorized safety-net advice

symptom progression

How a symptom has changed over time.

describe symptom progression

closed-loop communication

Exchange that confirms receipt and intended action.

use closed-loop communication

escalation outcome

Result of routing a concern, including whether it was received.

record the escalation outcome

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

call disposition CALL DISPOSITION

document the call disposition

Keep the meaning precise: Recorded outcome or next handling status of a call.

red flag RED FLAG

escalate a reported red flag

Keep the meaning precise: Finding or feature that may signal a serious concern requiring appropriate action.

C Phrases that move the work forward

Acknowledge the question

You want to know whether this can wait until next week.

Offer the immediate route

Nurse Dana is available through our clinical-call route now.

Separate the booking

The routine appointment does not answer the new clinical question.

Use the actual pathway

We need to follow the clinic's clinical-call process.

Protect continuity

We should use the verified contact process if the connection fails.

Keep facts attributed

I will relay what you report without adding a diagnosis.

Keep emergency priority

Applicable emergency procedures take precedence over routine call handling.

Record the outcome

The note must show whether the clinical transfer succeeded.

State the limit briefly

I cannot assess urgency or tell you it is safe to wait.

Avoid unsupported reassurance

I do not have a clinical assessment that would support that reassurance.

Preserve the request

I will pass on your question about how soon you need care.

Confirm the recipient

I will identify the person receiving the concern.

Do not claim connection early

A message in a queue is not the same as a completed transfer.

Avoid guessed advice

I will not invent treatment or safety-net instructions.

Check receipt

Please confirm that the receiving clinician has the concern.

Close the handoff

The timing question belongs with the available clinical team now.

Precision in real exchanges

Purpose: Avoid unsupported reassurance
Useful wording: I do not have a clinical assessment that would support that reassurance.

Purpose: Separate the booking
Useful wording: The routine appointment does not answer the new clinical question.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which response addresses the caller's need appropriately?

A. If you sound calm, next week is fine.

B. A routine appointment always means the concern can wait.

C. No details were supplied, so nothing is wrong.

D. I cannot assess whether waiting is safe; Dana is available now through our clinical-call route.

2. What does an unanswered transfer establish?

A. The issue is resolved.

B. The clinical connection has not been completed.

C. Dana received and assessed the concern.

D. The caller was reassured by a clinician.

3. A colleague needs the term for this meaning: Scheduled visit not itself proof that a new concern can wait. Which term fits?

A. identity check

B. reassurance

C. routine appointment

D. receiving clinician

D Choose the response that fits / continued

4. Rebuild the exchange from "After the clinical connection". Noor: "Ellis already had a booking next week. I included that as background, but I did not say the booking established a safe waiting period." Dana: [missing response] Noor: "My note currently says passed to nursing. That leaves out who received the call. I should name you and record the actual connection." Which response belongs in the gap?

A. That keeps the escalation outcome accurate. The call reached me; your note should say that clearly while leaving the clinical findings to the clinical record.

B. Exactly. Follow the clinic's dropped call process. Verified contact details and the actual unresolved concern matter; do not silently leave a failed connection marked complete.

C. Correct. A routine appointment does not assess a new concern. Do not let the appointment date become the only information the next person receives.

D. I received that question. Your note should not contain an urgency assessment from you. Receiving the call and assessing the concern are different events.

Answers and explanations, page 139

> **One useful distinction**
>
> A routine appointment is an administrative booking, not a clinical assessment of a new concern. Calm speech, a familiar patient, or an empty symptom field cannot establish that waiting is safe. Follow the actual clinical and emergency pathways.

E Conversation

After the clinical connection

After the clinical connection

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

warm transfer / dropped call / clinical-call route / receiving clinician / escalation outcome / routine appointment / emergency pathway / callback queue / urgency assessment / triage

01 **Noor:** Dana, thanks for taking Ellis's call immediately. I am checking my handoff note now, after the connection. Ellis asked whether a new concern could wait until next week.

02 **Dana:** I received that question. Your note should not contain an [1] _____ from you. Receiving the call and assessing the concern are different events.

03 **Noor:** I told Ellis I could not decide that and connected the call through our process. I did not ask Ellis to finish a long administrative discussion first.

04 **Dana:** Good. The request needed [2] _____, not a yes-or-no answer from reception. Keep the actual clinical response in the appropriate clinical record.

05 **Noor:** Ellis already had a booking next week. I included that as background, but I did not say the booking established a safe waiting period.

06 **Dana:** Correct. A [3] _____ does not assess a new concern. Do not let the appointment date become the only information the next person receives.

07 **Noor:** My note currently says passed to nursing. That leaves out who received the call. I should name you and record the actual connection.

08 **Dana:** Yes. The [4] _____ worked in this case. Record that factual outcome, not simply an attempt to put somebody through.

09 **Noor:** I gave you Ellis's question without describing Ellis as anxious or suggesting the concern was mild. Neither would have been a finding I had established.

10 **Dana:** As the [5] _____, I need the actual report. Your introduction helped me understand why Ellis was calling rather than making Ellis start with the appointment history again.

Conversation continues on the next page.

After the clinical connection

CONVERSATION / CONTINUED

11 **Noor:** I stayed for the introduction and confirmed the connection before leaving the call. I will describe what happened rather than just checking a transfer box.

12 **Dana:** That is the relevant [6] _____ detail. Keep the actual handoff time in the record; do not substitute the later time at which you finish writing.

13 **Noor:** For a future call, if the line disconnects before anyone confirms receipt, I cannot use today's completed status as the default.

14 **Dana:** Exactly. Follow the clinic's [7] _____ process. Verified contact details and the actual unresolved concern matter; do not silently leave a failed connection marked complete.

15 **Noor:** And if I only put a request into a message queue, the note should still show pending contact, not that the patient spoke to a nurse.

16 **Dana:** Right. A [8] _____ is not a clinical conversation. Use the appropriate follow-up and escalation process for the real situation, rather than assume the queue resolves urgency.

17 **Noor:** This record review happened after the immediate connection. We should never make an active caller wait while we discuss all these documentation examples.

18 **Dana:** Agreed. The actual [9] _____ takes precedence when needed. Do not use this case to decide that another caller is nonurgent.

19 **Noor:** I will record your confirmed receipt and my factual introduction. I will not add a diagnosis, waiting advice, or clinical outcome that you have not provided.

20 **Dana:** That keeps the [10] _____ accurate. The call reached me; your note should say that clearly while leaving the clinical findings to the clinical record.

CHECK THE GAPS

1. urgency assessment / **2.** triage / **3.** routine appointment / **4.** clinical-call route / **5.** receiving clinician / **6.** warm transfer / **7.** dropped call / **8.** callback queue / **9.** emergency pathway / **10.** escalation outcome

Why these answers fit, page 140

F Check accuracy and scope

After the clinical connection

Return to the case

Caller Ellis asked assistant Noor whether a new concern could wait until next week's routine appointment. Noor could not assess urgency and immediately connected Ellis with nurse Dana through the clinic process. Dana received the call. This later conversation reviews the handoff record; it does not delay the live clinical call. No symptom details, clinical assessment, or waiting advice are supplied. Noor must record the actual recipient and outcome without inventing a clinical conclusion. Applicable emergency procedures take priority over routine call handling or documentation.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. Which information must not be invented for a structured handoff?

A. A diagnosis or urgency decision outside Noor's role
B. The caller's actual question
C. The receiving person's name when confirmed
D. The actual transfer outcome

2. What takes precedence when applicable?

A. Completing an English practice dialogue
B. Obtaining a routine administrative update first
C. The actual emergency pathway
D. Waiting for next week's booking regardless of circumstances

Answers and explanations, page 142

Listen for the distinction

Switch roles. Repeat the distinction between a connected call and a queued message without adding a clinical result.

A calm caller can still need assessment

Tone of voice is not a reliable substitute for clinical information. Be respectful and direct when someone asks for reassurance you cannot provide. Name the available clinical route immediately, rather than giving a long explanation of your limitations.

F Say it / transfer to a new case

Preserve an interrupted clinical transfer

A clinical transfer for caller Ari disconnects before receipt is confirmed. Assistant Lee must use the clinic dropped-call process. Nurse Priya is the intended clinical recipient. No assessment has occurred.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

assessment / Priya / confirmed / dropped-call

01 **Lee:** Receipt has not been [1] _____.

02 **Colleague:** Use the clinic's [2] _____ process.

03 **Lee:** The intended recipient is nurse [3] _____.

04 **Colleague:** No clinical [4] _____ has occurred.

CHECK THE GAPS

1. confirmed / **2.** dropped-call / **3.** Priya / **4.** assessment

Why these answers fit, page 142

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "After the clinical connection". Noor: "For a future call, if the line disconnects before anyone confirms receipt, I cannot use today's completed status as the default." Dana: [missing response] Noor: "And if I only put a request into a message queue, the note should still show pending contact, not that the patient spoke to a nurse." Which response belongs in the gap?

A. I received that question. Your note should not contain an urgency assessment from you. Receiving the call and assessing the concern are different events.

B. Exactly. Follow the clinic's dropped call process. Verified contact details and the actual unresolved concern matter; do not silently leave a failed connection marked complete.

C. Correct. A routine appointment does not assess a new concern. Do not let the appointment date become the only information the next person receives.

D. That keeps the escalation outcome accurate. The call reached me; your note should say that clearly while leaving the clinical findings to the clinical record.

Answers and explanations, page 143

A Read the situation

LESSON 08 / RECORDS, CORRECTIONS, AND FOLLOW-UP OWNERSHIP

Correct an appointment message with a clear apology, verified booking details, and an accurate contact record.

> **Thursday, not Tuesday**
>
> Assistant Elena sent patient Robin a message incorrectly saying that a results discussion was booked Tuesday. The verified schedule shows Thursday at 14:00 with Dr Chen. Robin rearranged work after the incorrect message. Elena can correct the communication and confirm the actual booking, but no test results or clinical interpretation are supplied. The appointment itself has not been changed from Tuesday to Thursday; the earlier message was wrong. Elena must preserve that distinction, acknowledge the disruption, and document whether the correction actually reached Robin.

People: Robin, Patient; Elena, Medical assistant.

Read the record

Choose one answer per question using the facts above.

1. What does the verified schedule show?

A. An appointment already attended
B. A canceled results discussion
C. Thursday at 14:00 with Dr Chen
D. Tuesday at 14:00 with a different clinician

2. What caused Robin to rearrange work?

A. A supplied abnormal result
B. Elena's inaccurate Tuesday message
C. A confirmed clinical emergency
D. An actual schedule change from Tuesday

3. Which topic is not supplied for discussion?

A. The effect of the wrong message
B. The actual test results
C. The verified appointment details
D. The correction ownership

Answers and explanations, page 144

B Vocabulary in use

CORE TERMS

appointment confirmation
Verified communication of a scheduled visit and its details.

send an appointment confirmation

schedule of record
Authoritative schedule used to verify the actual booking.

check the schedule of record

rescheduling
Changing an actual appointment from one slot to another.

distinguish rescheduling from correction

result interpretation
Clinical explanation of what findings mean in context.

refer result interpretation

twenty-four-hour time
Time notation running from 00:00 through 23:59.

use twenty-four-hour time

read-back
Repeating important information to confirm accurate receipt.

request a read-back

incorrect notification
Message containing inaccurate information.

correct an incorrect notification

message correction
Communication replacing an inaccurate earlier message.

issue a message correction

results discussion
Appointment or conversation in which a clinician explains test findings.

confirm a results discussion

date verification
Checking the actual calendar date associated with a booking.

complete date verification

time-zone reference
Identification of the time zone when relevant to communication.

clarify the time-zone reference

correction timestamp
Recorded time at which an amendment or correction occurred.

preserve the correction timestamp

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

appointment confirmation APPOINTMENT CONFIRMATION

send an appointment confirmation

Keep the meaning precise: Verified communication of a scheduled visit and its details.

results discussion RESULTS DISCUSSION

confirm a results discussion

Keep the meaning precise: Appointment or conversation in which a clinician explains test findings.

B Precision and professional shorthand

EXTEND YOUR RANGE

original entry
Earlier record item retained under the applicable process.

preserve the original entry

addendum
Additional note linked to an existing record.

enter an addendum

audit trail
Record showing the sequence and authorship of events or changes.

maintain an audit trail

communication log
Record of relevant contacts and their outcomes.

update the communication log

contact attempt
Effort to reach someone without assuming successful contact.

document a contact attempt

successful contact
Confirmed communication with the intended recipient.

verify successful contact

acknowledgment
Confirmation of receipt or recognition of information.

record acknowledgment

follow-up owner
Person responsible for a specified unfinished next action.

name the follow-up owner

outstanding action
Task that remains to be completed.

track the outstanding action

service disruption
Practical interference caused by a service problem.

acknowledge the service disruption

escalation route
Defined path for a concern requiring additional attention or authority.

explain the escalation route

closure evidence
Information establishing that the intended task was actually completed.

retain closure evidence

Say it in context

Stress the highlighted expression. Keep the surrounding words connected; pause at the end of the phrase.

correction timestamp CORRECTION TIMESTAMP

preserve the correction timestamp

Keep the meaning precise: Recorded time at which an amendment or correction occurred.

contact attempt CONTACT ATTEMPT

document a contact attempt

Keep the meaning precise: Effort to reach someone without assuming successful contact.

C Phrases that move the work forward

Open the correction

I need to correct the appointment message I sent you.

Name the error

I wrote Tuesday, but that was incorrect.

Give the verified booking

The schedule confirms Thursday at 14:00 with Dr Chen.

Translate the time

Fourteen hundred is two in the afternoon.

Distinguish the event

This corrects my message; it is not a change from a Tuesday booking.

Apologize directly

I am sorry that my incorrect message disrupted your work arrangements.

Avoid a result inference

The appointment details do not tell us what the results mean.

Keep the role clear

Dr Chen will handle the clinical discussion.

Check accurate receipt

Please read back the verified day and time.

Preserve the original

The record should retain the earlier message and its correction appropriately.

Document contact honestly

An attempted call is not the same as speaking with you.

Name the owner

I am responsible for communicating this correction.

Track unfinished work

Any unsuccessful contact must remain an open action.

Offer the actual route

We can use the clinic's process for any unresolved service concern.

Avoid an unsupported promise

I cannot promise a remedy that has not been authorized.

Close with the verified facts

Thursday at 14:00 with Dr Chen is the confirmed booking.

Precision in real exchanges

Purpose: Translate the time
Useful wording: Fourteen hundred is two in the afternoon.

Purpose: Distinguish the event
Useful wording: This corrects my message; it is not a change from a Tuesday booking.

D Choose the response that fits

Choose one response for each situation. Preserve the facts, the decision boundary, and the purpose of the exchange.

1. Which correction preserves the history?

A. Your appointment has been moved from Tuesday even though the schedule never changed.
B. Tuesday and Thursday mean the same thing here.
C. The booking is unverified because a message was wrong.
D. My Tuesday message was wrong; the verified booking is Thursday at 14:00 with Dr Chen.

2. Which conclusion about results is unsupported?

A. The booking is for a results discussion.
B. Thursday means the results are definitely normal.
C. Dr Chen is the named clinician.
D. No results are supplied in the exercise.

3. A colleague needs the term for this meaning: Communication replacing an inaccurate earlier message. Which term fits?

A. outstanding action
B. message correction
C. audit trail
D. twenty-four-hour time

D Choose the response that fits / continued

4. Rebuild the exchange from "Thursday, not Tuesday". Robin: "I have the Tuesday message right here. Was that date wrong from the start, or did someone move my appointment without telling me?" Elena: [missing response] Robin: "Fourteen hundred means two in the afternoon, correct? I want to check the time as well as the day before I speak to my manager." Which response belongs in the gap?

A. You are right. This is not rescheduling. The earlier message was wrong; I should not describe that as moving an appointment that was booked for Tuesday.

B. We need the appropriate audit trail. I will preserve the earlier event and the correction through our process rather than erase the reason for your confusion.

C. No. A results discussion booking does not tell us what the results mean. I can confirm the appointment, but the clinical explanation belongs with Dr Chen.

D. I have checked the schedule of record. Thursday at 14:00 with Dr Chen is confirmed. I am sorry my incorrect message disrupted your work arrangements.

Answers and explanations, page 144

> **One useful distinction**
>
> Correcting Tuesday to Thursday in a message is not necessarily rescheduling. The verified booking was Thursday; the communication was wrong. Keep that history accurate so a later colleague does not infer a clinical or scheduling decision that never occurred.

E Conversation

Thursday, not Tuesday

Thursday, not Tuesday

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

escalation route / audit trail / results discussion / follow-up owner / message correction / rescheduling / schedule of record / successful contact / result interpretation / twenty-four-hour time

01 **Robin:** Elena, I arranged time off for the Tuesday results discussion, but the latest information says Thursday. Has the appointment been moved again?

02 **Elena:** I need to make a [1] _____. My Tuesday message was incorrect. The verified schedule shows Thursday at 14:00 with Dr Chen.

03 **Robin:** That matters because I changed my work shift after reading what you sent. I need to know whether Thursday is confirmed or just another possible date.

04 **Elena:** I have checked the [2] _____. Thursday at 14:00 with Dr Chen is confirmed. I am sorry my incorrect message disrupted your work arrangements.

05 **Robin:** I have the Tuesday message right here. Was that date wrong from the start, or did someone move my appointment without telling me?

06 **Elena:** You are right. This is not [3] _____. The earlier message was wrong; I should not describe that as moving an appointment that was booked for Tuesday.

07 **Robin:** Fourteen hundred means two in the afternoon, correct? I want to check the time as well as the day before I speak to my manager.

08 **Elena:** Correct. That is [4] _____ for two in the afternoon. Please keep Thursday, 14:00, and Dr Chen together when you read back the details.

09 **Robin:** Thursday at two in the afternoon with Dr Chen. Does the later day tell me anything about whether the results are normal or serious?

10 **Elena:** No. A [5] _____ booking does not tell us what the results mean. I can confirm the appointment, but the clinical explanation belongs with Dr Chen.

Conversation continues on the next page.

Thursday, not Tuesday

CONVERSATION / CONTINUED

11 **Robin:** I understand. Please do not let my question about the date become a note saying that someone already explained the results to me.

12 **Elena:** I will keep [6] _____ separate from this correction. The record should describe the appointment information communicated, not a clinical conversation that did not happen.

13 **Robin:** Will the earlier message remain visible appropriately? Otherwise a later colleague may think I simply chose the wrong day when I arranged work.

14 **Elena:** We need the appropriate [7] _____. I will preserve the earlier event and the correction through our process rather than erase the reason for your confusion.

15 **Robin:** And will you record that you actually reached me today? I would like the team to know I received and repeated back the Thursday information.

16 **Elena:** Yes. This is [8] _____, not merely an attempted call. I will document the actual communication and acknowledgment through the required process.

17 **Robin:** Please send the corrected details through the clinic's usual channel too. I need something accurate to refer to when I rearrange my shift.

18 **Elena:** Exactly. A named [9] _____ would need to keep that action visible. I own this correction and will record what has actually been communicated.

19 **Robin:** Thank you for acknowledging the work disruption. I may still raise that concern through the clinic process, but I now have the confirmed appointment details.

20 **Elena:** We can use the appropriate [10] _____ for that concern. The verified booking remains Thursday at 14:00 with Dr Chen, and I am sorry for the inaccurate Tuesday message.

CHECK THE GAPS

1. message correction / **2.** schedule of record / **3.** rescheduling / **4.** twenty-four-hour time / **5.** results discussion / **6.** result interpretation / **7.** audit trail / **8.** successful contact / **9.** follow-up owner / **10.** escalation route

Why these answers fit, page 145

F Check accuracy and scope

Thursday, not Tuesday

Return to the case

Assistant Elena sent patient Robin a message incorrectly saying that a results discussion was booked Tuesday. The verified schedule shows Thursday at 14:00 with Dr Chen. Robin rearranged work after the incorrect message. Elena can correct the communication and confirm the actual booking, but no test results or clinical interpretation are supplied. The appointment itself has not been changed from Tuesday to Thursday; the earlier message was wrong. Elena must preserve that distinction, acknowledge the disruption, and document whether the correction actually reached Robin.

Choose one answer per question. Use the case facts and keep any uncertainty or limits intact.

1. What should be recorded if Robin is not reached?

A. The contact attempt and the still-outstanding correction
B. Confirmed patient receipt
C. Completed results counseling
D. Robin's agreement to the booking

2. Which apology is strongest?

A. I am sorry you understood the Tuesday message differently.
B. I am sorry the schedule changed, although the verified booking never changed.
C. I am sorry for the inconvenience; the original date does not matter now.
D. I am sorry my incorrect Tuesday message disrupted your work arrangements.

Answers and explanations, page 147

Listen for the distinction

Switch roles. Repeat the apology and read-back, saying 14:00 and two in the afternoon as the same time.

Correct the message without rewriting the history

An apology is more useful when it names what went wrong and supplies the verified replacement information. Avoid describing an inaccurate message as a rescheduled visit if the booking never changed. Acknowledge the practical effect without making an unsupported compensation promise.

F Say it / transfer to a new case

Correct a different time error

Assistant Noor sent 09:00 in error. The verified booking is Friday at 11:30 with Dr Shah. The appointment itself has not changed. Patient Ari has now received the correction.

Complete the four gaps. With a partner, read alternate roles, then switch. Studying alone, read both roles and pause before each response.

WORD AND PHRASE BANK

Shah / 11:30 / 09:00 / rescheduling

01 **Noor:** My message incorrectly said [1] _____.

02 **Ari:** The verified time is [2] _____.

03 **Noor:** The clinician is Dr [3] _____.

04 **Ari:** This is a message correction, not [4] _____.

CHECK THE GAPS

1. 09:00 / **2.** 11:30 / **3.** Shah / **4.** rescheduling

Why these answers fit, page 147

Rebuild the exchange

Choose the missing response. Read the completed three-turn exchange aloud, changing roles on the second reading.

1. Rebuild the exchange from "Thursday, not Tuesday". Robin: "Will the earlier message remain visible appropriately? Otherwise a later colleague may think I simply chose the wrong day when I arranged work." Elena: [missing response] Robin: "And will you record that you actually reached me today? I would like the team to know I received and repeated back the Thursday information." Which response belongs in the gap?

A. You are right. This is not rescheduling. The earlier message was wrong; I should not describe that as moving an appointment that was booked for Tuesday.

B. We need the appropriate audit trail. I will preserve the earlier event and the correction through our process rather than erase the reason for your confusion.

C. No. A results discussion booking does not tell us what the results mean. I can confirm the appointment, but the clinical explanation belongs with Dr Chen.

D. I have checked the schedule of record. Thursday at 14:00 with Dr Chen is confirmed. I am sorry my incorrect message disrupted your work arrangements.

Answers and explanations, page 148

Review 4 / Meaning and context

Use language from the lessons you have completed. Choose one answer per question.

> **Case file**
>
> Revisit "Telephone concerns and clinical escalation" and "Records, corrections, and follow-up ownership". Choose precise terminology, then reconstruct the missing responses using both neighboring turns. The final two questions also revisit earlier conversations.

1. A colleague needs the term for this meaning: Evaluation of how promptly a concern requires attention. Which term fits?

A. urgency assessment
B. verified callback number
C. symptom progression
D. blind transfer

2. A colleague needs the term for this meaning: Checking the actual calendar date associated with a booking. Which term fits?

A. original entry
B. date verification
C. acknowledgment
D. appointment confirmation

3. Rebuild the exchange from "Clarify before leaving". Jules: "Could you show Dr Chen this line on the summary? It is this phrase, not the whole page, that I cannot make sense of." Mina: [missing response] Jules: "I would like a plain explanation, not just the same phrase repeated more loudly. I heard it; the meaning is what I need." Which response belongs in the gap?

A. Thank you for raising it. This is your after-visit summary. Please indicate the phrase, and I will pass the actual wording and your question to Dr Chen.
B. I will preserve the patient question with the wording. That gives Dr Chen the actual issue rather than a vague message that could miss it.
C. Recognizing a technical term does not always make the instruction clear. Asking again is reasonable; receiving the paper does not mean every question has been answered.
D. We can arrange an appropriate private discussion. I will tell Dr Chen that you want clarification before leaving and that the setting matters to you.

Review 4 / Meaning and context / continued

4. Rebuild the exchange from "Thursday, not Tuesday". Robin: "Please send the corrected details through the clinic's usual channel too. I need something accurate to refer to when I rearrange my shift." Elena: [missing response] Robin: "Thank you for acknowledging the work disruption. I may still raise that concern through the clinic process, but I now have the confirmed appointment details." Which response belongs in the gap?

A. Correct. That is twenty-four-hour time for two in the afternoon. Please keep Thursday, 14:00, and Dr Chen together when you read back the details.

B. I will keep result interpretation separate from this correction. The record should describe the appointment information communicated, not a clinical conversation that did not happen.

C. Exactly. A named follow-up owner would need to keep that action visible. I own this correction and will record what has actually been communicated.

D. I have checked the schedule of record. Thursday at 14:00 with Dr Chen is confirmed. I am sorry my incorrect message disrupted your work arrangements.

Answers and explanations, page 158

Language for the next case

Book the right language support

spoken-language preference

Mei stated Mandarin for spoken communication; Cantonese in the copied appointment note conflicts with that confirmed preference.

Jo: The recorded spoken-language preference is Mandarin. The Cantonese entry was copied incorrectly. Please correct that entry through our record process and preserve the reason.

written-language preference

Traditional Chinese is Mei's stated choice for written materials; it should not be inferred from the spoken-language field or replaced by it.

Jo: Exactly. The written-language preference guides the material request. Check that the available document is the approved version in that form, not just any document labeled Chinese.

qualified interpreter

The clinic has arranged a qualified healthcare interpreter for Mandarin; a relative's presence for support does not replace that booking.

Jo: No. Keep our qualified interpreter booking. Mei asked for her sister's support, not for us to withdraw the language service.

video interpreting

The booked service uses video for the live spoken exchange; it is not a written translation service or merely a telephone appointment reminder.

Jo: Please do. Record that it is video interpreting, including the appropriate service details. A booking confirmation does not prove the interpreter has joined tomorrow's session.

short segments

Short segments let the interpreter relay each part accurately and allow the patient to ask questions without relying on a long end-of-explanation summary.

Jo: Yes. Use short segments and pause for interpretation. Preserve the patient's questions too, not just the clinician's instructions.

teach-back

Teach-back checks the clarity of an explanation by hearing the patient's understanding in their own words; a yes response or fluent English is not the test.

Jo: That is teach-back. The interpreter relays the response; the clinician resolves any clinical misunderstanding.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

Book the right language support

Before tomorrow's visit, assistant Mina and coordinator Jo check Mei's recorded preferences: spoken Mandarin, written Traditional Chinese. Mei requests her sister's support. The clinic has booked a qualified video interpreter for Mandarin; a copied appointment note incorrectly says Cantonese. No diagnosis or treatment discussion is taking place here.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

spoken-language preference / video interpreting / qualified interpreter / short segments / teach-back / written-language preference

01 **Mina:** Jo, the interpreter booking says Mandarin, but tomorrow's appointment note says Cantonese. Both are attached to Mei's visit. Which preference did we actually confirm with her?

02 **Jo:** The recorded [1] _____ is Mandarin. The Cantonese entry was copied incorrectly. Please correct that entry through our record process and preserve the reason.

03 **Mina:** Her written preference is Traditional Chinese. I should keep that as a separate field, rather than assume that the spoken language tells me which written materials she wants.

04 **Jo:** Exactly. The [2] _____ guides the material request. Check that the available document is the approved version in that form, not just any document labeled Chinese.

05 **Mina:** Mei wants her sister in the visit. Should I cancel the interpreter and put family interpreting instead?

06 **Jo:** No. Keep our [3] _____ booking. Mei asked for her sister's support, not for us to withdraw the language service.

Conversation / continued

07 **Mina:** I will tell the team that distinction. We should address Mei directly, rather than asking her sister to answer every question because it seems quicker.

08 **Jo:** Yes. The clinician should speak to Mei and allow the interpreter to relay the exchange. The sister can contribute within the arrangement Mei agrees to; she does not automatically speak for her.

09 **Mina:** The service is by video. I can check the room equipment and booking connection before the visit, without opening a clinical conversation in a public area.

10 **Jo:** Please do. Record that it is [4] _____, including the appropriate service details. A booking confirmation does not prove the interpreter has joined tomorrow's session.

Conversation continues on the next page.

Book the right language support

CONVERSATION / CONTINUED

11 **Mina:** For the handoff, I will say Mandarin interpreter booked and Traditional Chinese materials requested. I will not say all language needs are complete before the document and connection checks.

12 **Jo:** Good. If the required document is unavailable, bring that back to me. Do not substitute an unreviewed automatic translation for a clinical instruction and mark it approved.

13 **Mina:** The clinician asked whether the interpreter could simply summarize a long explanation at the end. Would shorter sections make the exchange easier to check?

14 **Jo:** Yes. Use [5] _____ and pause for interpretation. Preserve the patient's questions too, not just the clinician's instructions.

15 **Mina:** Mei may want to discuss something without her sister present. We need to offer that opportunity respectfully, without treating the request as a family disagreement.

16 **Jo:** Correct. Follow our private-conversation process and ask Mei about her wishes. The interpreter's confidentiality obligations remain important whether or not a family member is present.

17 **Mina:** For checking an explanation, the clinician can ask Mei to describe the plan in her own words through the interpreter, rather than just asking whether she understands.

18 **Jo:** That is [6] _____. The interpreter relays the response; the clinician resolves any clinical misunderstanding.

19 **Mina:** I will correct Cantonese to the confirmed Mandarin preference, keep the written preference separate, retain the interpreter booking, and check the materials and video arrangements.

20 **Jo:** Thank you. I will check the approved document request. We will leave the remaining preparation tasks visible rather than mark the visit language support complete prematurely.

CHECK THE GAPS

1. spoken-language preference / **2.** written-language preference / **3.** qualified interpreter / **4.** video interpreting / **5.** short segments / **6.** teach-back

Why these answers fit, page 161

Two preferences, two arrangements

Luis confirms spoken Spanish and written English. His adult son will attend for support. The clinic has booked a qualified Spanish telephone interpreter. The connection has not yet occurred; approved English paperwork is available.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

Spanish / English / telephone / support

01 **Assistant:** Luis's spoken preference is [1] _____ .

02 **Coordinator:** His written preference is [2] _____ .

03 **Assistant:** The booked interpreter will join by [3] _____ .

04 **Coordinator:** The son's attendance is for [4] _____ .

CHECK THE GAPS

1. Spanish / **2.** English / **3.** telephone / **4.** support

Why these answers fit, page 161

Keep the distinction audible

Confirm spoken and written language preferences separately and distinguish family support from qualified interpreting.

Language for the next case

A number needs its unit

source measurement

The saved scale output establishes 160.0 pounds at 09:12; appearance or the patient's previous weight would not establish this measurement's unit.

Dana: Check the source measurement, including its unit and time. Do not decide which label looks right from the patient's appearance or a remembered earlier weight.

unit conversion

A unit conversion changes the numerical value and unit together while representing the same measured quantity; relabeling 160.0 pounds as kilograms changes its meaning.

Dana: Correct. A unit conversion preserves the quantity by changing both number and unit appropriately. Also check whether the system already converts values, so the same reading is not converted twice.

audit trail

The audit trail preserves the original entry, correction, reason, and authorship under the clinic process; an unexplained overwrite would hide the recording error.

Dana: Yes. The audit trail matters if somebody has already used the draft. Tell me which records or workflows may have received it so we can investigate promptly.

72.6

160.0 multiplied by 0.45359237 equals 72.5747792 kilograms, which rounds to 72.6 kilograms at one decimal place.

Dana: To one decimal place, enter 72.6 kilograms through the correction process. Keep the original pounds reading and the conversion identifiable; those are not two separate weighings.

height

The separate 170-centimeter entry describes height; its conversion to 1.70 meters is unrelated to converting the weight from pounds to kilograms.

Dana: Right. Keep height separate from weight. Use the verified value and required unit for each field, not a copied unit from the previous row.

measurement time

The measurement occurred at 09:12; the later correction time is a separate record event and must not make the conversion look like a new weighing.

Dana: Preserve the measurement time as nine twelve and record the correction time separately as required. Converting an existing reading does not mean a new measurement occurred.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

A number needs its unit

Fictional record check: today's saved scale output is 160.0 lb at 09:12. A draft entry incorrectly says 160.0 kg. Height was recorded separately as 170 cm. Use 1 lb = 0.45359237 kg; round the converted weight to one decimal place. Nurse Dana reviews the correction; clinical use of the draft is not yet checked.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

72.6 / audit trail / height / unit conversion / measurement time / source measurement

01 **Rosa:** Dana, I found a mismatch before closing the rooming record. The saved scale output says one hundred sixty point zero pounds, but the draft says kilograms.

02 **Dana:** Check the [1] _____, including its unit and time. Do not decide which label looks right from the patient's appearance or a remembered earlier weight.

03 **Rosa:** The saved output is clear: 160.0 lb at nine twelve today. If our field requires kilograms, multiplying by the supplied factor gives seventy-two point five seven four eight.

04 **Dana:** To one decimal place, enter [2] _____ kilograms through the correction process. Keep the original pounds reading and the conversion identifiable; those are not two separate weighings.

05 **Rosa:** So changing the unit label alone would be wrong. One hundred sixty kilograms is not another way of expressing one hundred sixty pounds.

06 **Dana:** Correct. A [3] _____ preserves the quantity by changing both number and unit appropriately. Also check whether the system already converts values, so the same reading is not converted twice.

Conversation / continued

07 **Rosa:** The height entry is one hundred seventy centimeters. In meters, that is one point seven zero. It is not one hundred seventy inches.

08 **Dana:** Right. Keep [4] _____ separate from weight. Use the verified value and required unit for each field, not a copied unit from the previous row.

09 **Rosa:** Would it help to write that the corrected weight is normal? The number looks more plausible now.

10 **Dana:** No. Plausibility is not a clinical assessment. We are correcting a recording error, not assigning a weight category, calculating a treatment dose, or declaring the patient healthy.

Conversation continues on the next page.

A number needs its unit

CONVERSATION / CONTINUED

11 **Rosa:** I will retain the original entry and record what changed and why under our process. I should not delete the history and make it look as if no error occurred.

12 **Dana:** Yes. The [5] _____ matters if somebody has already used the draft. Tell me which records or workflows may have received it so we can investigate promptly.

13 **Rosa:** I have not checked that yet. I will not say no effect just because the error was in a draft field. You need to know the downstream use is unresolved.

14 **Dana:** Thank you. I will handle the clinical implications through the appropriate review. Your job is to supply the measurement evidence and report the discrepancy, not estimate harm from the arithmetic alone.

15 **Rosa:** Should the converted entry carry the time I make the correction, or nine twelve when the patient was actually weighed?

16 **Dana:** Preserve the [6] _____ as nine twelve and record the correction time separately as required. Converting an existing reading does not mean a new measurement occurred.

17 **Rosa:** If the scale record had lacked a unit, I would need clarification through our procedure rather than automatically apply the pounds-to-kilograms factor.

18 **Dana:** Exactly. This case gives a verified unit. When the source is uncertain, multiplying by a familiar factor can make an unsupported assumption look precise.

19 **Rosa:** Read-back: original 160.0 pounds at 09:12; converted 72.6 kilograms to one decimal; separate height 170 centimeters, or 1.70 meters. Correction history retained; downstream use still being checked.

20 **Dana:** Confirmed. Bring the corrected record for the required check. The arithmetic is clear, but it does not by itself close the review of where the incorrect entry may have gone.

CHECK THE GAPS

1. source measurement / **2.** 72.6 / **3.** unit conversion / **4.** height / **5.** audit trail / **6.** measurement time

Why these answers fit, page 163

Convert once and preserve the source

A verified scale output is 132.0 lb at 10:18. The record requires kilograms to one decimal; use 1 lb = 0.45359237 kg. A separate verified height is 165 cm. The correction is entered at 10:31.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

10:18 / 1.65 / 59.9 / 10:31

01 **Assistant:** The weight rounds to [1] _____ kilograms.

02 **Nurse:** The original measurement time remains [2] _____ .

03 **Assistant:** The separate height is [3] _____ meters.

04 **Nurse:** The record was corrected at [4] _____ .

CHECK THE GAPS

1. 59.9 / **2.** 10:18 / **3.** 1.65 / **4.** 10:31

Why these answers fit, page 163

Keep the distinction audible

Read back a measurement with its unit and explain a traceable correction without inventing a new measurement or clinical conclusion.

Language for the next case

A clear trace is not a diagnosis

lead-off warning

A lead-off warning concerns the recording connection; it is not itself a diagnosis about the patient's heart or proof that all clinical findings are artifact.

Dr Shah: Start with the technical history. A lead-off warning is not a clinical diagnosis. Tell me what was observed and checked, without deciding what the waveform means.

Signal quality

Signal quality concerns the recording's clarity and interference; it is distinct from whether the patient's ECG has a clinically important finding.

Dr Shah: Report the actual technical observations. Signal quality and clinical interpretation answer different questions. Do not turn easier to read into a normal-heart conclusion.

repeat acquisition

Repeat acquisition means obtaining a new recording at 11:10; it does not erase the original 11:06 trace or establish a clinical change between them.

Dr Shah: Thank you. Keep the repeat acquisition distinguishable from the original. A difference between traces needs review, not an assumption that the patient's condition changed.

electrodes

The ten skin electrodes acquire the signals used for a standard twelve-lead ECG; the number of electrodes is not the number of displayed electrical views.

Dr Shah: Yes. The electrodes make contact with the skin. The twelve leads represent different electrical views, not twelve separate adhesive patches.

Automated interpretation

Automated interpretation is the device's analysis, not a completed clinician review; its wording must not be presented as Dr Shah's confirmed conclusion.

Dr Shah: Correct. Automated interpretation supports the review but does not replace it. I need the actual recordings and patient context, not just a sentence from the device.

reviewing clinician

The reviewing clinician has received the material but must still review it; receipt alone does not establish interpretation or a discharge decision.

Dr Shah: Right. Receipt by the reviewing clinician is one stage. The clinical findings and next steps belong in the appropriate clinical record after the actual assessment.

Read the phrases aloud. Then complete the conversation on the following pages using the word bank.

Conversation

A clear trace is not a diagnosis

Assistant Leo is trained and authorized for ECG acquisition under this fictional clinic's procedure, not interpretation. A 12-lead ECG recorded at 11:06 has a lead-off warning. After the prescribed technical checks, an authorized repeat is recorded at 11:10. Its automated wording is unconfirmed. Dr Shah receives both traces and the patient's question.

Complete the numbered gaps. Use each bank entry once; some entries contain several words.

WORD AND PHRASE BANK

repeat acquisition / electrodes / Automated interpretation / lead-off warning / Signal quality / reviewing clinician

01 **Leo:** Dr Shah, I have the original eleven-oh-six ECG and the repeat at eleven ten. The first recording showed a lead-off warning. Both are labeled and retained for your review.

02 **Dr Shah:** Start with the technical history. A [1] _____ is not a clinical diagnosis. Tell me what was observed and checked, without deciding what the waveform means.

03 **Leo:** I completed the prescribed technical checks within my authorized role, then recorded the approved repeat. I have documented those checks; I have not attributed any clinical finding to a loose connection.

04 **Dr Shah:** Good. Keep the acquisition history attached. The patient should not be told everything is normal just because the technical warning no longer appears on the second recording.

05 **Leo:** The patient asked why a twelve-lead test uses ten sticky patches. I explained that the patches are electrodes and that lead has another meaning in the test name.

06 **Dr Shah:** Yes. The [2] _____ make contact with the skin. The twelve leads represent different electrical views, not twelve separate adhesive patches.

Conversation / continued

07 **Leo:** I did not give a placement lesson or change the prescribed positions to improve the picture. Any acquisition difficulty follows our equipment instructions and clinic procedure.

08 **Dr Shah:** Exactly. Correct patient identification and recording details matter too. A technically clear trace with the wrong patient label would still be an unsafe record.

09 **Leo:** The repeat looks easier to read on screen. May I describe that as better signal quality while leaving the clinical meaning to you?

10 **Dr Shah:** Report the actual technical observations. [3] _____ and clinical interpretation answer different questions. Do not turn easier to read into a normal-heart conclusion.

Conversation continues on the next page.

A clear trace is not a diagnosis

CONVERSATION / CONTINUED

11 **Leo:** The machine also printed an automated statement. I have left it identified as machine-generated and unconfirmed, rather than copying it into a message as your conclusion.

12 **Dr Shah:** Correct. [4] _____ supports the review but does not replace it. I need the actual recordings and patient context, not just a sentence from the device.

13 **Leo:** The patient asked whether the repeat means something serious was found. I said you would explain the findings, and that I repeated the recording for the documented technical reason.

14 **Dr Shah:** That keeps the explanation within your role without minimizing the question. Pass any current concern through the clinical route promptly; a technical check must not delay necessary care.

15 **Leo:** For the record, both recordings retain their own times. I have not overwritten eleven oh six with eleven ten or made the first trace disappear.

16 **Dr Shah:** Thank you. Keep the [5] _____ distinguishable from the original. A difference between traces needs review, not an assumption that the patient's condition changed.

17 **Leo:** The chart therefore shows the initial warning, documented checks, original and repeat times, and your receipt. It does not say interpreted, normal, or cleared to leave.

18 **Dr Shah:** Right. Receipt by the [6] _____ is one stage. The clinical findings and next steps belong in the appropriate clinical record after the actual assessment.

19 **Leo:** My handoff is complete: both traces, technical history, and the patient's question are with you. No clinical result has been communicated by me.

20 **Dr Shah:** Received. I will review the recordings and speak with the patient. Retain your factual acquisition note without adding a result that I have not provided.

CHECK THE GAPS

1. lead-off warning / **2.** electrodes / **3.** Signal quality / **4.** Automated interpretation / **5.** repeat acquisition / **6.** reviewing clinician

Why these answers fit, page 165

Name the stage that actually occurred

A standard 12-lead ECG uses 10 skin electrodes. The initial trace is timed 14:02 and an authorized repeat 14:07. Dr Malik has received both, but no clinical interpretation is supplied.

Complete the exchange using each phrase once. Read both roles, then switch roles and repeat with the bank covered.

WORD AND PHRASE BANK

14:02 / supplied / 10 / 14:07

01 **Assistant:** This standard twelve-lead recording uses [1] _____ skin electrodes.

02 **Clinician:** The original trace is timed [2] _____ .

03 **Assistant:** The repeat is timed [3] _____ .

04 **Clinician:** A clinical interpretation is not yet [4] _____ .

CHECK THE GAPS

1. 10 / **2.** 14:02 / **3.** 14:07 / **4.** supplied

Why these answers fit, page 165

Keep the distinction audible

Distinguish ECG electrodes, recorded leads, technical warnings, and clinical interpretation during a clinician handoff.

01 / Answers and explanations

A / Read the situation

A1. What does the schedule show?

A. An annual review

The schedule lists an annual review, while the patient's expectation differs.

Return to the exercise, page 4

A2. What should Sam do with the name preference?

D. Use Alex while preserving the verified record match.

Alex is the preferred spoken name; that does not justify a duplicate record or an unverified identifier change.

Return to the exercise, page 4

A3. Who can clarify the visit plan?

C. Nurse Dana

Dana is the available clinical route, and Sam cannot determine suitability independently.

Return to the exercise, page 4

D / Response choices

D1. Which handoff preserves both facts?

A. The schedule says annual review; Alex expected to discuss a recent concern.

The statement preserves the scheduled category and reported expectation without inventing fault or a clinical decision.

Return to the exercise, page 8

D2. What does using Alex change here?

B. The respectful form of spoken address, not the verified identity match

The preferred name affects address while record matching and other decisions remain separate.

Return to the exercise, page 8

01 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Issue the patient brings to the current visit. Which term fits?

A. presenting concern

A Correct. presenting concern: Issue the patient brings to the current visit. The usual combination is "record the presenting concern".

B Not here. patient expectation: What a patient believes will happen during a service or visit. The usual combination is "acknowledge the patient expectation".

C Not here. patient identifier: Information used to match a person with the correct record. The usual combination is "confirm patient identifiers".

D Not here. visit plan: Intended approach to the current appointment. The usual combination is "clarify the visit plan".

Return to the exercise, page 8

D4. Rebuild the exchange from "The schedule and the expectation". Alex: "Good. I worry that the appointment category means nobody will hear the reason I actually came. Do I need to book something else?" Sam: [missing response] Alex: "I do not want to explain personal details here in front of everyone. Can the question reach Dana without a discussion across reception?" Which response belongs in the gap?

B. I cannot decide clinical suitability from the schedule label. Dana can clarify how the visit should address your concern before we assume that another booking is required.

A Not here. This line belongs to turn 8, after Alex says: "I do not want to explain personal details here in front of everyone. Can the question reach Dana without a discussion across reception?" It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Sam's response in turn 6. It connects Alex's preceding point with Alex's follow-up. Clinical suitability requires the appropriate clinical judgment; Sam cannot decide it from the appointment label alone.

C Not here. This line belongs to turn 20, after Alex says: "All right. Can you let Dana know I am here? I would like to settle the visit plan before I explain everything again." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 16, after Alex says: "Please also make sure the team uses Alex. Sometimes one person agrees, but the preference disappears as soon as I move to another room." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 8

E / Conversation

1. scheduled purpose

Turn 2. The scheduled purpose is the annual review recorded in the booking, which differs from Alex's stated expectation.

01 / Answers and explanations / continued

2. preferred name

Turn 4. The preferred name is Alex, while the verified record name remains Alexandra and must stay correctly linked.

3. clinical suitability

Turn 6. Clinical suitability requires the appropriate clinical judgment; Sam cannot decide it from the appointment label alone.

4. presenting concern

Turn 8. The presenting concern is what Alex wants addressed, and it should reach the clinical team without unnecessary public disclosure.

5. patient expectation

Turn 10. Patient expectation describes what Alex believed the visit would address, not merely dissatisfaction with an administrative reminder.

6. billing category

Turn 12. A billing category may affect charging, but the scenario supplies no verified charge or coverage decision from the visit label.

7. clinical handoff

Turn 14. The clinical handoff passes the mismatch to Dana, the identified clinical route, without Sam inventing a treatment decision.

8. identity match

Turn 16. The identity match links Alex to the verified existing record and must remain intact while the preferred address is communicated.

9. visit plan

Turn 18. The visit plan concerns what this encounter will address and is the question Dana can clarify with Alex.

10. appointment type

Turn 20. The appointment type remains the recorded annual review, distinct from the clinical clarification now requested.

Return to the conversation, page 10

01 / Answers and explanations / continued

F / Accuracy and scope

F1. Which reassurance is unsupported?

C. This visit will definitely resolve the concern at no additional cost.

Neither the clinical outcome nor any billing decision is established by the facts.

Return to the exercise, page 12

F2. Which action is appropriate?

A. Pass the mismatch to Dana while keeping the correctly matched record.

The available route is clinical clarification with accurate identification, not an unauthorized record or treatment change.

Return to the exercise, page 12

F / Guided speaking

1. follow-up

Lee: The recorded visit is a follow-up.

Turn 1. Follow-up is the scheduled category stated in this scenario.

2. initial consultation

Morgan: I expected an initial consultation.

Turn 2. The patient's expectation differs from the recorded follow-up category.

3. Inez

Lee: Nurse Inez can clarify the plan.

Turn 3. Inez is the available clinical professional named in the facts.

4. established

Lee: Clinical suitability is not yet established.

Turn 4. No clinical suitability decision has been supplied for this mismatch.

Return to the conversation, page 13

01 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "The schedule and the expectation". Alex: "Please check before anyone promises it is covered. I took time off for this visit, and I need to understand what will actually happen today." Sam: [missing response] Alex: "Please also make sure the team uses Alex. Sometimes one person agrees, but the preference disappears as soon as I move to another room." Which response belongs in the gap?

B. I can confirm the schedule and your expectation, then request a clinical handoff to Dana. I cannot promise how the clinician will resolve the concern.

A Not here. This line belongs to turn 18, after Alex says: "Thank you. I do not need you to change the appointment yourself; I need someone qualified to clarify what today's visit can include." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Sam's response in turn 14. It connects Alex's preceding point with Alex's follow-up. The clinical handoff passes the mismatch to Dana, the identified clinical route, without Sam inventing a treatment decision.

C Not here. This line belongs to turn 12, after Alex says: "Is this a billing problem too? I have heard different visit categories can mean different charges, and I do not want a surprise." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 10, after Alex says: "Will you tell Dana that I expected this to be the concern visit, rather than just saying that I am unhappy about the reminder?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 13

02 / Answers and explanations

A / Read the situation

A1. What onset did Pat report?

C. Monday

Monday is the onset reported by Pat, not an independently measured event.

Return to the exercise, page 14

A2. What did Rosa observe?

B. Pat answering questions while seated

Only the seated question-answering observation is supplied; measurements and conclusions are absent.

Return to the exercise, page 14

A3. What should remain unassessed in the report?

A. Cause and severity

Rosa has not assessed cause or severity and must not manufacture those findings.

Return to the exercise, page 14

D / Response choices

D1. Which report is supported?

B. Pat reports tiredness since Monday; I observed Pat answering questions while seated.

The supported report preserves both supplied sources without adding a cause, severity, or safety judgment.

Return to the exercise, page 19

D2. What belongs in the temperature field when no measurement is available?

C. No invented numeric value

No measurement is supplied; neither a previous value nor an impression can establish today's temperature.

Return to the exercise, page 19

02 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Length of time an experience or condition has continued. Which term fits?

D. duration

A Not here. baseline: Usual or earlier state used for comparison. The usual combination is "establish the baseline".

B Not here. clinical assessment: Qualified evaluation of health information and the patient's condition. The usual combination is "request clinical assessment".

C Not here. pulse rate: Number of palpable arterial pulses per unit of time. The usual combination is "measure pulse rate".

D Correct. duration: Length of time an experience or condition has continued. The usual combination is "document symptom duration".

Return to the exercise, page 19

D4. Rebuild the exchange from "Report, observation, assessment". Rosa: "The handoff draft says looks fine. I will replace that with what I saw: Pat was seated and answering my questions." Dana: [missing response] Rosa: "I also need to flag the blank measurement fields. I have no values to report; those fields should not be read as normal findings." Which response belongs in the gap?

A. Exactly. Keep the observation specific. Answering questions while seated does not establish the severity of tiredness or whether a concern can safely wait.

A Correct. This is Dana's response in turn 6. It connects Rosa's preceding point with Rosa's follow-up. The observation is Pat answering questions while seated; it does not establish a diagnosis, severity, or normal measurements.

B Not here. This line belongs to turn 16, after Rosa says: "The handoff can be short: Pat reports tiredness since Monday; observed answering questions while seated; cause and severity not assessed by me." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 14, after Rosa says: "I also cannot say this is a change from Pat's normal condition unless I have an established usual state to compare it with." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 10, after Rosa says: "Should I write that Pat denied other symptoms? None are included in the information I am handing over, but I have not established their absence." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 19

E / Conversation

1. source attribution

Turn 2. Source attribution identifies Pat as the source of the symptom report and Rosa as the source of the observation.

02 / Answers and explanations / continued

2. onset

Turn 4. Onset is the stated beginning of the symptom; Monday is attributed to Pat rather than independently established.

3. observation

Turn 6. The observation is Pat answering questions while seated; it does not establish a diagnosis, severity, or normal measurements.

4. Vital signs

Turn 8. Vital signs require actual measurements and context; expected or typical values cannot fill missing clinical data.

5. pertinent negative

Turn 10. A pertinent negative is an established relevant absence, not a conclusion drawn from an unmentioned symptom.

6. patient-reported symptom

Turn 12. The patient-reported symptom is the experienced tiredness described by Pat; attribution remains necessary even when it is paraphrased.

7. baseline

Turn 14. A baseline supplies the known usual or earlier state needed for a meaningful comparison; it is not supplied here.

8. unassessed

Turn 16. Unassessed means not evaluated through the relevant process, not evaluated and found normal or harmless.

9. Clinical assessment

Turn 18. Clinical assessment is the qualified evaluation still needed; it must not be inferred from Rosa's limited rooming observation.

10. severity

Turn 20. Severity remains unassessed in Rosa's report and cannot be inferred merely from Pat speaking while seated.

Return to the conversation, page 21

02 / Answers and explanations / continued

F / Accuracy and scope

F1. Why is "Pat is fine" inadequate?

C. It substitutes a broad conclusion for the limited observation.

Fine does not describe the actual observation and implies a clinical reassurance that Rosa has not established.

Return to the exercise, page 23

F2. Which statement about other symptoms is accurate?

B. No other symptoms are supplied; that does not prove their absence.

Silence in the case does not constitute a negative clinical history or assessment.

Return to the exercise, page 23

F / Guided speaking

1. dizziness

Noor: Lee reports dizziness since breakfast.

Turn 1. Dizziness is the symptom attributed to Lee in the facts.

2. breakfast

Inez: The reported onset is breakfast.

Turn 2. Breakfast is the reported starting point, not a measured cause.

3. seated

Noor: I observed Lee speaking while seated.

Turn 3. The supplied observation concerns speaking in a seated position.

4. unassessed

Inez: The cause remains unassessed.

Turn 4. No assessment of the cause has been supplied here.

Return to the conversation, page 24

02 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Report, observation, assessment". Rosa: "I also cannot say this is a change from Pat's normal condition unless I have an established usual state to compare it with." Dana: [missing response] Rosa: "The handoff can be short: Pat reports tiredness since Monday; observed answering questions while seated; cause and severity not assessed by me." Which response belongs in the gap?

D. Right. A baseline is not something we should invent. Your report can remain useful without adding a comparison that has not been obtained.

A Not here. This line belongs to turn 6, after Rosa says: "The handoff draft says looks fine. I will replace that with what I saw: Pat was seated and answering my questions." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 10, after Rosa says: "Should I write that Pat denied other symptoms? None are included in the information I am handing over, but I have not established their absence." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 16, after Rosa says: "The handoff can be short: Pat reports tiredness since Monday; observed answering questions while seated; cause and severity not assessed by me." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Dana's response in turn 14. It connects Rosa's preceding point with Rosa's follow-up. A baseline supplies the known usual or earlier state needed for a meaningful comparison; it is not supplied here.

Return to the exercise, page 24

03 / Answers and explanations

A / Read the situation

A1. What does Ren report?

D. Another clinic stopped one medicine.

Ren reports an outside change, but the instruction is not available for verification.

Return to the exercise, page 27

A2. What conflicts with that report?

A. The medicine remains on the local list.

The local list still includes the medicine, creating the stated discrepancy.

Return to the exercise, page 27

A3. What can Dana do?

D. Review the discrepancy with the clinician

Dana is the identified route for appropriate review, not a supplied treatment decision.

Return to the exercise, page 27

D / Response choices

D1. Which note is accurate?

D. Patient reports outside discontinuation; local list still includes medicine; instruction unavailable; referred to Dana.

The note preserves the two sources, missing instruction, and review route without inventing a treatment decision.

Return to the exercise, page 31

D2. What does the active local entry establish by itself?

D. The medicine remains listed locally, not that a new instruction has been verified.

A record status does not automatically resolve a conflicting report or create a new prescription.

Return to the exercise, page 31

03 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Direction originating from another healthcare source. Which term fits?

D. outside instruction

A Not here. duplicate entry: Repeated record item that may need clarification. The usual combination is "investigate a duplicate entry".

B Not here. allergy history: Account of suspected or confirmed allergic reactions with relevant details. The usual combination is "document allergy history".

C Not here. dosage form: Physical form of a medicine, such as tablet or solution. The usual combination is "confirm the dosage form".

D Correct. outside instruction: Direction originating from another healthcare source. The usual combination is "verify the outside instruction".

Return to the exercise, page 31

D4. Rebuild the exchange from "Two sources, one unresolved entry". Ren: "Then please do not remove it and say everything is sorted either. I want the right person to check what the other clinic actually intended." Talia: [missing response] Ren: "I appreciate that. Sometimes questions about medicines sound as though I am being accused of not following instructions, even when I am trying to explain a change." Which response belongs in the gap?

C. Exactly. We need the outside instruction and appropriate review. Nurse Dana can take the discrepancy to the clinician; I cannot verify the change myself from this conversation.

A Not here. This line belongs to turn 4, after Ren says: "Does the computer mean I should be taking it after all? I do not want two clinics giving me opposite directions without explaining why." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 12, after Ren says: "The medicine being marked active is what confused me. Is that label always current, or can the record need updating after another clinic makes a change?" It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Talia's response in turn 6. It connects Ren's preceding point with Ren's follow-up. The outside instruction is the missing source needed for appropriate verification; its absence does not prove Ren's report is false.

D Not here. This line belongs to turn 18, after Ren says: "Please make sure any current question about what I should do reaches the clinical team. I do not want the record discussion mistaken for actual advice." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 31

E / Conversation

1. patient report

Turn 2. The patient report attributes the claimed outside change to Ren without pretending the original instruction has been verified.

03 / Answers and explanations / continued

2. local medication list

Turn 4. The local medication list is the current organization's record, not automatic proof of a verified present treatment instruction.

3. outside instruction

Turn 6. The outside instruction is the missing source needed for appropriate verification; its absence does not prove Ren's report is false.

4. adherence

Turn 8. Adherence concerns use in relation to an agreed plan; a conflict between records does not establish deliberate nonadherence.

5. verification source

Turn 10. A verification source is the document or professional used to confirm information; Ren's attributed report is not a substitute for that review.

6. active entry

Turn 12. An active entry describes the system status and may require reconciliation; it cannot independently settle the reported outside change.

7. Strength

Turn 14. Strength describes active ingredient per product unit and is different from the amount prescribed for a dose.

8. discrepancy

Turn 16. A discrepancy is the unresolved difference between Ren's report and the local record, not a confirmed treatment error or new instruction.

9. Medication reconciliation

Turn 18. Medication reconciliation compares and resolves medication information through the authorized process; merely documenting a conflict does not complete it.

10. discontinuation

Turn 20. Discontinuation is the claimed stopping of the medicine; it remains a reported change rather than a verified decision in this conversation.

Return to the conversation, page 33

03 / Answers and explanations / continued

F / Accuracy and scope

F1. Which reply crosses Talia's stated role?

B. Restart it until we hear otherwise.

Restarting is a treatment instruction that Talia is not authorized to supply in this case.

Return to the exercise, page 35

F2. Which additional conclusion is unsupported?

D. The discrepancy proves an allergy.

No reaction or allergy details are supplied; a record discrepancy cannot establish either.

Return to the exercise, page 35

F / Guided speaking

1. dose

Assistant: Ellis reports an outside dose change.

Turn 1. The patient reports a dose change, not an established allergy.

2. unchanged

Nurse: The local record remains unchanged.

Turn 2. The local record has not changed in the supplied facts.

3. unavailable

Assistant: The outside instruction is unavailable.

Turn 3. The outside instruction is not available for verification here.

4. Priya

Nurse: The receiving owner is Priya.

Turn 4. Priya has explicitly accepted the query for appropriate review.

Return to the conversation, page 36

03 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Two sources, one unresolved entry". Ren: "I also do not have the exact strength or dose in front of me. I would rather say that than give you a number from memory." Talia: [missing response] Ren: "Can Dana speak with me about it? The list worries me because I still need to know which instruction I should follow." Which response belongs in the gap?

B. That is helpful. Strength and dose are different details, and neither should be guessed. We will preserve what is known and identify what requires verification.

A Not here. This line belongs to turn 20, after Ren says: "Then I understand the position: I reported an outside change, your list still includes the medicine, and Dana will review the difference with the clinician." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Talia's response in turn 14. It connects Ren's preceding point with Ren's follow-up. Strength describes active ingredient per product unit and is different from the amount prescribed for a dose.

C Not here. This line belongs to turn 6, after Ren says: "Then please do not remove it and say everything is sorted either. I want the right person to check what the other clinic actually intended." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 16, after Ren says: "Can Dana speak with me about it? The list worries me because I still need to know which instruction I should follow." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 36

04 / Answers and explanations

A / Read the situation

A1. Which event occurred at 09:20 according to the requisition?

D. Collection

The requisition explicitly labels 09:20 as the collection time.

Return to the exercise, page 37

A2. What does 10:05 describe?

D. Arrival on the courier sheet

The courier sheet records arrival, which is a different event from collection.

Return to the exercise, page 37

A3. What does the interval establish by itself?

A. A 45-minute difference between the recorded events

The time difference can be calculated, but no test-specific acceptability criteria or result is supplied.

Return to the exercise, page 37

D / Response choices

D1. Which combined entry preserves the facts?

D. Collection 09:20; arrival 10:05

The two original times remain attached to their stated events.

Return to the exercise, page 41

D2. Why should 09:20 not be replaced merely for consistency?

D. The replacement would change a collection event into an arrival time.

Matching the numbers would falsify the supplied event meaning rather than resolve a real discrepancy.

Return to the exercise, page 41

04 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Time an item reached the stated destination. Which term fits?

C. arrival time

A Not here. temperature condition: Required or recorded temperature circumstances. The usual combination is "document temperature conditions".

B Not here. acceptance criteria: Requirements for accepting a specimen or process. The usual combination is "check acceptance criteria".

C Correct. arrival time: Time an item reached the stated destination. The usual combination is "verify the arrival time".

D Not here. specimen source: Body site or material origin relevant to the test. The usual combination is "record the specimen source".

Return to the exercise, page 41

D4. Rebuild the exchange from "Two times, two events". Ben: "Let me read it back: collected at nine twenty; arrival at ten oh five. The difference is forty-five minutes, but each time has its own meaning." Imani: [missing response] Ben: "I was about to enter ten oh five in a field labeled received. I should check what that field means before assuming the courier entry is enough." Which response belongs in the gap?

A. That preserves the arrival time correctly. We also should not rename arrival as a later laboratory event unless the record actually defines it that way.

A Correct. This is Imani's response in turn 6. It connects Ben's preceding point with Ben's follow-up. The arrival time is 10:05 on the courier sheet; its meaning remains separate from collection and other laboratory events.

B Not here. This line belongs to turn 8, after Ben says: "I was about to enter ten oh five in a field labeled received. I should check what that field means before assuming the courier entry is enough." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 12, after Ben says: "A colleague called that a quick delivery and marked the specimen acceptable. We have the journey time, but have we checked the requirements for this test?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 2, after Ben says: "I have two different times on this paperwork. The requisition says 09:20, but the courier sheet says 10:05. Should I make them match?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 41

E / Conversation

1. event label

Turn 2. The event label identifies what happened at each time; different events need not have matching timestamps.

04 / Answers and explanations / continued

2. collection time

Turn 4. The collection time is 09:20 on the requisition and must not be replaced with the separate arrival time.

3. arrival time

Turn 6. The arrival time is 10:05 on the courier sheet; its meaning remains separate from collection and other laboratory events.

4. Receipt time

Turn 8. Receipt time refers to the receiving service's documented event and must not be inferred from an undefined arrival field.

5. specimen label

Turn 10. The specimen label supports identification; the two different event times do not themselves establish an identity mismatch.

6. transport interval

Turn 12. The transport interval is a calculated difference between defined events, not an independent judgment of specimen acceptability.

7. Acceptance criteria

Turn 14. Acceptance criteria are the applicable requirements for the specimen and test; they are not supplied by the time difference alone.

8. record amendment

Turn 16. A record amendment is a traceable correction through the approved process, not an unexplained overwrite to make fields look consistent.

9. Turnaround time

Turn 18. Turnaround time needs defined start and end events; the collection-to-arrival interval does not establish result availability.

10. requisition

Turn 20. The requisition remains the source of the collection entry, while the courier sheet supplies the distinct arrival entry.

Return to the conversation, page 43

04 / Answers and explanations / continued

F / Accuracy and scope

F1. Which conclusion is unsupported?

D. The specimen is acceptable because only 45 minutes elapsed.

No test-specific acceptance criteria or relevant handling facts support an acceptability decision.

Return to the exercise, page 45

F2. If a destination field is unclear, what is the next step?

A. Clarify the required event with the laboratory through the approved process.

The field meaning must be established before transferring a time into it.

Return to the exercise, page 45

F / Guided speaking

1. 08:40

Assistant: Collection was recorded at 08:40.

Turn 1. The collection record supplies 08:40 as its event time.

2. 09:15

Colleague: Arrival was recorded at 09:15.

Turn 2. The courier sheet supplies 09:15 as the arrival time.

3. distinct

Assistant: The event labels must remain distinct.

Turn 3. The times refer to separate events and should not be merged.

4. clarification

Colleague: The destination field needs clarification.

Turn 4. Its meaning is unclear and must be established before entry.

Return to the conversation, page 46

04 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Two times, two events". Ben: "Then I will avoid writing acceptable or rejected. Neither result follows just from the times we have read on these two documents." Imani: [missing response] Ben: "If the laboratory later confirms that a field genuinely was entered incorrectly, we can correct it through the proper process while keeping the history clear." Which response belongs in the gap?

B. Correct. Acceptance criteria belong to the real laboratory process. A language clarification about timestamps is not a quality decision.

A Not here. This line belongs to turn 20, after Ben says: "I will keep both source references, preserve 09:20 as collection and 10:05 as arrival, and clarify the receiving field before entering anything there." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Imani's response in turn 14. It connects Ben's preceding point with Ben's follow-up. Acceptance criteria are the applicable requirements for the specimen and test; they are not supplied by the time difference alone.

C Not here. This line belongs to turn 18, after Ben says: "I also should not tell the patient that these times predict when a result will be ready. Collection-to-arrival is not the entire testing process." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 4, after Ben says: "So replacing the earlier time would not simply clean up the paperwork. It would say the specimen was collected later than the requisition records." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 46

05 / Answers and explanations

A / Read the situation

A1. What happened Tuesday?

D. The specialist office received referral F8.

Tuesday is the recorded receipt event, not attendance or booking.

Return to the exercise, page 49

A2. What is Friday for?

A. Lia's status check

The only Friday commitment is Lia checking the referral status.

Return to the exercise, page 49

A3. What remains unconfirmed?

A. An appointment date

No appointment has been booked in the supplied facts.

Return to the exercise, page 49

D / Response choices

D1. Which update accurately uses all three days?

B. Sent Monday, received Tuesday, and Lia checks status Friday; no appointment is booked.

Each day is attached to its actual event, and the absence of a booking remains explicit.

Return to the exercise, page 53

D2. Which sentence creates a false expectation?

D. We will see you at the specialist on Friday.

Friday is only a coordinator checkpoint, not a supplied appointment.

Return to the exercise, page 53

05 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Practice or department providing care in a particular specialty. Which term fits?

C. specialist office

A Not here. referral criteria: Requirements used by a service to assess an appropriate referral. The usual combination is "clarify referral criteria".

B Not here. referral completion: Completion of the intended referral process, with its definition made clear. The usual combination is "document referral completion".

C Correct. specialist office: Practice or department providing care in a particular specialty. The usual combination is "check with the specialist office".

D Not here. booking confirmation: Verified notice that an appointment has been scheduled. The usual combination is "obtain booking confirmation".

Return to the exercise, page 53

D4. Rebuild the exchange from "Friday is a status check". Morgan: "Let me get this straight. It went out Monday and reached them Tuesday. What exactly is Lia doing on Friday if I am not booked in?" Omar: [missing response] Morgan: "Does under review mean the referral has been rejected, or that the office is still deciding what happens next?" Which response belongs in the gap?

C. The referral identifier is F8. It was sent Monday, received Tuesday, and Lia has agreed to check the status Friday.

A Not here. This line belongs to turn 12, after Morgan says: "I nearly asked for Friday off work. Please make the next message clear about whether it is an update or an actual appointment offer." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 8, after Morgan says: "Does under review mean the referral has been rejected, or that the office is still deciding what happens next?" It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Omar's response in turn 6. It connects Morgan's preceding point with Morgan's follow-up. The referral identifier F8 links the conversation to the specific request, preserving which case the three dates describe.

D Not here. This line belongs to turn 2, after Morgan says: "I heard the specialist received my referral on Tuesday, and someone mentioned Friday. What time should I arrive for the appointment?" It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 53

E / Conversation

1. follow-up checkpoint

Turn 2. The follow-up checkpoint is Lia's Friday status check, not a booked time for Morgan to attend the specialist.

05 / Answers and explanations / continued

2. receipt acknowledgment

Turn 4. Receipt acknowledgment confirms the referral reached the specialist office; it does not establish an appointment or completed clinical review.

3. referral identifier

Turn 6. The referral identifier F8 links the conversation to the specific request, preserving which case the three dates describe.

4. Under review

Turn 8. Under review describes the current assessment stage; it is not a supplied rejection or confirmed booking.

5. referral coordinator

Turn 10. The referral coordinator owns the administrative status check here, not the specialist's clinical decision or appointment availability.

6. booking confirmation

Turn 12. Booking confirmation would establish a scheduled appointment, which is specifically absent from the supplied referral status.

7. appointment offer

Turn 14. An appointment offer is a proposed time that still needs the relevant agreement and confirmation, not automatic attendance.

8. receiving service

Turn 16. The receiving service's acknowledgment concerns referral receipt and does not by itself verify the patient's insurance coverage.

9. clinical priority

Turn 18. Clinical priority requires the appropriate clinical assessment and cannot be inferred from a routine coordinator follow-up date.

10. care coordination

Turn 20. Care coordination keeps the referral status, owner, and next action connected without falsely reporting that the specialist visit is booked.

Return to the conversation, page 55

05 / Answers and explanations / continued

F / Accuracy and scope

F1. What does receipt establish?

C. The referral reached the receiving office, not that a slot is booked.

Receipt is an earlier administrative stage and does not prove later decisions or attendance.

Return to the exercise, page 57

F2. How should a new clinical concern be handled?

B. Through the clinic's appropriate clinical route rather than waiting for Lia's routine check

Administrative follow-up timing does not establish clinical safety or replace assessment.

Return to the exercise, page 57

F / Guided speaking

1. G3

Assistant: The referral identifier is G3.

Turn 1. G3 identifies the specific referral in this second scenario.

2. Wednesday

Patient: Receipt was confirmed Wednesday.

Turn 2. Wednesday is the receipt day, not an appointment date.

3. under review

Assistant: The status remains under review.

Turn 3. The receiving service has not supplied a booked appointment.

4. Monday

Patient: Noor will check on Monday.

Turn 4. Monday is the coordinator checkpoint rather than a confirmed visit.

Return to the conversation, page 58

05 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Friday is a status check". Morgan: "If they offer a time later, I may need to explain my work schedule and transport arrangements. I cannot assume every offered slot is possible." Omar: [missing response] Morgan: "I also wondered whether the referral being received means my plan has approved the visit. I do not want to confuse the two processes." Which response belongs in the gap?

C. An appointment offer should be handled through the booking process. Relevant access needs can be communicated appropriately rather than assuming an offered time has already been accepted.

A Not here. This line belongs to turn 4, after Morgan says: "I thought received meant they had accepted me into a slot. Is there still another step before I can arrange time away from work?" It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 18, after Morgan says: "What if the concern I was referred for changes before Friday? Should I simply wait because that is when Lia has promised to check?" It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Omar's response in turn 14. It connects Morgan's preceding point with Morgan's follow-up. An appointment offer is a proposed time that still needs the relevant agreement and confirmation, not automatic attendance.

D Not here. This line belongs to turn 10, after Morgan says: "Who is Lia in this process? I need to know whether she is the person checking the status or the clinician deciding about my care." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 58

06 / Answers and explanations

A / Read the situation

A1. Which document contains the unclear phrase?

B. The after-visit summary

The brief identifies the after-visit summary as the source.

Return to the exercise, page 59

A2. Who is available to explain the clinical instruction?

C. Dr Chen

Dr Chen is available while Mina cannot reinterpret the instruction.

Return to the exercise, page 59

A3. What does Jules request?

D. An explanation before leaving, away from reception

The patient requests timely clarification in an appropriate private setting.

Return to the exercise, page 59

D / Response choices

D1. Which response best preserves Jules's request?

A. Dr Chen is available; I will pass on this phrase and arrange the appropriate private explanation before you leave.

The response preserves the specific question, clinician route, timing, and privacy request.

Return to the exercise, page 64

D2. What does handing over the summary establish?

D. The document was provided, not that the patient understands every instruction.

Document delivery and understanding are distinct events requiring different evidence.

Return to the exercise, page 64

06 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Specialized word used within a professional field. Which term fits?

A. technical term

A Correct. technical term: Specialized word used within a professional field. The usual combination is "explain a technical term".

B Not here. qualified interpreter: Person with the relevant skills and qualifications to interpret accurately in context. The usual combination is "request a qualified interpreter".

C Not here. reinterpretation: Assigning a meaning that changes or newly determines the original instruction. The usual combination is "avoid unauthorized reinterpretation".

D Not here. understanding check: Method used to establish whether an explanation was understood. The usual combination is "conduct an understanding check".

Return to the exercise, page 64

D4. Rebuild the exchange from "Clarify before leaving". Jules: "I would prefer not to discuss the personal details across reception. There are several people waiting, and this is not something I want them to hear." Mina: [missing response] Jules: "Thank you. I sometimes nod because I recognize a term, then realize I do not understand how it applies to the instruction." Which response belongs in the gap?

D. We can arrange an appropriate private discussion. I will tell Dr Chen that you want clarification before leaving and that the setting matters to you.

A Not here. This line belongs to turn 2, after Jules says: "Mina, I have not left yet because one phrase on this paper is unclear. I can read the words, but I do not know what I am meant to do." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 10, after Jules says: "Could you show Dr Chen this line on the summary? It is this phrase, not the whole page, that I cannot make sense of." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 8, after Jules says: "Thank you. I sometimes nod because I recognize a term, then realize I do not understand how it applies to the instruction." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Mina's response in turn 6. It connects Jules's preceding point with Jules's follow-up. A private discussion addresses Jules's request to ask away from reception without announcing the personal details publicly.

Return to the exercise, page 64

E / Conversation

1. after-visit summary

Turn 2. The after-visit summary is the document containing the unclear phrase, so the handoff should preserve that source.

06 / Answers and explanations / continued

2. clinical instruction

Turn 4. A clinical instruction concerns care and requires the appropriate professional explanation; Mina cannot invent its meaning in this role.

3. private discussion

Turn 6. A private discussion addresses Jules's request to ask away from reception without announcing the personal details publicly.

4. technical term

Turn 8. A technical term may be familiar in appearance without being understood in its clinical context, which is why clarification remains necessary.

5. patient question

Turn 10. The patient question concerns a specific phrase and its practical meaning, not merely a nonspecific request for more information.

6. plain language

Turn 12. Plain language addresses understandable wording; louder repetition of the same unclear phrase does not resolve its meaning.

7. teach-back

Turn 14. Teach-back helps the professional check the explanation by hearing the patient's understanding, rather than accepting a yes answer as sufficient.

8. instruction source

Turn 16. The instruction source preserves the actual wording and context for Dr Chen instead of an altered summary created during the handoff.

9. revised plan

Turn 18. A revised plan would require an authorized clinical change, which has not occurred merely because Jules asked for clarification.

10. question handoff

Turn 20. The question handoff includes the document, specific phrase, privacy request, and need for clarification while Jules remains at the clinic.

Return to the conversation, page 66

06 / Answers and explanations / continued

F / Accuracy and scope

F1. Which statement misuses teach-back?

B. It is a test to prove the patient is intelligent enough.

Teach-back evaluates communication and understanding rather than judging the patient's intelligence.

Return to the exercise, page 68

F2. What must remain unchanged in Mina's handoff?

C. The original instruction and the substance of the patient's question

Preserving the source and question allows the clinician to clarify without distortion.

Return to the exercise, page 68

F / Guided speaking

1. abbreviation

Lee: The unclear item is an abbreviation.

Turn 1. The supplied question concerns an abbreviation in the visit summary.

2. leaving

Ari: I want clarification before leaving.

Turn 2. Ari is requesting explanation before leaving the clinic.

3. Malik

Lee: Dr Malik is available.

Turn 3. Dr Malik is the available clinician identified in this scenario.

4. private

Ari: I would like the explanation to be private.

Turn 4. The facts specify an appropriate private explanation with the clinician.

Return to the conversation, page 69

06 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Clarify before leaving". Jules: "After Dr Chen explains it, can I say back what I would actually do? I sometimes say yes too quickly when someone asks whether I understand." Mina: [missing response] Jules: "I also want the original wording available during the conversation. Otherwise someone might explain a shortened version and miss the part that confused me." Which response belongs in the gap?

A. Yes. That kind of teach-back can help the clinician identify what still needs clarification. It is not a test of your intelligence or a reason to feel embarrassed.

A Correct. This is Mina's response in turn 14. It connects Jules's preceding point with Jules's follow-up. Teach-back helps the professional check the explanation by hearing the patient's understanding, rather than accepting a yes answer as sufficient.

B Not here. This line belongs to turn 8, after Jules says: "Thank you. I sometimes nod because I recognize a term, then realize I do not understand how it applies to the instruction." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 2, after Jules says: "Mina, I have not left yet because one phrase on this paper is unclear. I can read the words, but I do not know what I am meant to do." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 20, after Jules says: "Please tell Dr Chen that I am still here and that I want to understand the instruction before going. I appreciate having a clear next step." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 69

07 / Answers and explanations

A / Read the situation

A1. What is Ellis asking Noor to decide?

B. Whether a new concern can safely wait until next week's visit

The caller seeks a clinical timing decision rather than an administrative fact.

Return to the exercise, page 72

A2. Who is available now?

A. Nurse Dana through the clinical-call route

Dana is specifically available now for the proper clinical route.

Return to the exercise, page 72

A3. What does the lack of supplied symptom details establish?

D. The exercise has not established urgency or safety to wait.

Missing clinical information cannot be turned into a reassuring assessment or a negative finding.

Return to the exercise, page 72

D / Response choices

D1. Which response addresses the caller's need appropriately?

D. I cannot assess whether waiting is safe; Dana is available now through our clinical-call route.

The response avoids an unsupported assessment and identifies the immediate clinical route.

Return to the exercise, page 76

D2. What does an unanswered transfer establish?

B. The clinical connection has not been completed.

An attempted transfer is not evidence of a received or completed clinical assessment.

Return to the exercise, page 76

07 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Scheduled visit not itself proof that a new concern can wait. Which term fits?

C. routine appointment

A Not here. identity check: Process of confirming who is communicating. The usual combination is "complete the identity check".

B Not here. reassurance: Statement intended to reduce concern, which must not exceed evidence or authority. The usual combination is "avoid unsupported reassurance".

C Correct. routine appointment: Scheduled visit not itself proof that a new concern can wait. The usual combination is "distinguish a routine appointment".

D Not here. receiving clinician: Clinical professional accepting the concern or handoff. The usual combination is "identify the receiving clinician".

Return to the exercise, page 76

D4. Rebuild the exchange from "After the clinical connection". Noor: "Ellis already had a booking next week. I included that as background, but I did not say the booking established a safe waiting period." Dana: [missing response] Noor: "My note currently says passed to nursing. That leaves out who received the call. I should name you and record the actual connection." Which response belongs in the gap?

C. Correct. A routine appointment does not assess a new concern. Do not let the appointment date become the only information the next person receives.

A Not here. This line belongs to turn 20, after Noor says: "I will record your confirmed receipt and my factual introduction. I will not add a diagnosis, waiting advice, or clinical outcome that you have not provided." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 14, after Noor says: "For a future call, if the line disconnects before anyone confirms receipt, I cannot use today's completed status as the default." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Dana's response in turn 6. It connects Noor's preceding point with Noor's follow-up. An existing routine booking does not assess a new concern or establish a safe waiting period.

D Not here. This line belongs to turn 2, after Noor says: "Dana, thanks for taking Ellis's call immediately. I am checking my handoff note now, after the connection. Ellis asked whether a new concern could wait until next week." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 76

E / Conversation

1. urgency assessment

Turn 2. Urgency assessment is a clinical judgment, not a decision Noor made when connecting the call.

07 / Answers and explanations / continued

2. triage

Turn 4. Triage addresses urgency and care needs; asking whether a concern can wait requires more than a booking check.

3. routine appointment

Turn 6. An existing routine booking does not assess a new concern or establish a safe waiting period.

4. clinical-call route

Turn 8. This is the established pathway used to connect Ellis with Dana; its actual outcome must be recorded.

5. receiving clinician

Turn 10. Dana confirmed receipt. Naming her does not authorize Noor to invent a clinical conclusion on her behalf.

6. warm transfer

Turn 12. Noor introduced the concern and confirmed Dana received the call, rather than merely attempting an unconfirmed transfer.

7. dropped call

Turn 14. A dropped call needs the actual continuity process; interruption does not establish successful clinical contact.

8. callback queue

Turn 16. A callback queue holds requests awaiting contact, not evidence that a clinical conversation or assessment occurred.

9. emergency pathway

Turn 18. Applicable emergency procedures take priority over administrative work or this later language-practice review.

10. escalation outcome

Turn 20. Dana's receipt is confirmed; no diagnosis, completed assessment, or waiting advice is supplied here.

Return to the conversation, page 78

07 / Answers and explanations / continued

F / Accuracy and scope

F1. Which information must not be invented for a structured handoff?

A. A diagnosis or urgency decision outside Noor's role

A framework organizes real information; it does not authorize fabricated clinical judgments.

Return to the exercise, page 80

F2. What takes precedence when applicable?

C. The actual emergency pathway

Emergency procedures must not be delayed by a routine booking or language exercise.

Return to the exercise, page 80

F / Guided speaking

1. confirmed

Lee: Receipt has not been confirmed.

Turn 1. The connection ended before anyone confirmed clinical receipt of the concern.

2. dropped-call

Colleague: Use the clinic's dropped-call process.

Turn 2. The scenario specifies the established process for interrupted contact.

3. Priya

Lee: The intended recipient is nurse Priya.

Turn 3. Priya is the named clinical recipient for this transfer.

4. assessment

Colleague: No clinical assessment has occurred.

Turn 4. The facts explicitly state that clinical assessment has not occurred.

Return to the conversation, page 81

07 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "After the clinical connection". Noor: "For a future call, if the line disconnects before anyone confirms receipt, I cannot use today's completed status as the default." Dana: [missing response] Noor: "And if I only put a request into a message queue, the note should still show pending contact, not that the patient spoke to a nurse." Which response belongs in the gap?

B. Exactly. Follow the clinic's dropped call process. Verified contact details and the actual unresolved concern matter; do not silently leave a failed connection marked complete.

A Not here. This line belongs to turn 2, after Noor says: "Dana, thanks for taking Ellis's call immediately. I am checking my handoff note now, after the connection. Ellis asked whether a new concern could wait until next week." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Dana's response in turn 14. It connects Noor's preceding point with Noor's follow-up. A dropped call needs the actual continuity process; interruption does not establish successful clinical contact.

C Not here. This line belongs to turn 6, after Noor says: "Ellis already had a booking next week. I included that as background, but I did not say the booking established a safe waiting period." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 20, after Noor says: "I will record your confirmed receipt and my factual introduction. I will not add a diagnosis, waiting advice, or clinical outcome that you have not provided." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 81

08 / Answers and explanations

A / Read the situation

A1. What does the verified schedule show?

C. Thursday at 14:00 with Dr Chen

The schedule confirms Thursday at 14:00 with Dr Chen.

Return to the exercise, page 82

A2. What caused Robin to rearrange work?

B. Elena's inaccurate Tuesday message

The case identifies the incorrect message as the reason for the work disruption.

Return to the exercise, page 82

A3. Which topic is not supplied for discussion?

B. The actual test results

No result values or clinical interpretations are part of this exercise.

Return to the exercise, page 82

D / Response choices

D1. Which correction preserves the history?

D. My Tuesday message was wrong; the verified booking is Thursday at 14:00 with Dr Chen.

The original error was in the message, while the schedule supplies the verified Thursday booking.

Return to the exercise, page 86

D2. Which conclusion about results is unsupported?

B. Thursday means the results are definitely normal.

Scheduling details do not establish the content or clinical significance of test results.

Return to the exercise, page 86

08 / Answers and explanations / continued

D3. A colleague needs the term for this meaning: Communication replacing an inaccurate earlier message. Which term fits?

B. message correction

A Not here. outstanding action: Task that remains to be completed. The usual combination is "track the outstanding action".

B Correct. message correction: Communication replacing an inaccurate earlier message. The usual combination is "issue a message correction".

C Not here. audit trail: Record showing the sequence and authorship of events or changes. The usual combination is "maintain an audit trail".

D Not here. twenty-four-hour time: Time notation running from 00:00 through 23:59. The usual combination is "use twenty-four-hour time".

Return to the exercise, page 86

D4. Rebuild the exchange from "Thursday, not Tuesday". Robin: "I have the Tuesday message right here. Was that date wrong from the start, or did someone move my appointment without telling me?" Elena: [missing response] Robin: "Fourteen hundred means two in the afternoon, correct? I want to check the time as well as the day before I speak to my manager." Which response belongs in the gap?

A. You are right. This is not rescheduling. The earlier message was wrong; I should not describe that as moving an appointment that was booked for Tuesday.

A Correct. This is Elena's response in turn 6. It connects Robin's preceding point with Robin's follow-up. Rescheduling changes an actual appointment; here the booking remained Thursday and only the Tuesday message was wrong.

B Not here. This line belongs to turn 14, after Robin says: "Will the earlier message remain visible appropriately? Otherwise a later colleague may think I simply chose the wrong day when I arranged work." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 10, after Robin says: "Thursday at two in the afternoon with Dr Chen. Does the later day tell me anything about whether the results are normal or serious?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 4, after Robin says: "That matters because I changed my work shift after reading what you sent. I need to know whether Thursday is confirmed or just another possible date." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 86

E / Conversation

1. message correction

Turn 2. The message correction replaces Elena's inaccurate Tuesday notification; the actual booking was not changed from Tuesday to Thursday.

08 / Answers and explanations / continued

2. schedule of record

Turn 4. The schedule of record is the verified source showing Thursday at 14:00 with Dr Chen, rather than another tentative message.

3. rescheduling

Turn 6. Rescheduling changes an actual appointment; here the booking remained Thursday and only the Tuesday message was wrong.

4. twenty-four-hour time

Turn 8. Twenty-four-hour time expresses two in the afternoon as 14:00, which is the verified booking time in this case.

5. results discussion

Turn 10. A results discussion identifies the purpose of the appointment and does not reveal the findings or their clinical significance.

6. result interpretation

Turn 12. Result interpretation is a clinical explanation that has not occurred in this conversation, which concerns only the booking correction.

7. audit trail

Turn 14. The audit trail preserves the inaccurate original message and its correction, so the sequence is not rewritten as patient error.

8. successful contact

Turn 16. Successful contact means Robin actually received the corrected details; it is different from a call attempt or unsent message.

9. follow-up owner

Turn 18. A follow-up owner remains responsible for unfinished communication; an unsuccessful contact attempt must not automatically close the task.

10. escalation route

Turn 20. The escalation route provides a real process for the unresolved service concern without inventing an unauthorized remedy or compensation promise.

Return to the conversation, page 88

08 / Answers and explanations / continued

F / Accuracy and scope

F1. What should be recorded if Robin is not reached?

A. The contact attempt and the still-outstanding correction

An unsuccessful attempt does not prove that the patient received or acknowledged the corrected details.

Return to the exercise, page 90

F2. Which apology is strongest?

D. I am sorry my incorrect Tuesday message disrupted your work arrangements.

The strongest apology identifies the actual message error and work disruption without shifting blame or inventing a schedule change.

Return to the exercise, page 90

F / Guided speaking

1. 09:00

Noor: My message incorrectly said 09:00.

Turn 1. The error was the 09:00 time in the message.

2. 11:30

Ari: The verified time is 11:30.

Turn 2. The schedule confirms 11:30 rather than the earlier message time.

3. Shah

Noor: The clinician is Dr Shah.

Turn 3. Dr Shah is the clinician named in the verified booking.

4. rescheduling

Ari: This is a message correction, not rescheduling.

Turn 4. The appointment itself did not change; only the communication was corrected.

Return to the conversation, page 91

08 / Answers and explanations / continued

F / Rebuild the exchange

1. Rebuild the exchange from "Thursday, not Tuesday". Robin: "Will the earlier message remain visible appropriately? Otherwise a later colleague may think I simply chose the wrong day when I arranged work." Elena: [missing response] Robin: "And will you record that you actually reached me today? I would like the team to know I received and repeated back the Thursday information." Which response belongs in the gap?

B. We need the appropriate audit trail. I will preserve the earlier event and the correction through our process rather than erase the reason for your confusion.

A Not here. This line belongs to turn 6, after Robin says: "I have the Tuesday message right here. Was that date wrong from the start, or did someone move my appointment without telling me?" It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Elena's response in turn 14. It connects Robin's preceding point with Robin's follow-up. The audit trail preserves the inaccurate original message and its correction, so the sequence is not rewritten as patient error.

C Not here. This line belongs to turn 10, after Robin says: "Thursday at two in the afternoon with Dr Chen. Does the later day tell me anything about whether the results are normal or serious?" It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 4, after Robin says: "That matters because I changed my work shift after reading what you sent. I need to know whether Thursday is confirmed or just another possible date." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 91

Review 1 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Periodic review visit recorded as occurring yearly. Which term fits?

A. annual review

A Correct. annual review: Periodic review visit recorded as occurring yearly. The usual combination is "attend an annual review".

B Not here. scheduled purpose: Reason for a visit as recorded in the booking. The usual combination is "state the scheduled purpose".

C Not here. clinical handoff: Transfer of relevant information to a receiving clinical professional. The usual combination is "complete a clinical handoff".

D Not here. recorded name: Name shown in the relevant patient record. The usual combination is "verify the recorded name".

Return to the exercise, page 25

2. A colleague needs the term for this meaning: Observed or measured finding, with its source and context. Which term fits?

D. objective finding

A Not here. vital signs: Physiological measurements such as pulse and temperature. The usual combination is "record measured vital signs".

B Not here. unit of measurement: Standard used to express a measured quantity. The usual combination is "include the unit of measurement".

C Not here. rooming: Preparing a patient for the clinical encounter under the clinic process. The usual combination is "complete the rooming process".

D Correct. objective finding: Observed or measured finding, with its source and context. The usual combination is "document an objective finding".

Return to the exercise, page 25

Review 1 / Explanations / continued

3. Rebuild the exchange from "The schedule and the expectation". Alex: "Will you tell Dana that I expected this to be the concern visit, rather than just saying that I am unhappy about the reminder?" Sam: [missing response] Alex: "Is this a billing problem too? I have heard different visit categories can mean different charges, and I do not want a surprise." Which response belongs in the gap?

D. I will preserve that patient expectation. The handoff will say the schedule lists an annual review and you expected a discussion of a recent concern.

A Not here. This line belongs to turn 12, after Alex says: "Is this a billing problem too? I have heard different visit categories can mean different charges, and I do not want a surprise." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 14, after Alex says: "Please check before anyone promises it is covered. I took time off for this visit, and I need to understand what will actually happen today." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 18, after Alex says: "Thank you. I do not need you to change the appointment yourself; I need someone qualified to clarify what today's visit can include." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Sam's response in turn 10. It connects Alex's preceding point with Alex's follow-up. Patient expectation describes what Alex believed the visit would address, not merely dissatisfaction with an administrative reminder.

Return to the exercise, page 25

Review 1 / Explanations / continued

4. Rebuild the exchange from "Report, observation, assessment". Rosa: "Have you received those points? I do not want the phrase answering questions to be passed on as a formal severity rating later." Dana: [missing response] Rosa: "I will record the report accordingly and preserve what I actually observed. I will not add a measurement, diagnosis, or assurance about waiting." Which response belongs in the gap?

B. I have received the report and its limits. Clinical assessment is separate, and the concern will follow our clinical process rather than an administrative assumption.

A Not here. This line belongs to turn 10, after Rosa says: "Should I write that Pat denied other symptoms? None are included in the information I am handing over, but I have not established their absence." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Dana's response in turn 18. It connects Rosa's preceding point with Rosa's follow-up. Clinical assessment is the qualified evaluation still needed; it must not be inferred from Rosa's limited rooming observation.

C Not here. This line belongs to turn 12, after Rosa says: "I will keep the original wording in quotation marks when I use it exactly. If I summarize, I will still identify Pat as the source." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 6, after Rosa says: "The handoff draft says looks fine. I will replace that with what I saw: Pat was seated and answering my questions." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 25

Review 2 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Medicine record held by the current organization. Which term fits?

B. local medication list

A Not here. historical entry: Record of earlier information rather than necessarily current treatment. The usual combination is "distinguish a historical entry".

B Correct. local medication list: Medicine record held by the current organization. The usual combination is "review the local medication list".

C Not here. dietary supplement: Product used to supplement dietary intake, subject to relevant regulation. The usual combination is "ask about dietary supplements".

D Not here. strength: Amount of active ingredient in a specified unit of product. The usual combination is "distinguish strength from dose".

Return to the exercise, page 47

2. A colleague needs the term for this meaning: Information attached to the specimen container for identification. Which term fits?

D. specimen label

A Not here. insufficient quantity: Amount below what is required for the intended test. The usual combination is "report insufficient quantity".

B Not here. requisition: Request document specifying laboratory testing and relevant information. The usual combination is "check the requisition".

C Not here. analytical phase: Stage in which the laboratory examination or measurement is performed. The usual combination is "distinguish the analytical phase".

D Correct. specimen label: Information attached to the specimen container for identification. The usual combination is "verify the specimen label".

Return to the exercise, page 47

Review 2 / Explanations / continued

3. Rebuild the exchange from "Report, observation, assessment". Rosa: "Should I write that Pat denied other symptoms? None are included in the information I am handing over, but I have not established their absence." Dana: [missing response] Rosa: "I will keep the original wording in quotation marks when I use it exactly. If I summarize, I will still identify Pat as the source." Which response belongs in the gap?

C. No. A pertinent negative must come from the relevant question or assessment. Lack of a statement in this report is not a denial.

A Not here. This line belongs to turn 16, after Rosa says: "The handoff can be short: Pat reports tiredness since Monday; observed answering questions while seated; cause and severity not assessed by me." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 6, after Rosa says: "The handoff draft says looks fine. I will replace that with what I saw: Pat was seated and answering my questions." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Dana's response in turn 10. It connects Rosa's preceding point with Rosa's follow-up. A pertinent negative is an established relevant absence, not a conclusion drawn from an unmentioned symptom.

D Not here. This line belongs to turn 14, after Rosa says: "I also cannot say this is a change from Pat's normal condition unless I have an established usual state to compare it with." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 47

Review 2 / Explanations / continued

4. Rebuild the exchange from "Two times, two events". Ben: "I also should not tell the patient that these times predict when a result will be ready. Collection-to-arrival is not the entire testing process." Imani: [missing response] Ben: "I will keep both source references, preserve 09:20 as collection and 10:05 as arrival, and clarify the receiving field before entering anything there." Which response belongs in the gap?

D. Exactly. Turnaround time must have defined start and end points. These entries do not supply an analysis completion or result-release time.

A Not here. This line belongs to turn 20, after Ben says: "I will keep both source references, preserve 09:20 as collection and 10:05 as arrival, and clarify the receiving field before entering anything there." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 4, after Ben says: "So replacing the earlier time would not simply clean up the paperwork. It would say the specimen was collected later than the requisition records." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 14, after Ben says: "Then I will avoid writing acceptable or rejected. Neither result follows just from the times we have read on these two documents." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Imani's response in turn 18. It connects Ben's preceding point with Ben's follow-up. Turnaround time needs defined start and end events; the collection-to-arrival interval does not establish result availability.

Return to the exercise, page 47

Review 3 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Organization or team to which a referral is sent. Which term fits?

D. receiving service

A Not here. supporting documentation: Information accompanying a request to support its assessment. The usual combination is "supply supporting documentation".

B Not here. attendance: Actual presence at an appointment or service. The usual combination is "confirm attendance".

C Not here. under review: Being assessed through a process before a further decision or action. The usual combination is "explain under review".

D Correct. receiving service: Organization or team to which a referral is sent. The usual combination is "contact the receiving service".

Return to the exercise, page 70

2. A colleague needs the term for this meaning: Explanation that makes meaning more definite or understandable. Which term fits?

B. clarification

A Not here. health literacy: Ability to find, understand, and use health information and services. The usual combination is "support health literacy".

B Correct. clarification: Explanation that makes meaning more definite or understandable. The usual combination is "request clarification".

C Not here. after-visit summary: Document summarizing relevant information and instructions after a clinical encounter. The usual combination is "review the after-visit summary".

D Not here. private discussion: Conversation conducted with appropriate protection from unnecessary hearing. The usual combination is "arrange a private discussion".

Return to the exercise, page 70

Review 3 / Explanations / continued

3. Rebuild the exchange from "Two times, two events". Ben: "The patient details match in these records. I was only looking at the two times; I do not have evidence of a labeling problem." Imani: [missing response] Ben: "A colleague called that a quick delivery and marked the specimen acceptable. We have the journey time, but have we checked the requirements for this test?" Which response belongs in the gap?

D. That would be a separate specimen label or identification question. No identity discrepancy is established here, so we should not turn an event-time difference into a patient mismatch.

A Not here. This line belongs to turn 8, after Ben says: "I was about to enter ten oh five in a field labeled received. I should check what that field means before assuming the courier entry is enough." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 2, after Ben says: "I have two different times on this paperwork. The requisition says 09:20, but the courier sheet says 10:05. Should I make them match?" It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 6, after Ben says: "Let me read it back: collected at nine twenty; arrival at ten oh five. The difference is forty-five minutes, but each time has its own meaning." It addresses that point, not the missing turn in the supplied exchange.

D Correct. This is Imani's response in turn 10. It connects Ben's preceding point with Ben's follow-up. The specimen label supports identification; the two different event times do not themselves establish an identity mismatch.

Return to the exercise, page 70

Review 3 / Explanations / continued

4. Rebuild the exchange from "Clarify before leaving". Jules: "So there is no new instruction from this conversation yet. The next step is for Dr Chen to clarify the existing phrase privately before I leave." Mina: [missing response] Jules: "Please tell Dr Chen that I am still here and that I want to understand the instruction before going. I appreciate having a clear next step." Which response belongs in the gap?

A. Correct. No revised plan has been established here. I am arranging the explanation, not changing the clinical direction.

A Correct. This is Mina's response in turn 18. It connects Jules's preceding point with Jules's follow-up. A revised plan would require an authorized clinical change, which has not occurred merely because Jules asked for clarification.

B Not here. This line belongs to turn 4, after Jules says: "Could you just explain what it means? I know everyone is busy, but I do not want to go home and make the wrong assumption." It addresses that point, not the missing turn in the supplied exchange.

C Not here. This line belongs to turn 16, after Jules says: "I also want the original wording available during the conversation. Otherwise someone might explain a shortened version and miss the part that confused me." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 12, after Jules says: "I would like a plain explanation, not just the same phrase repeated more loudly. I heard it; the meaning is what I need." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 70

Review 4 / Explanations

Cumulative review

1. A colleague needs the term for this meaning: Evaluation of how promptly a concern requires attention. Which term fits?

A. urgency assessment

A Correct. urgency assessment: Evaluation of how promptly a concern requires attention. The usual combination is "obtain an urgency assessment".

B Not here. verified callback number: Return-contact number checked through the appropriate procedure. The usual combination is "obtain a verified callback number".

C Not here. symptom progression: How a symptom has changed over time. The usual combination is "describe symptom progression".

D Not here. blind transfer: Transfer without a direct introduction or confirmed contextual handoff. The usual combination is "avoid an unsupported blind transfer".

Return to the exercise, page 92

2. A colleague needs the term for this meaning: Checking the actual calendar date associated with a booking. Which term fits?

B. date verification

A Not here. original entry: Earlier record item retained under the applicable process. The usual combination is "preserve the original entry".

B Correct. date verification: Checking the actual calendar date associated with a booking. The usual combination is "complete date verification".

C Not here. acknowledgment: Confirmation of receipt or recognition of information. The usual combination is "record acknowledgment".

D Not here. appointment confirmation: Verified communication of a scheduled visit and its details. The usual combination is "send an appointment confirmation".

Return to the exercise, page 92

Review 4 / Explanations / continued

3. Rebuild the exchange from "Clarify before leaving". Jules: "Could you show Dr Chen this line on the summary? It is this phrase, not the whole page, that I cannot make sense of." Mina: [missing response] Jules: "I would like a plain explanation, not just the same phrase repeated more loudly. I heard it; the meaning is what I need." Which response belongs in the gap?

B. I will preserve the patient question with the wording. That gives Dr Chen the actual issue rather than a vague message that could miss it.

A Not here. This line belongs to turn 2, after Jules says: "Mina, I have not left yet because one phrase on this paper is unclear. I can read the words, but I do not know what I am meant to do." It addresses that point, not the missing turn in the supplied exchange.

B Correct. This is Mina's response in turn 10. It connects Jules's preceding point with Jules's follow-up. The patient question concerns a specific phrase and its practical meaning, not merely a nonspecific request for more information.

C Not here. This line belongs to turn 8, after Jules says: "Thank you. I sometimes nod because I recognize a term, then realize I do not understand how it applies to the instruction." It addresses that point, not the missing turn in the supplied exchange.

D Not here. This line belongs to turn 6, after Jules says: "I would prefer not to discuss the personal details across reception. There are several people waiting, and this is not something I want them to hear." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 92

Review 4 / Explanations / continued

4. Rebuild the exchange from "Thursday, not Tuesday". Robin: "Please send the corrected details through the clinic's usual channel too. I need something accurate to refer to when I rearrange my shift." Elena: [missing response] Robin: "Thank you for acknowledging the work disruption. I may still raise that concern through the clinic process, but I now have the confirmed appointment details." Which response belongs in the gap?

C. Exactly. A named follow-up owner would need to keep that action visible. I own this correction and will record what has actually been communicated.

A Not here. This line belongs to turn 8, after Robin says: "Fourteen hundred means two in the afternoon, correct? I want to check the time as well as the day before I speak to my manager." It addresses that point, not the missing turn in the supplied exchange.

B Not here. This line belongs to turn 12, after Robin says: "I understand. Please do not let my question about the date become a note saying that someone already explained the results to me." It addresses that point, not the missing turn in the supplied exchange.

C Correct. This is Elena's response in turn 18. It connects Robin's preceding point with Robin's follow-up. A follow-up owner remains responsible for unfinished communication; an unsuccessful contact attempt must not automatically close the task.

D Not here. This line belongs to turn 4, after Robin says: "That matters because I changed my work shift after reading what you sent. I need to know whether Thursday is confirmed or just another possible date." It addresses that point, not the missing turn in the supplied exchange.

Return to the exercise, page 92

Additional case 1 / Explanations

Extended conversation

1. spoken-language preference

Turn 2. Mei stated Mandarin for spoken communication; Cantonese in the copied appointment note conflicts with that confirmed preference.

2. written-language preference

Turn 4. Traditional Chinese is Mei's stated choice for written materials; it should not be inferred from the spoken-language field or replaced by it.

3. qualified interpreter

Turn 6. The clinic has arranged a qualified healthcare interpreter for Mandarin; a relative's presence for support does not replace that booking.

4. video interpreting

Turn 10. The booked service uses video for the live spoken exchange; it is not a written translation service or merely a telephone appointment reminder.

5. short segments

Turn 14. Short segments let the interpreter relay each part accurately and allow the patient to ask questions without relying on a long end-of-explanation summary.

6. teach-back

Turn 18. Teach-back checks the clarity of an explanation by hearing the patient's understanding in their own words; a yes response or fluent English is not the test.

Return to the conversation, page 95

Transfer exchange

1. Spanish

Assistant: Luis's spoken preference is Spanish .

Turn 1. Luis explicitly chose Spanish for conversation; the English paperwork preference does not override it.

Additional case 1 / Explanations / continued

2. English

Coordinator: His written preference is English .

Turn 2. The supplied record separately confirms English for written materials, rather than Spanish by assumption.

3. telephone

Assistant: The booked interpreter will join by telephone .

Turn 3. The booked service is telephone interpreting, not video or an already completed connection.

4. support

Coordinator: The son's attendance is for support .

Turn 4. The son is attending for support; his presence does not replace the arranged qualified interpreter.

Return to the conversation, page 98

Additional case 2 / Explanations

Extended conversation

1. source measurement

Turn 2. The saved scale output establishes 160.0 pounds at 09:12; appearance or the patient's previous weight would not establish this measurement's unit.

2. 72.6

Turn 4. 160.0 multiplied by 0.45359237 equals 72.5747792 kilograms, which rounds to 72.6 kilograms at one decimal place.

3. unit conversion

Turn 6. A unit conversion changes the numerical value and unit together while representing the same measured quantity; relabeling 160.0 pounds as kilograms changes its meaning.

4. height

Turn 8. The separate 170-centimeter entry describes height; its conversion to 1.70 meters is unrelated to converting the weight from pounds to kilograms.

5. audit trail

Turn 12. The audit trail preserves the original entry, correction, reason, and authorship under the clinic process; an unexplained overwrite would hide the recording error.

6. measurement time

Turn 16. The measurement occurred at 09:12; the later correction time is a separate record event and must not make the conversion look like a new weighing.

Return to the conversation, page 100

Transfer exchange

1. 59.9

Assistant: The weight rounds to 59.9 kilograms.

Turn 1. 132.0 multiplied by 0.45359237 is 59.87419284, which rounds to 59.9 kilograms.

Additional case 2 / Explanations / continued

2. 10:18

Nurse: The original measurement time remains 10:18 .

Turn 2. The patient was weighed at 10:18; 10:31 describes the later correction, not a new weighing.

3. 1.65

Assistant: The separate height is 1.65 meters.

Turn 3. There are one hundred centimeters in a meter, so 165 centimeters equals 1.65 meters.

4. 10:31

Nurse: The record was corrected at 10:31 .

Turn 4. The supplied correction time is 10:31 and must remain distinct from the measurement time.

Return to the conversation, page 103

Additional case 3 / Explanations

Extended conversation

1. lead-off warning

Turn 2. A lead-off warning concerns the recording connection; it is not itself a diagnosis about the patient's heart or proof that all clinical findings are artifact.

2. electrodes

Turn 6. The ten skin electrodes acquire the signals used for a standard twelve-lead ECG; the number of electrodes is not the number of displayed electrical views.

3. Signal quality

Turn 10. Signal quality concerns the recording's clarity and interference; it is distinct from whether the patient's ECG has a clinically important finding.

4. Automated interpretation

Turn 12. Automated interpretation is the device's analysis, not a completed clinician review; its wording must not be presented as Dr Shah's confirmed conclusion.

5. repeat acquisition

Turn 16. Repeat acquisition means obtaining a new recording at 11:10; it does not erase the original 11:06 trace or establish a clinical change between them.

6. reviewing clinician

Turn 18. The reviewing clinician has received the material but must still review it; receipt alone does not establish interpretation or a discharge decision.

Return to the conversation, page 105

Transfer exchange

1. 10

Assistant: This standard twelve-lead recording uses 10 skin electrodes.

Turn 1. The standard twelve-lead ECG uses ten electrodes to obtain twelve electrical views.

Additional case 3 / Explanations / continued

2. 14:02

Clinician: The original trace is timed 14:02 .

Turn 2. The first recording retains 14:02 even though a later repeat was obtained.

3. 14:07

Assistant: The repeat is timed 14:07 .

Turn 3. The repeat is a separate acquisition at 14:07, not a correction to the first trace's timestamp.

4. supplied

Clinician: A clinical interpretation is not yet supplied .

Turn 4. Receipt of the traces is confirmed, but the facts do not supply a clinical interpretation or clearance.

Return to the conversation, page 108

Find a phrase by purpose

Clarifying the visit at check-in

Use the preferred name: Thank you, Alex; I will use that name.

State the schedule: The schedule lists an annual review.

Acknowledge the expectation: You expected to discuss a recent concern.

Full phrase bank, page 7

Rooming and factual patient reports

Attribute the words: Pat reports feeling tired since Monday.

Quote accurately: Pat said, "I have felt tired since Monday."

Describe the observation: I observed Pat answering questions while seated.

Full phrase bank, page 18

Medication histories with clear sources

Acknowledge the report: You report that another clinic stopped this medicine.

State the local source: The local medication list still includes it.

Name the discrepancy: Those two sources need to be reconciled.

Full phrase bank, page 30

Laboratory paperwork and specimen queries

Identify the first field: The requisition says collection at 09:20.

Identify the second field: The courier sheet says arrival at 10:05.

Name the distinction: Those times describe different events.

Full phrase bank, page 40

Find a phrase by purpose / continued

Referrals and appointment dependencies

Identify the referral: We are discussing referral F8.

State transmission: It was sent Monday.

State receipt: The specialist office received it Tuesday.

Full phrase bank, page 52

Supporting questions during a clinical visit

Welcome the question: Thank you for telling me which part is unclear.

Identify the document: The phrase is in your after-visit summary.

Preserve the wording: I will keep the original wording with your question.

Full phrase bank, page 63

Telephone concerns and clinical escalation

Acknowledge the question: You want to know whether this can wait until next week.

State the limit briefly: I cannot assess urgency or tell you it is safe to wait.

Offer the immediate route: Nurse Dana is available through our clinical-call route now.

Full phrase bank, page 75

Records, corrections, and follow-up ownership

Open the correction: I need to correct the appointment message I sent you.

Name the error: I wrote Tuesday, but that was incorrect.

Give the verified booking: The schedule confirms Thursday at 14:00 with Dr Chen.

Full phrase bank, page 85

Vocabulary index

72.6
Case 2, p. 99

abbreviation
Lesson 06, p. 60

acceptance criteria
Lesson 04, p. 39

access barrier
Lesson 05, p. 51

accessible format
Lesson 06, p. 61

accession number
Lesson 04, p. 38

acknowledgment
Lesson 08, p. 84

active entry
Lesson 03, p. 28

addendum
Lesson 08, p. 84

adherence
Lesson 03, p. 29

adverse reaction
Lesson 03, p. 29

after-visit summary
Lesson 06, p. 60

allergy history
Lesson 03, p. 29

ambiguity
Lesson 06, p. 60

analytical phase
Lesson 04, p. 39

annual review
Lesson 01, p. 5

appointment availability
Lesson 05, p. 50

appointment confirmation
Lesson 08, p. 83

appointment offer
Lesson 05, p. 51

appointment slot
Lesson 01, p. 6

appointment type
Lesson 01, p. 5

arrival time
Lesson 04, p. 38

as-needed instruction
Lesson 03, p. 29

associated symptom
Lesson 02, p. 15

attendance
Lesson 05, p. 51

audit trail
Lesson 08, p. 84, Case 2, p. 99

Automated interpretation
Case 3, p. 104

baseline
Lesson 02, p. 15

billing category
Lesson 01, p. 6

blind transfer
Lesson 07, p. 73

blood pressure
Lesson 02, p. 16

booking confirmation
Lesson 05, p. 50

call disposition
Lesson 07, p. 73

callback commitment
Lesson 07, p. 73

callback queue
Lesson 07, p. 73

cancellation slot
Lesson 05, p. 51

care coordination
Lesson 05, p. 51

change from baseline
Lesson 02, p. 15

check-in
Lesson 01, p. 5

chief complaint
Lesson 01, p. 5

chunk and check
Lesson 06, p. 60

clarification
Lesson 06, p. 60

clinical assessment
Lesson 02, p. 16

clinical escalation
Lesson 07, p. 73

Vocabulary index / continued

clinical handoff
Lesson 01, p. 6

clinical instruction
Lesson 06, p. 60

clinical priority
Lesson 05, p. 51

clinical suitability
Lesson 01, p. 6

clinical-call route
Lesson 07, p. 73

closed-loop communication
Lesson 07, p. 74

closure evidence
Lesson 08, p. 84

collection method
Lesson 04, p. 38

collection time
Lesson 04, p. 38

communication log
Lesson 08, p. 84

consultation report
Lesson 05, p. 51

contact attempt
Lesson 08, p. 84

correction timestamp
Lesson 08, p. 83

courier sheet
Lesson 04, p. 38

date verification
Lesson 08, p. 83

delegation
Lesson 01, p. 6

demographic details
Lesson 01, p. 5

dependency
Lesson 05, p. 51

dietary supplement
Lesson 03, p. 29

discontinuation
Lesson 03, p. 28

discrepancy
Lesson 03, p. 28

dosage form
Lesson 03, p. 29

dose
Lesson 03, p. 29

dropped call
Lesson 07, p. 74

duplicate entry
Lesson 03, p. 28

duration
Lesson 02, p. 15

electrodes
Case 3, p. 104

emergency pathway
Lesson 07, p. 74

encounter
Lesson 01, p. 5

escalation outcome
Lesson 07, p. 74

escalation route
Lesson 08, p. 84

event label
Lesson 04, p. 39

follow-up checkpoint
Lesson 05, p. 50

follow-up owner
Lesson 08, p. 84

frequency
Lesson 03, p. 29

functional impact
Lesson 02, p. 15

health literacy
Lesson 06, p. 61

height
Case 2, p. 99

hemolysis
Lesson 04, p. 39

historical entry
Lesson 03, p. 28

identity check
Lesson 07, p. 74

identity match
Lesson 01, p. 6

incorrect notification
Lesson 08, p. 83

instruction source
Lesson 06, p. 61

Vocabulary index / continued

insufficient quantity
Lesson 04, p. 39

jargon
Lesson 06, p. 60

lead-off warning
Case 3, p. 104

measurement time
Lesson 02, p. 16, Case 2, p. 99

medication reconciliation
Lesson 03, p. 28

name pronunciation
Lesson 01, p. 6

objective finding
Lesson 02, p. 15

omission
Lesson 03, p. 28

original entry
Lesson 08, p. 84

outstanding action
Lesson 08, p. 84

oxygen saturation
Lesson 02, p. 16

patient identifier
Lesson 01, p. 5

patient report
Lesson 03, p. 28

pertinent negative
Lesson 02, p. 17

postanalytical phase
Lesson 04, p. 39

preferred name
Lesson 01, p. 5

presenting concern
Lesson 01, p. 5

pulse rate
Lesson 02, p. 16

question handoff
Lesson 06, p. 62

reassurance
Lesson 07, p. 74

receipt time
Lesson 04, p. 38

receiving service
Lesson 05, p. 50

interpretation
Lesson 06, p. 61

language access
Lesson 06, p. 61

local medication list
Lesson 03, p. 28

medication history
Lesson 03, p. 28

message correction
Lesson 08, p. 83

new concern
Lesson 07, p. 73

observation
Lesson 02, p. 15

onset
Lesson 02, p. 15

outside instruction
Lesson 03, p. 28

over-the-counter medicine
Lesson 03, p. 29

patient expectation
Lesson 01, p. 6

patient question
Lesson 06, p. 61

patient-reported symptom
Lesson 02, p. 15

plain language
Lesson 06, p. 60

preanalytical phase
Lesson 04, p. 38

prescribing clinician
Lesson 03, p. 28

private discussion
Lesson 06, p. 61

qualified interpreter
Lesson 06, p. 61, Case 1, p. 94

read-back
Lesson 08, p. 83

receipt acknowledgment
Lesson 05, p. 50

receiving clinician
Lesson 07, p. 73

record amendment
Lesson 04, p. 39

Vocabulary index / continued

recorded name
Lesson 01, p. 5

referral
Lesson 05, p. 50

referral coordinator
Lesson 05, p. 50

referral identifier
Lesson 05, p. 50

registration
Lesson 01, p. 5

rejection criteria
Lesson 04, p. 39

requisition
Lesson 04, p. 38

respiratory rate
Lesson 02, p. 16

results discussion
Lesson 08, p. 83

revised plan
Lesson 06, p. 62

route
Lesson 03, p. 29

safety-net advice
Lesson 07, p. 74

schedule of record
Lesson 08, p. 83

scope of practice
Lesson 01, p. 6

service disruption
Lesson 08, p. 84

short segments
Case 1, p. 94

Signal quality
Case 3, p. 104

source measurement
Case 2, p. 99

specimen
Lesson 04, p. 38

specimen label
Lesson 04, p. 38

spoken-language preference
Case 1, p. 94

subjective information
Lesson 02, p. 15

red flag
Lesson 07, p. 74

referral completion
Lesson 05, p. 51

referral criteria
Lesson 05, p. 51

referring clinician
Lesson 05, p. 50

reinterpretation
Lesson 06, p. 60

repeat acquisition
Case 3, p. 104

rescheduling
Lesson 08, p. 83

result interpretation
Lesson 08, p. 83

reviewing clinician
Case 3, p. 104

rooming
Lesson 02, p. 15

routine appointment
Lesson 07, p. 73

SBAR
Lesson 07, p. 74

scheduled purpose
Lesson 01, p. 6

sent status
Lesson 05, p. 50

severity
Lesson 02, p. 15

show-me method
Lesson 06, p. 60

source attribution
Lesson 02, p. 17

specialist office
Lesson 05, p. 50

specimen integrity
Lesson 04, p. 39

specimen source
Lesson 04, p. 38

strength
Lesson 03, p. 29

successful contact
Lesson 08, p. 84

Vocabulary index / continued

supervision
Lesson 01, p. 6

supporting documentation
Lesson 05, p. 51

symptom onset
Lesson 07, p. 74

symptom progression
Lesson 07, p. 74

teach-back
Lesson 06, p. 60, Case 1, p. 94

technical term
Lesson 06, p. 60

temperature
Lesson 02, p. 16

temperature condition
Lesson 04, p. 39

time-zone reference
Lesson 08, p. 83

translation
Lesson 06, p. 61

transport interval
Lesson 04, p. 38

transport medium
Lesson 04, p. 39

triage
Lesson 07, p. 73

turnaround time
Lesson 04, p. 39

twenty-four-hour time
Lesson 08, p. 83

unassessed
Lesson 02, p. 16

under review
Lesson 05, p. 50

understanding check
Lesson 06, p. 61

unit conversion
Case 2, p. 99

unit of measurement
Lesson 02, p. 16

urgency assessment
Lesson 07, p. 73

verification source
Lesson 03, p. 29

verified callback number
Lesson 07, p. 74

video interpreting
Case 1, p. 94

visit agenda
Lesson 01, p. 5

visit plan
Lesson 01, p. 6

vital signs
Lesson 02, p. 16

waiting list
Lesson 05, p. 51

warm transfer
Lesson 07, p. 73

written-language preference
Case 1, p. 94

References and publication notes

All cases, organizations, figures, and conversations are fictional teaching material, not transcripts. Communication practice does not replace qualified advice or current workplace procedures.

[1] American Association of Medical Assistants. State Scope of Practice Laws.

US context for jurisdiction-dependent duties and delegation. The fictional roles in this book do not authorize clinical work. Source checked 10 October 2026.

https://www.aama-ntl.org/publications/state-scope-of-practice-laws

[2] AHRQ. Use the Teach-Back Method: Tool 5.

Communication context for checking an explanation without shaming the patient. Clinical instruction questions remain with the appropriately authorized professional. Source checked 10 October 2026.

https://www.ahrq.gov/health-literacy/improve/precautions/tool5.html

[3] AHRQ. Make Referrals Easy: Tool 21.

Background on referral coordination, clear instructions, and follow-up. Original referral cases separate receipt, review, booking, and completed attendance. Source checked 10 October 2026.

https://www.ahrq.gov/health-literacy/improve/precautions/tool21.html

[4] CDC. Collect Adult Blood Culture Sets.

A specific laboratory example distinguishing collection documentation and specimen receipt. The book does not teach blood collection or universal specimen handling. Source checked 10 October 2026.

https://www.cdc.gov/lab-quality/php/preventing-adult-blood-culture-contamination/collect.html

Companion course

The website offers the eight-lesson course and related practice. This book can be used independently. No audio access or external account is needed for its exercises.

https://englishladder.com/efsp-medical-assistants.html

Listen carefully. Clarify the details.

Eight clinic cases: visit expectations, patient reports, medication lists, specimen records, referrals, instruction questions, clinical calls, and appointment corrections.

Clarifying the visit at check-in

An appointment label describes a scheduled category, not a clinical judgment about every concern raised that day. Do not promise that the concern will be resolved, reject it automatically, or invent a billing outcome from the schedule alone.

Rooming and factual patient reports

Not reported does not mean absent, and not measured does not mean normal. A pertinent negative requires a relevant question or assessment. Do not populate a record from expectation or convert an empty field into reassurance.

Medication histories with clear sources

An active entry can reflect the state of a record rather than a verified current instruction. Conversely, a patient report deserves documentation but does not automatically complete reconciliation. Preserve both until the authorized review resolves the discrepancy.

Laboratory paperwork and specimen queries

Collection, courier arrival, laboratory receipt, analysis, and result release are distinct events. Do not assume that arrival on a transport sheet means formal laboratory receipt unless the record defines it that way. Always preserve the stated event label.

Phil Stilwell

© 2026 Phil Stilwell. All rights reserved.

No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the author, except as permitted by law.

English Ladder / English for Work / Revised 2026 edition

This publication includes AI-generated text and cover illustration, revised and checked during production. Brand artwork remains the English Ladder mark. This edition makes no claim of independent professional certification.

Typeface: Bitstream Vera, embedded under its accompanying license. Illustration created with OpenAI's native image generator for this edition. The copyright notice is not a claim to ownership of third-party frameworks or references.